



Pediatrics

QBank Review Team

الشعبة: **B**

اللهم لا سهل إلا ما جعلته سهلاً وأنت تجعل الحزن إذا شئت سهلاً

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1. A 4-week-old male infant presents with progressive, non-bilious projectile vomiting after feeds. The infant remains hungry after vomiting. Physical examination reveals mild dehydration and a small, firm mass in the epigastrium. Laboratory studies show hypochloremic metabolic alkalosis.

What is the most likely diagnosis and appropriate management?

- A. Sepsis; intravenous antibiotics and fluid resuscitation
- B. Intestinal malrotation; urgent exploratory laparotomy
- C. Gastroesophageal reflux disease; reassurance and feeding modification
- D. Pyloric stenosis; correct electrolytes followed by surgical pyloromyotomy

2. A 6-month-old infant presents with diarrhea and poor feeding. Serum sodium level is 165 mEq/L.

What is the most appropriate rehydration strategy?

- A. Rapid correction over 6–8 hours
- B. Bolus followed by rapid hypotonic fluid
- C. Rehydration over 48 hours
- D. Rehydration over 24 hours

3. A 14-month-old girl has had difficulty breathing since last night. Her parents report nasal congestion, a hoarse cough, and subjective fevers for the past 2 days. Last night, they also noticed “noisy breathing.” The girl has been drinking normally and making good wet diapers. Vital signs: respiratory rate 38/min, heart rate 120 bpm, temperature 38.4°C. She has nasal congestion on exam and inspiratory stridor when she is crying that resolves when she becomes calm. Her breath sounds are clear except for transmitted upper airway noise.

What is the most appropriate next step for this patient?

- A. Prescribe oral dexamethasone
- B. Oral course of amoxicillin
- C. Administer racemic epinephrine
- D. Neck radiograph

4. A 4-day-old newborn has lost 5% of their birth weight. The baby is breastfeeding well and has adequate urine output.

What is the most appropriate interpretation?

- A. Indicates poor milk supply
- B. Abnormal weight loss
- C. Suggests dehydration
- D. Normal physiological weight loss

5. The finding of marked leukocytosis in the presence of severe recurrent necrotizing bacterial infections in a child with a history of delayed separation of the umbilical cord most likely suggests:

- A. A leukocyte adhesion defect
- B. Chronic granulomatous disease
- C. Shwachman-Diamond syndrome
- D. Agammaglobulinemia

6. An 11-month-old boy has been experiencing a productive cough and congestion for 4 days. He was born full-term and his medical history is unremarkable. His parents report that his cough has been gradually worsening and that he has now developed a wheeze. He feeds slightly less than usual but continues to make wet diapers. On examination: temperature 37.5°C, heart rate 130 bpm, respiratory rate 30/min, blood pressure 85/60 mmHg, oxygen saturation 95% on room air. He looks well and in no acute distress. Lung exam reveals bilateral mild subcostal retractions and bilateral intermittent wheezing. Which one of the following next steps in care is most appropriate for this patient?

- A. Obtain chest X-ray
- B. Provide supportive care in the outpatient setting
- C. Obtain nasopharyngeal swab for virologic testing
- D. Refer to nearest emergency department

7. A 6-year-old child is admitted for elective surgery. He is well-hydrated and afebrile, with normal glucose and sodium levels.

What is the most appropriate type of maintenance IV fluid prescription?

- A. Ringer's lactate at maintenance rate
- B. 5% dextrose in 0.2% saline
- C. 5% dextrose in water
- D. 5% dextrose in 0.9% saline

8. A 9-year-old boy presents with recurrent sneezing, nasal itching, clear rhinorrhea, and nasal congestion for several months, worse in the spring. Physical examination shows pale, boggy nasal turbinates. Which of the following is the most appropriate diagnostic and management approach?

- A. Measure serum IgE levels only and prescribe oral decongestants
- B. Order nasal culture and start oral antibiotics
- C. Request CT scan of sinuses and start systemic corticosteroids
- D. Perform allergy testing if needed and initiate intranasal corticosteroids

9. A 9-year-old girl is brought to the emergency department after taking an unknown number of her grandmother's aspirin tablets for a headache approximately 4 hours ago. Since then, she has had vomiting, rapid breathing, and complaints of "loud ringing" in her ears.

Vital signs:

- HR: 124/min
- RR: 34/min
- Temp: 38°C
- BP: 100/60 mmHg
- SpO₂: 99% on room air

Exam:

- Breathing rapidly and deeply
- Warm, flushed skin
- Diaphoretic
- Alert but anxious
- Mild epigastric tenderness

Labs:

- Salicylate level: 45 mg/dL
- ABG: pH 7.48
- HCO₃⁻: 16 mEq/L
- K⁺: 3.2 mEq/L
- Anion gap: elevated

What is the next management step?

- A. Give only IV fluids and repeat salicylate level in 6 hours
- B. Start activated charcoal and administer IV sodium bicarbonate infusion
- C. Intubate to correct hyperventilation
- D. Start hemodialysis immediately

10. A 10-month-old infant presents with profuse watery diarrhea for 3 days. He is dehydrated and tachypneic.

Arterial blood gas shows:

- pH: 7.18
- PaCO₂: 30 mmHg
- HCO₃⁻: 14 mEq/L

What is the most appropriate interpretation?

- A. Mixed respiratory and metabolic alkalosis
- B. Metabolic acidosis due to bicarbonate loss
- C. Respiratory alkalosis due to hyperventilation
- D. Metabolic alkalosis due to volume depletion

11. A 6-year-old boy is brought to your clinic by his mother due to a recurrent cough and wheezing over the past 4 months. She reports that his symptoms are often triggered by playing football or catching a common cold.

On physical examination, he appears well, but you hear mild end -expiratory wheezing. No fever or signs of acute illness are present. Chest X -ray is normal.

Which of the following features most supports a diagnosis of asthma in this child?

- A. Productive cough with green sputum for 3 weeks
- B. Cough and wheezing that improve with bronchodilator therapy
- C. Persistent daily cough without wheezing
- D. Fever and chills with chest pain

12. A 6-year-old child presents with chronic constipation and painful defecation. The parents report stool-withholding behavior. Examination reveals a normal anus and a fecal mass in the abdomen. There is occasional bright red blood on the toilet paper.

What is the most likely diagnosis and initial management?

- A. Functional constipation; stool softeners and behavioral modification
- B. Hirschsprung disease; rectal biopsy
- C. Inflammatory bowel disease; colonoscopy and steroids
- D. Meckel diverticulum; surgical resection

13. An 8-year-old girl is brought to the physician by her mother because of a 6 -month history of episodic dry cough, shortness of breath, and chest tightness. She has seasonal allergic rhinitis. Physical examination shows high-pitched expiratory wheezes throughout both lungs. Pulmonary function testing shows an FEV₁ of 70% (N $\geq$ 80%). Which of the following drugs would be most effective at reducing bronchial inflammation in this patient?

- A. Oral theophylline
- B. Inhaled glucocorticoid
- C. Inhaled ipratropium bromide
- D. Inhaled salmeterol

14. A 6-year-old child with a history of eczema presents to the emergency department with sudden onset of generalized urticaria, lip swelling, wheezing, and hypotension shortly after eating peanuts.

What is the most appropriate initial management, and what is the key long -term precaution?

- A. Give intramuscular epinephrine; prescribe an epinephrine auto-injector and provide an emergency action plan
- B. Give nebulized salbutamol; reassure parents that this is a self-limited reaction
- C. Administer oral antihistamines; advise avoidance of peanuts only
- D. Start intravenous corticosteroids; perform skin -prick testing immediately

15. A 25-day-old full-term infant presents with progressive jaundice. The parents noticed that the baby's stool looked clay -colored. On exam, the baby has hepatomegaly. Laboratory results reveal total bilirubin 15 mg/dL, with direct (conjugated) bilirubin 9 mg /dL.

Which of the following is the most likely diagnosis?

- A. Breast milk jaundice
- B. Hemolytic disease of the newborn
- C. Physiological jaundice
- D. Biliary atresia

16. A 7-year-old boy has bedwetting at night but is dry during the day. He has no urinary symptoms and a normal examination. What is the most appropriate first step?

- A. Order renal ultrasound
- B. Reassure parents and initiate behavioral therapy (bedwetting alarm)
- C. Diagnose urinary tract infection
- D. Start anticholinergic medication

17. A 9-year-old girl presents to the emergency room with fever, erythema, tenderness, and edema over her left eye. On examination, there is limited ocular movement and decreased visual acuity. What is the most appropriate management step?

- A. To administer intravenous immunoglobulin

- B. To treat with antibiotics and urgent ophthalmological consultation
- C. To start epinephrine immediately
- D. To do antinuclear antibodies (ANAs) level

18. A 2-year-old child came for pediatrics clinic for routine checkup for developmental assessment. He is expected to be able to:

- A. Draw a square
- B. Ride a tricycle and tie their shoes
- C. Use 10 words and combine them into long sentences
- D. Run and kick a ball

19. Mother breastfeed her child every (2-3h), choose the appropriate answer:

- a. Its appropriate to breastfeed on demand
- b. Feeding every (2-3h) is considered excess
- c. The mother should wait for the child to cry
- d. The mother should breastfeed the child more

20. A 5-year-old boy wakes up suddenly at night appearing frightened and confused, and he does not remember the event. What is the most likely diagnosis?

- A. Night terrors

21. A child with brucellosis was treated with rifampicin but still has fever after 10 days. What is the next step in management?

- A. Continue rifampicin alone for one more week
- B. Add trimethoprim-sulfamethoxazole to rifampicin
- C. Stop rifampicin
- D. Start biologics

22. A 5-month-old infant presents with recurrent infections since 2 months of age including Pneumocystis jirovecii pneumonia and oral thrush. Labs show severe lymphopenia and chest X-ray shows absent thymic shadow. What is the most likely diagnosis?

- A. X-linked agammaglobulinemia
- B. Severe combined immunodeficiency
- C. DiGeorge syndrome
- D. Chronic granulomatous disease

23. A fetus has high risk for Down syndrome and the mother wants a definitive invasive test to confirm trisomy 21. What is the most appropriate investigation?

- A. Non-invasive prenatal testing
- B. CVS or amniocentesis
- C. Nuchal translucency ultrasound
- D. Maternal serum screening

24. A 7-year-old unvaccinated child presents with fever and vesicular rash in different stages of healing. What is the best management?

- A. Oral acyclovir
- B. IV antiviral therapy
- C. Corticosteroids
- D. Antibiotics

25. A patient develops syncope after prolonged standing. ECG and EEG are normal. What is the most likely diagnosis?

- A. Vasovagal syncope
- B. Orthostatic hypotension

26. Which laboratory finding is most consistent with hemolytic anemia?

- A. High reticulocyte count
- B. Low hemoglobin only
- C. Low MCV
- D. Prolonged PT

27. A child developed hematuria shortly after an upper respiratory infection and complement level is normal. What is the most likely diagnosis?

- A. Post-streptococcal glomerulonephritis
- B. IgA nephropathy

28. A healthy 10-week-old infant has Hb 10 g/dL and normal MCV with no symptoms. What is the next step?

- A. Start oral iron
- B. Reassure
- C. Hemoglobin electrophoresis
- D. Coombs test

29. A neonate presents with respiratory distress and scaphoid abdomen. What is the diagnosis?

- A. Congenital diaphragmatic hernia
- B. Pectus excavatum
- C. Pulmonary sequestration
- D. Choanal atresia

30. A child has a hard thyroid nodule with cervical lymphadenopathy. What is the next step?

- A. Thyroid ultrasound + FNA
- B. Observation
- C. Antibiotics
- D. Reassure

31. A child started a new antiepileptic drug and after 2–3 weeks developed fever, rash, eosinophilia, and elevated liver enzymes. What is the diagnosis?

- A. DRESS
- B. SJS
- C. TEN
- D. AGEF

32. A neonate has metabolic acidosis, hypoglycemia, and mild hyperammonemia with family history of neonatal death. What is the most likely diagnosis?

- A. Organic acidemia
- B. Urea cycle defect
- C. Branched-chain amino acid disorder
- D. Fatty acid oxidation defect

33. A child has seizures lasting 3 minutes only during fever. What is the diagnosis?

- A. Simple febrile seizure
- B. Complex febrile seizure
- C. Status epilepticus
- D. Absence seizure

34. A child has brief staring spells with eyelid fluttering and generalized spikes on EEG. What is the diagnosis?

- A. Juvenile absence epilepsy
- B. Childhood absence epilepsy
- C. Focal epilepsy

D. Myoclonic epilepsy

35. A child can sit with support and shows interest in food. What is the most appropriate feeding recommendation?

- A. Start cow's milk
- B. Start iron-rich complementary foods
- C. Delay solids until 9 months
- D. Avoid allergenic food

36. A child has proteinuria negative in morning and +2 in afternoon. What is the diagnosis?

- A. Orthostatic proteinuria
- B. Nephrotic syndrome
- C. Transient proteinuria

37. A 4-month-old infant with fever and gram-positive diplococci. What is the organism?

- A. *Streptococcus pneumoniae*
- B. Group A streptococcus
- C. Group B streptococcus

38. A 9-month-old introduced to cow milk develops anemia. What is the diagnosis?

- A. Iron deficiency anemia

39. A 1-year-old starting solid food. What should be introduced?

- A. Iron-rich food
- B. Avoid allergenic food

40. Schistocytes on peripheral smear after diarrhea. What is the causative organism?

- A. *E. coli* O157:H7
- B. Non-typhoid salmonella
- C. *Entamoeba histolytica*

41. A child has limited hip abduction since birth. What is the diagnosis?

- A. Perthes disease
- B. Developmental dysplasia of hip

42. How to screen a 25-week preterm for IVH?

- A. MRI
- B. CT
- C. Cranial ultrasound
- D. EEG

43. A seizure lasting 7 minutes. What is the classification?

- A. Status epilepticus

44. A patient develops hypersensitivity after antiepileptic drug. What is the diagnosis?

- A. DRESS

45. High anion gap metabolic acidosis in child. What is the cause?

- A. Urea cycle defect
- B. Organic acidemia

46. Maple syrup urine disease is due to deficiency of:
47. Full-term neonate with unilateral chest distension and respiratory distress. Diagnosis?
A. Pneumothorax
B. Meconium aspiration
C. Diaphragmatic hernia
48. Case about CMV case with jaundice. Intra hepatic?
A. Neonatal hepatitis
B. Gallbladder obstruction
C. Biliary atresia
49. Severely dehydrated child weighing 12 kg. What is fluid bolus?
A. 240 ml NS
50. Child with obesity. Most likely complication?
A. Obstructive sleep apnea
51. Child with ADHD. Key diagnostic criterion?
A. Functional impairment
52. Child rides tricycle at what age?
A. 2 years
B. 3 years
C. 4 years
53. A child with bronchiolitis symptoms (cough, wheeze, winter). Diagnosis?
A. Asthma
B. Bronchiolitis
54. Physiological change after birth during transition:
A. Decrease pulmonary vascular resistance
55. Most common constipation type in a 1-year-old:
A. Functional constipation
56. Child with abdominal mass, hypertension, hematuria. Diagnosis?
A. Wilms tumor
57. A child presents with arthritis and associated eye involvement (uveitis). Which laboratory finding is most likely to be positive?
A. Antinuclear antibody (ANA)
58. A patient presents with a characteristic malar rash over. What is the most likely diagnosis?
A. Systemic lupus erythematosus (SLE)
59. An unvaccinated child is exposed to measles but does not develop the disease because most children in the community are vaccinated. What is this concept called?
A. Community protection (مدري إذا مبيسلز او لا لكن نفس الفكره بالضبط)

60. A child is found to have elevated phenylalanine levels. What is the most likely diagnosis?

A. Phenylketonuria (PKU)

61. A 4-week-old male newborn is brought to the physician for a well-child examination. He was born at 40 weeks' gestation and weighed 3 kg. He now weighs 4.3 kg. There is no family history of serious illness. He is at the 50th percentile for length and weight. Vital signs are within normal limits. Examination shows an ejection systolic murmur best heard at the upper left sternal edge, grade 3/6 and a fixed and widely split-second heart sound. The lungs are clear to auscultation. The remainder of the examination shows no abnormalities. Which of the following is the most likely explanation for this patient's clinical findings?

A. Left-to-right shunt through the ventricular septum

B. Left-to-right shunt through the atrial septum

C. Right-to-left shunt through the ventricular septum

D. Right-to-left shunt through the atrial septum

62. While you were reviewing multiple medical files for different pediatric patients, you decided to analyze the growth chart of each. Which one will be considered abnormal?

A. Six-month-old girl with her height corresponding to the 20th percentile

B. Seven-year-old boy with his weight dropping from the 90th % to the 25th % percentile over one year

C. Three-month-old baby with a weight corresponding to the 10th percentile

D. Ten-year-old with BMI (body mass index) less than the 50th percentile

63. You diagnosed a 14-year-old boy with constitutional growth delay. The parents asked you about this diagnosis and if it affects the boy's final height. Which of the following are consistent with this diagnosis?

A. Prognosis for a normal adult height is good

B. The child was undernourished during his first year of life

C. The child was small for size at birth

D. Bone age is normal compared to chronological age

64. A 10-year-old boy presented with swelling in the face developed 8 days after tonsillitis treatment. You suspected acute post-streptococcal glomerulonephritis. Which of the following findings is most commonly associated with post-streptococcal glomerulonephritis?

A. High serum IgA

B. High Anti-GBM antibody

C. Blood clots in the urine sample

D. Low serum C3 levels

65. A 3-year-old child came to the pediatric clinic for developmental assessment. He should be able to?

A. Skip with both feet

B. Write their name

C. Hop on one foot

D. Ride a tricycle

66. A 5-year-old boy presented to the ER with generalized edema. You suspected nephrotic syndrome. Which of the following is mandatory for the diagnosis of Nephrotic Syndrome?

A. Presence of edema

B. Urinary protein excretion greater than 100 mg/m²/day

C. Hyperlipidemia

D. Serum albumin concentration less than 3 g/dL

67. A 3-month-old infant with fever has a catheterized urine sample that shows 100,000 CFU/mL of *Klebsiella pneumoniae* and ≥ 5 WBC/HPF on urine microscopy. How should this finding be interpreted?

A. Likely asymptomatic bacteriuria

B. Normal finding in this age group

C. Contaminated sample; repeat the test

D. Urinary tract infection requiring treatment

68. Four-month-old male infant presents with tachypnea, tachycardia, and hepatomegaly. On auscultation, he has a soft pansystolic murmur at the lower left sternal border. A chest radiograph shows cardiomegaly and increased pulmonary markings. There are normal oxygen saturations in all 4 extremities. Which of the following is the most likely diagnosis in this patient?

- A. Inborn error of metabolism
- B. Large ventricular septal defect
- C. Small ventricular septal defect
- D. Tetralogy of Fallot

69. A 16-year-old boy came to the pediatric clinic for developmental assessment. What is the initial physical manifestation of puberty in males?

- A. Testicular enlargement
- B. Growth spurt
- C. Emergence of early pubic hair
- D. Thelarche

70. A 15-year-old female presents with a 2-month history of fatigue, generalized joint and muscle pain, and a malar rash. Labs show positive ANA, low complement levels, and proteinuria on urinalysis. What is the most likely diagnosis?

- A Ankylosing spondylitis
- B Systemic lupus erythematosus
- C Sjogren's syndrome
- D Rheumatoid arthritis

71. At a 4-month well-child visit, a mother is distressed because her baby's grandmother believes she should have started feeding her baby solid food at two months of age. What is the recommended age to introduce foods other than milk into the infant's diet?

- A 12 months
- B 8 months
- C 6 months
- D 10 months

72. An infant is born to an HIV-infected mother. The mother received effective antiretroviral therapy (ART) during pregnancy, and undetectable at delivery. The infant is started on post-exposure prophylaxis. Which of the following diagnostic approaches is used to determine the HIV infection status of this infant in the first few months of life?

- A- HIV ELISA antibody test at 1 month of age.
- B- HIV DNA PCR testing at 2 weeks, 1-2 months, and 4-6 months of age.
- C- HIV RNA PCR (viral load) at 6 weeks of age as the sole diagnostic test.
- D- CD4+ T-cell count at birth and monthly thereafter.

73. A 2-year-old boy presents with a history of vomiting and diarrhea for several days. Physical examination reveals lethargy, tachypnea, and poor skin turgor. He refuses to take oral fluids, so you decide to begin intravenous fluid administration. What is the initial management?

- A- Normal saline at 10 mL/kg
- B- Bolus of isotonic solution at 20 mL/kg
- C- Dextrose and 0.45% normal saline at the maintenance rate
- D- Wait for laboratory results for serum electrolytes

74. A 6-month-old presents with a 3-day history of high fever followed by the appearance of a maculopapular rash as the fever subsided. What is the most likely diagnosis?

- A- Measles
- B- Scarlet fever
- C- Roseola infantum (HHV-6)

D- Rubella

75. A 3-year-old boy is brought to the emergency department by his parents with fever, barking cough, and stridor. He had a history of upper respiratory tract infection symptoms two days ago. On examination, the patient was vitally stable, and chest examination revealed inspiratory stridor with no other findings. What is the most appropriate next step?

- A- Start a course of oral Augmentin
- B- Reassure the parents that the febrile illness is due to croup and send the patient home
- C- Give one dose of dexamethasone
- D- Shift the patient to the operating room to secure the airway

76. A 3-year-old boy presents with recurrent skin abscesses caused by *Staphylococcus aureus* and several episodes of *Aspergillus* pneumonia. His past medical history is significant for delayed separation of the umbilical cord (at 5 weeks of age) and recurrent gingivitis. Physical examination reveals poor wound healing. A complete blood count shows marked neutrophilia. Which underlying immunodeficiency is most consistent with this clinical picture?

- A- Leukocyte Adhesion Defect (LAD)
- B- Severe Combined Immunodeficiency (SCID)
- C- X-linked Agammaglobulinemia (XLA)
- D- Complement C3 Deficiency

77. A 6-year-old girl with no significant medical history is evaluated for an 8-month history of intermittent left knee pain with morning stiffness. Her mother has noticed her limping 2 to 3 times per week in the morning or after long car rides. Examination of the left knee demonstrates mild warmth, a moderate joint effusion, and a flexion contracture. Of the following, the laboratory finding most associated with an increased risk of ocular complications in this child is a(n):

- A- Elevated C-reactive protein level
- B- Elevated rheumatoid factor
- C- Microcytic hypochromic anemia
- D- Elevated antinuclear antibody titer

78. A child having episodes of dropping on the floor crying arching his back and screaming when he's denied a toy, he does this twice a week and each episode last around 10 minutes. What the best advice for dealing with this?

- A. Start immediate psychotherapy
- B. Rule out autism
- C. Ignore him during the behavior

79. A nurse is preparing to administer a Bacille Calmette–Guérin (BCG) vaccine to a newborn. By which route should this vaccine be administered?

- A. Subcutaneous
- B. Intramuscular
- C. Intradermal
- D. Oral

80. A 2-month-old infant is brought to the clinic for routine immunizations. He receives a combination vaccine that includes DTaP, IPV, Hib, Hepatitis B, PCV, and Rotavirus. What is the primary benefit of administering these vaccines in a combined form?

- A. Providing multiple antigens at once produces stronger immunity
- B. Reducing the number of injections and clinic visits required
- C. Increasing the likelihood of mild fever and local reaction to stimulate better immune memory
- D. Eliminating the need for future booster doses

81. A 3-week-old infant presents with recurrent episodes of projectile, non-bilious vomiting. The baby is irritable and losing weight. On abdominal examination, a firm, olive-shaped mass is palpated in the right upper quadrant/epigastric region. What is the most likely diagnosis?

- A. Duodenal atresia
- B. Hypertrophic pyloric stenosis
- C. Gastroesophageal reflux disease
- D. Intussusception

82. A 13-year-old girl presents with a history of constipation. She reports passing plate-like stools with streaks of bright-red blood on the surface. She denies abdominal pain, diarrhea, vomiting, or systemic symptoms. What is the most likely diagnosis?

- A. Anal fissures
- B. Peptic ulcer disease
- C. Esophageal varices
- D. Meckel's diverticulum

83. A 7-year-old boy is brought to the emergency department with a 2-day history of fever, vomiting, and rapid breathing. On examination, he appears dehydrated and is breathing deeply (Kussmaul respiration). His labs are as follows:

Na⁺: 138 mEq/L

Cl⁻: 96 mEq/L

HCO₃⁻: 10 mEq/L

K⁺: 4.0 mEq/L

Glucose: 350 mg/dL

The resident decides to calculate the anion gap to help evaluate his metabolic acidosis.

Which of the following is the correct formula for calculating the serum anion gap?

- A. $(\text{Na}^+ + \text{K}^+) - (\text{Cl}^- + \text{HCO}_3^-)$
- B. $\text{Na}^+ - (\text{Cl}^- + \text{HCO}_3^-)$
- C. $(\text{Na}^+ - \text{Cl}^-) \div \text{HCO}_3^-$
- D. $(\text{Na}^+ + \text{HCO}_3^-) - \text{Cl}^-$

84. An 8-year-old girl is brought to the emergency department after developing dizziness and shortness of breath during a school exam. She appears anxious and is breathing rapidly.

Arterial blood gas analysis shows:

pH: 7.55

pCO₂: 22 mmHg

HCO₃⁻: 22 mEq/L

Which of the following is the most likely acid-base disorder?

- A. Respiratory alkalosis due to hyperventilation
- B. Mixed metabolic alkalosis and respiratory alkalosis
- C. Respiratory acidosis with metabolic compensation
- D. Metabolic acidosis with respiratory compensation

85. Parents report that their 7-year-old child frequently wakes up terrified at night and recalls a vivid dream. The next day, the child clearly remembers the dream. Which of the following is true about this case?

- A. The child does not remember the dream
- B. Immediate psychotherapy should be started
- C. He is experiencing nightmares
- D. It occurs during non-REM sleep

86. A 4-year-old boy is brought to the emergency department after being found collapsed at home. On arrival, he is apneic, pulseless, and has cold extremities. What is the best next step in management?

- A. Provide rescue breaths only, as children usually arrest from respiratory failure
- B. Begin chest compressions with two fingers in the center of the chest at a depth of 2 cm
- C. Start chest compressions with one or two hands on the lower half of the sternum, pressing to a depth of ~5 cm (1/3 of chest diameter), allowing full recoil, at a ratio of 15:2 with two rescuers

D. Initiate compressions with both hands on the upper sternum at a depth of 6–8 cm, at a ratio of 30:2 regardless of rescuers

87. A 12-year-old boy presents with abdominal pain, vomiting, and rapid deep breathing. On examination, he is drowsy, tachycardic, hypotensive, and has dry mucous membranes.

Investigations show:

Blood glucose: 420 mg/dL

Arterial pH: 7.12

HCO₃⁻: 10 mEq/L

Na⁺: 130 mEq/L

K⁺: 4.2 mEq/L

Urine: positive for ketones

What is the most appropriate initial step in management?

A. Start intravenous insulin infusion immediately

B. Administer 10–20 mL/kg bolus of 0.9% normal saline

C. Begin potassium replacement

D. Administer sodium bicarbonate infusion

88. A 6-year-old child weighs 12 kg and presents to the emergency department in shock with severe dehydration. The physician decides to begin fluid resuscitation with 0.9% normal saline.

What volume should be given as an initial fluid bolus?

A. 120 mL

B. 180 mL

C. 240 mL

D. 360 mL

89. A 12-year-old Saudi boy from Riyadh presents with fever, sweats, malaise, and arthralgia. His parents report that he recently visited a farm and drank unpasteurized camel's milk.

Which organism is the most likely cause of his illness?

A. *Brucella melitensis*

B. *Brucella abortus*

C. *Brucella suis*

D. *Brucella canis*

90. A newborn infant is noted to have a sacral swelling covered by a thin membrane. The physician suspects a myelomeningocele.

What is the most appropriate next step in diagnosis?

A. Plain X-ray of the spine

B. Ultrasound of the sacral swelling

C. MRI of the spine

D. CT scan of the brain

91. A newborn delivered by vacuum extraction develops a progressive scalp swelling. The swelling crosses suture lines, feels fluctuant, and is increasing in size, now extending to the orbits and the nape of the neck. The infant becomes pale and tachycardic.

What is the most likely diagnosis?

A. Caput succedaneum

B. Cephalohematoma

C. Subgaleal hemorrhage

D. Intracranial hemorrhage

92. A 9-year-old child with absence seizures was started on ethosuximide therapy. He has now been seizure-free for 2 years.

What is the next best step in management?

A. Continue AEs for life

- B. Stop AEs immediately
- C. Gradually taper and discontinue AEs
- D. Switch to valproic acid for maintenance

93. A 7-year-old girl has had vomiting and diarrhea for 3 days. The vomiting stopped yesterday, but the diarrhea continues. She appears well-hydrated, her vital signs are normal, and her examination is unremarkable. Her father asks what dietary changes should be made.

What is the most appropriate advice?

- A. Discontinue solids until the diarrhea stops
- B. Continue with an age-appropriate diet
- C. Give only clear fluids until symptoms resolve
- D. Avoid milk and dairy products until diarrhea stops

94. A 10-year-old boy with asthma is maintained on an inhaled corticosteroid (ICS) and uses salbutamol as a reliever. He typically uses his medications about 3 days per week. He has good inhaler technique and adherence. In the past year he had two exacerbations triggered by viral infections that required oral corticosteroids but otherwise remains stable with no daily symptoms.

What is the most appropriate next step in management?

- A. Continue his current regimen
- B. Add a long-acting beta agonist (LABA) to his treatment
- C. Stop ICS and switch to LABA monotherapy
- D. Escalate to systemic corticosteroids for prevention

95. A 3-day-old male infant, born vaginally without complications, was discharged home shortly after birth. He is now noted to be jaundiced. Laboratory tests reveal predominantly unconjugated hyperbilirubinemia. He is otherwise healthy, feeding well, and his examination is unremarkable.

What is the most likely diagnosis?

- A. Hemolytic disease of the newborn
- B. Breastfeeding jaundice
- C. Physiological jaundice
- D. Biliary atresia

96. A 6-week-old infant presents with persistent jaundice. The parents report that his stools are clay-colored and his urine is dark. On examination, the baby appears otherwise well.

What is the most likely diagnosis?

- A. Physiological jaundice
- B. Breast milk jaundice
- C. Hemolytic disease of the newborn
- D. Biliary atresia

97. A 7-year-old boy is brought to the emergency department with a 6-day history of persistent fever reaching up to 40.2°C. On examination, he has tender cervical lymphadenopathy and a generalized maculopapular rash. His blood pressure is 80/45 mmHg, heart rate 140 bpm, and respiratory rate 32/min. Laboratory investigations show sodium 129 mEq/L, potassium 3.1 mEq/L, and CRP 120 mg/L. Despite these acute findings, the most concerning long-term complication of his condition is which of the following?

- A. Coronary artery aneurysm
- B. Septic shock
- C. Rheumatic heart disease
- D. Acute glomerulonephritis

98. A 12-year-old girl presents to the pediatric clinic for a routine follow-up. She is healthy with no complaints. On examination, her blood pressure is found to be between the 90th and 95th percentile for her age, sex, and height. What is the next best step in management?

- A. Start antihypertensive medication
- B. Refer to pediatric nephrology

- C. Repeat the blood pressure measurement on at least two additional occasions
- D. Order renal ultrasound immediately

99. Baby girl, she can transfer object, sit with support, mimicking voices what is her age?

- A. 12 months
- B. 2 months
- C. 6 months
- D. 4 months

100. Case of Neonate has been 46 hour without passing meconium, He had bilious vomiting(green).

- A. Duodenal atresia
- B. Hirschsprung disease
- C. Necrotic enterocolitis
- D. Pyloric stenosis

101. Case of patient with RDS symptoms. What is the cause for this condition?

- A. Surfactant deficiency

102. Case of newborn that is unstable, they have given him positive pressure, then chest compressions, it showed slightly improvement but still tachycardiac. What is the best next step?

- A. Give epinephrine by ET tube until vascular access ready

103. Case of patient with disorder of sex development. Which of the following labs confirm congenital adrenal hyperplasia?

- A. High 17-Hydroxyprogesterone.
- B. High Cortisol.

104. 2-year-old girl came to your clinic for a follow-up after her first febrile urinary tract infection (UTI); she finished the antibiotics course yesterday. All her symptoms resolved, and her examination was unremarkable. What is the next best step in her management?

- A- Reassurance and follow her in the clinic after 2-6 weeks
- B- Do a Renal and bladder ultrasound (RBUS) after 2-6 weeks
- C- Send another urine culture to document the sterility
- D -Do a Renal and bladder ultrasound (RBUS) today

105. A baby delivered by caesarean section because of ruptured membranes, his apgar score were 1-3 at 3-5 minutes respectively. PPV, intubation and chest compression were done and IV EPINEPHRINE was given. What measurement should be done to preserve neurological function?

- A. Therapeutic hypothermia
- B. Mannitol
- C. Antibiotics
- D. Thiamine

106. full-term infant presents with acrocyanosis but no respiratory distress. His oxygen saturation is 98%, and his mucus membranes remain pink. What is the most appropriate next step in management?

- A Obtain an echocardiogram
- B Start prostaglandin E1 infusion
- C Perform a hyperoxia test
- D Reassure parents and provide routine neonatal care

107. Case of 9-month-old that his parents saying that he is not growing well. He is exclusively breastfeeding. His mother says he is feeding well. The examination showed was normal and he was alert. Which of the following is the reason of failure to thrive?

- A. Calories intake

- B. Psychosocial
- C. Gastric malabsorption

108. Patient came with hypotonia and Muscle weakness of proximal lower limb. his mother has same problem and there are some family members from mother side having same presentation, what is the mode of inheritance?

- A. X-Linked Dominant
- B. Autosomal Dominant
- C. Autosomal Recessive
- D. Mitochondrial

109. Case about a boy with a previous history of Upper respiratory tract infection 1 week ago, as well as he didn't go to toilet for the past 3 days. presents to you now with an odd smelling urine. Which of the following is the strongest risk factor for this condition?

- A. URTI
- B. Male gender
- C. Constipation
- D. Posterior Urethra Valve

110. A 5-year-old boy presents with chronic diarrhea, abdominal pain, and failure to thrive. His mother reports that he often complains of bloating after meals. The family history is significant for autoimmune diseases. A serological test reveals elevated levels of tissue transglutaminase antibodies (tTG-IgA). Based on this scenario, what is the most appropriate next step in managing this child?

- A Referral for an upper gastrointestinal endoscopy with biopsy
- B Testing for other food allergies
- C Immediate initiation of a gluten-free diet
- D Prescribing anti-inflammatory medication

111. 6-year-old boy is brought to the pediatric clinic by his parents due to concerns about his growth and behavior. They report that he has been increasingly irritable and has episodes of confusion. The family history reveals that the boy's mother has a sister with a metabolic disorder. Upon examination, the boy is noted to have a significantly low weight for his age, and his height is below the 5th percentile. Laboratory tests reveal:

Blood glucose: 50 mg/dL (normal: 70-100 mg/dL)

Serum ammonia: 200 umol/L (normal: 15-50 umol/L)

Urine organic acids: positive for elevated levels of branched-chain amino acids.

What is the primary mechanism of the disorder suspected in this case?

- A Defective glucose transport
- B Excessive glucose production by the liver
- C Enzyme deficiency leads to the accumulation of toxic metabolites
- D Insulin resistance

NOT
sure!

112. A 9-month-old girl had fever for the past 3 days (40c), she developed a maculopapular rash after her fever went away. What organism is the most likely cause of this infection?

- A. Roseola infantum
- B. Parvovirus B19
- C. Measles virus
- D. Varicella zoster virus

113. 5-year-old child presents to the clinic with a high fever, cough, runny nose, and red eyes. The mother reports that the child has had a rash that started on the face and spread downwards to the trunk and extremities over the past three days. Upon examination, the rash is red, blotchy, and slightly raised, with some areas merging. Which of the following characteristics best describes the skin rash associated with measles in this scenario?

- A The rash is characterized by small, flat, red spots that do not blanch when pressed.
- B The rash consists of vesicular lesions that are itchy and crusty over time.

C The rash is maculopapular, begins on the face, and spreads to the trunk and limbs, often merging into larger areas.

D The rash is a well-defined, scaly plaque in a butterfly distribution on the face.

114. Newborn is delivered at term. Apgar scores are 6 and 8 at 1 and 5 minutes, respectively. Examination showed that he is at the 2n percentile for head circumference and at 10 percentile for length. He has a small Jaw, small eyes, and low-set ears. There is a keyhole-shaped defect in the iris of the left eye. He has seven fingers on each hand. There is a cleft palate. A holosystolic murmur is heard at the left lower sternal border. The heels are prominent, with convex-shaped deformity of plantar surface of the feet.

Which of the following is the most likely cause of these finding?

A Trisomy 13

B Fragile X syndrome

C Trisomy 18

D Trisomy 21

115. Case of patient with light hair and musty urine, he was diagnosed phenylketonuria. What is the management?

C. Phenylalanine free diet

116. Patient with poor controlled diabetes HBA1c will be high after which of the following?

A. 8 hours

B. 1 week

C. 1 months

D. 3 months

117. two weeks-old preterm baby is noted to have bloody stool. Her birth weight was 800 grams. She was given formula milk from day two of life and now feeding 14 ml every 2hours. She passed stools at birth. She looks pale, and sick looking. Her abdomen is distended and tender. Plain abdominal radiography showed air in the bowel wall. Her vital sign : temperature 36.8 heart rate : 160/min, Blood pressure 40/20mmg, respiratory rate 50/min. What is the MOST likely diagnosis ?

A Meconium Ileus

B Hirschsprung Disease

C Necrotizing Enterocolitis

D Cystic fibrosis

118. Case about a mom who found an abdominal mass for her daughter, she's hypertensive and have hematuria. What is the diagnosis?

C. Wilms tumor

D. Nueroblastoma

119. A 30 kg child what is the maintenance?

A-1700

B-1500

120. 4 weeks infant on formula since birth with yellowish sclera and skin, poor feeding, vomiting, dark urine,pale stool. Bilirubin test showed Total Bilirubin 157, high direct bilirubin, imaging test was done. Which of the following is most important to investigate?

A. Biliary atresia

B. Hemolytic blood disorder

C. Physiological breastfeeding hyperbilirubinemia

121. A 2 months old (long case) subdural hematoma is seen what is the most likely cause ?

A-Non-accidental trauma

122. Pt developed jaundice, Mother O+ baby A+, Coombs test +ve his condition is due to?

- A. ABO incompatibility
- B. Rh incompatibility
- C. Isoimmunization

NOT SURE! 123. Clear case about Biliary atresia, there was pale stool, the direct bilirubin is high. What is the golden standard for biliary atresia

- A. ALT (elevated liver enzymes)
- B. US Ultrasound
- C. Liver biopsy

124. Which of the following is the definition of prematurity.

- A- Less than 38 weeks
- B- less than 37 weeks
- C- less 40 weeks
- D- less than 34 weeks

? 125. 17 years old presented to you with amenorrhea Tanner stage 5 her sisters have menses at 12 what investigation to do?

- A- karyotyping
- B- prolactin levels
- C- estradiol

126. 13 years old girl amenorrhea short (said how tall she is) her mother is (taller) father is (taller) what is most likely diagnosis for this?

- A- Testicular feminization
- B- Turner syndrome

127. 18-month-old female child presents with a blood-streaked stool. The stool is positive on occult blood testing. Which of the following diagnoses is most likely?

- a) Anal fissure.
- b) Peptic ulcer disease,
- c) Mallory- Weiss tear
- d) Inflammatory bowel disease.

128. A 9 months old presented to the clinic with a fever of 38.7°C and ear pulling. On physical examination you found erythematous bulging right tympanic membrane with purulent effusion. You diagnosed him with otitis media. What is the most likely organism that caused this infection?

- A. Group B streptococcus
- B. Streptococcus pneumoniae
- C. Group A streptococcus
- D. Streptococcus viridans.

129. A case about a patient with recurrent chest infection and oral thrush, labs show neutropenia, leukopenia and further investigation revealed no T-cells, but B-cell and NK are intact What is your diagnosis?

- A- SCID
- B- X-linked Agammaglobulinemia
- C- hyper IgM
- D- Wiskott-Aldrich syndrome

130. 2-year-old boy is brought to the clinic by his mother, who is concerned about his growth. He is below the 3rd percentile for weight and height. He has a history of frequent respiratory infections and has been increasingly irritable. On physical examination, the pediatrician notes visible muscle wasting and dry skin, but there is no edema.

What is the most likely diagnosis?

- A Kwashiorkor
- B Marasmus
- C Organic Failure to Thrive (OFTT)
- D Non-organic Failure to Thrive (NOFTT)

131. Female with knee pain. It has association with ocular defect. What will be also associated?

- A. Erythrocyte sedimentation rate
- B. Anti-nuclear antibody

132. 1 year old patient normal palpable thyroid gland TSH elevated low T4 what do you suspect?

- A-Thyroid dysgenesis
- B-Thyroid receptor block antibody
- C-Thyroid dyshormonogenesis
- D-Central hypothyroidism

133. The expected post-natal normal cardiac and respiratory transition is very necessary for the newborn to achieve the normal adult flow circulation. Which of the following physiological changes is expected to happen in post-natal transition?

- A. Pulmonary vascular resistance decreases
- B. Decrease pulmonic flow
- C. Ductus arteriosus remains patent
- D. Increased prostaglandins concentration

134. 11 year old boy is brought to the emergency department with a 1-day history of right sided abdominal pain, fever and vomiting. On examination, he has a temperature of 37.9C and heart rate of 112. Abdominal examination revealed rebound tenderness in the right iliac fossa. What is the most likely cause?

- a) Diverticulitis
- b) Appendicitis
- c) Pancreatitis
- d) Carcinoma

135. a question about how to measure the anion gap

- A- $NA - (CL + bicarbonate)$

136. What is the first sign of female puberty?

- a- Formation of breast buds

137. Child with fever for 5 days and has kawasaki. What would find in examination?

- A. bilateral exudative conjunctivitis
- B. Cracked lip

138. long case about asthmatics patient came with exacerbating attack, blood gas was ph 7.49, co was low bicarb was 21, what gas changes he is showing?

- A respiratory alkalosis
- B respiratory acidosis
- C metabolic acidosis
- D metabolic alkalosis

139. a long case about 1 year old male, looks ill his fever 36.5, he refused to feed since 2days, he didn't urinate much these priod, the mother when she changed his diaper she found pink color on the diaper, and the tender in palpating pubic area , what is the next to do forthis baby?

- A) Transurethral Catheterization (confirmed by dr)
- B) Renal Ultrasound

C) VCUG

140. 9 years old boy recently diagnosed with nephrotic syndrome, which of the following is indication for renal biopsy in child with nephrotic syndrome ?

(تكرر هذا النوع من الاسئلة بأكثر من صيغه بالميد والفانيل وبأغلب التجميعات)

A-Age between 1 and 8

B-Protein/creatinine ration of >2.5 mg/ dl

C-Serum albumin concentration less than 2.5 g/dl

D-Hypertension

141. A case about a young boy who's always had a large head since he was born (Above the 95th percentile).

Upon further history taking, his father informs you that most family members have the same condition (having a large head). What's the next step in management?

A. Genetic testing

B. Reassurance

142. A case about a patient who presented with anaphylactic shock after a peanut allergy) What's your management?

A. Epinephrine intramuscularly

143. 9-year-old boy collapses during soccer practice. He had no warning signs before the episode. His father died suddenly at age 40. What is the most appropriate next step in evaluation?

A Perform an ECG and echocardiogram to evaluate for a cardiac etiology

B Advise increased fluid and salt intake

C Obtain a brain MRI to rule out neurological conditions

D Conduct a tilt-table test for further assessment

144. A 26-month-old child, his mom, came to the clinic because she was concerned about his speech. The child speaks only one word in sentences and has 20 words in his vocabulary. In history, he did not respond when she talked to him. Also, he has a particular food he likes to eat, and when his mom holds his face, he gets angry.

What is the diagnosis?

A. ADHD

B. Deafness

C. speech delay problem

D. autism

145. 3 years old boy with history of recurrent wheezing episodes. He was born preterm 36 weeks, had respiratory distress syndrome received oxygen via nasal cannula for 24 hour after birth. He was admitted last winter season with bronchiolitis. Since then, he had recurrent attacks of cough, shortness of breath. Symptoms usually start with upper respiratory tract infection, exposure to dust and cold weather. Parents mentioned that his older brother had similar symptoms. On examination, his temperature 36.9C, pulse 100 bpm, respiratory rate 22 b/m O2 saturation was 98% on room air. He had allergic shiner, nasal congestion. Lungs are clear on auscultation. Which of the following best predict future asthma development?

A Presence of allergic shiner

B Brother with asthma symptoms

C History of prematurity

146. 14 yrs girl, comes for short stature in comparison to her classmates. Her father states that he had short stature throughout his teenage years and is now 177cm. She is 3th percentile for length and 35th for weight. Her breasts are Tanner stage 2, absent pubic and axillary hair. Xray of her left wrist and hand shows 11 years bone age. What would be the management?

A. Reassurance and follow up

B. Pelvic ultrasound

C. Growth hormone

D. Karyotyping

147. An 8 year old has sneezing, itching, nasal congestion, and dark under eyes, —, in winter. What is true?

- A. Eosinophils release in early stage
- B. Skin test to identify causative allergen
- C.
- D.

148. A girl in her breakfast had peanuts, 10 minutes later had trouble breathing etc. paramedics found angioedema, urticaria, and hypotension What is the pathophysiology of anaphylaxis?

- A. IgE mediated with mast and etc
- B. Autoimmune the body attacks itself
- C. Delayed hypersensitivity

149. months old has (36.8 or 37.8) fever. He looks alert and playfull what is correct?

- A. Minor illness and fever are not contraindication to immunization
- B. Fever alone is contraindicated
- C. If a child has a fever it is contradicted to have DTaP
- D. " rest of the options were unlikely to be the answer"

150. A child presents with fever , lethargy, neck stiffness and irritability CSF analysis were done and showed turbid fluid with high WBC mainly polymorph high protein and low glucose which of the following is most likely diagnosis ?

- A. bacterial meningitis
- B. viral meningitis
- C. meningitis due to TB
- D. viral encephalitis

151. A case about a girl with recurrent chest infection and oral thrush, labs show neutropenia, leukopenia and further investigation revealed no T-cells, but B-cell and NK are present. What is your diagnosis?

- A. SCID
- B. X-linked Agammaglobulinemia
- C. hyper IgM
- D. Wiskott-Aldrich syndrome

152. An 8-year-old is brought to the clinic by his parents. The patient is overweight and looks much older than other children in the same age group. Weight and height are greater than the 95th percentile for the child's age, and body mass index is 30 kg/m². The rest of the examination is normal. Which of the following is the most appropriate management?

- A- Lifestyle modification
- B- Referral for surgical intervention
- C- Prescription obesity medication
- D- Observation and revaluation in 12 months

153. A 5-year-old child managed in the Pediatric Intensive Care Unit for diabetic ketoacidosis with insulin infusion and fluid correction. The lab results show: Sodium 138 (134-146 mmol/L), Potassium 2.8 (3.5-5.1 mmol/L), Chloride 101 (97-108 mmol/L), Bicarbonate 22 (21-28 mmol/L). Which of the following is the most likely explanation of low potassium?

- A- Vomiting
- B- Bicarbonate therapy
- C- Diuretics therapy
- D- Insulin therapy

154. A 5-month-old infant presents with severe diarrhea for 3 days. On examination, the infant appears lethargic and dehydrated. Laboratory results reveal: pH: 7.22 (normal 7.35-7.45), HCO_3^- : 15 mEq/L (normal 22-26), PaCO_2 : 32 mmHg (normal 35-45). What is the primary acid-base disturbance?

- A- Metabolic acidosis with respiratory compensation
- B- Respiratory acidosis
- C- Mixed metabolic and respiratory acidosis
- D- Metabolic alkalosis with respiratory compensation

155. 4 year old girl is brought to clinic by her parents for a routine check up. They express concerns about her speech development. Upon examination, you notes that she has flat facial profile, slanted eye, and a single transverse palmar crease. What is the blood test can confirm the diagnosis?

- A Fluorescence in situ hybridization (FISH)
- B Karyotyping
- C Complete blood count
- D Polymerase Chain Reaction (PCR)

156. Case of 3 month infant female with hip pain (suspected DDH). She delivered in breech position. What is the best modality for diagnosis.

- A. US
- B. X-ray
- C. MRI
- D. CT scan

157. An exclusively breast-fed, 5-day-old boy is brought to the physician by his mother for a routine examination. He was born at term and delivery was uncomplicated. He received all standard treatment and testing prior to being discharged from the hospital 3 days ago. Examination shows no abnormalities. Without receiving additional supplementation at this time, this newborn is at greatest risk of developing which of the following conditions?

- A Rickets
- B Microcytic anemia
- C Scaly dermatitis
- D Intracranial bleed

158. An 18-hour old neonate observed in the nursery to have a yellowish discoloration of the skin. He is a product of full term and unremarkable pregnancy. The baby is formula-fed since birth. The blood group of the mother is (O+ve) and the blood group of the baby is A+ve. On physical examination the neonate weighs 3200 g with scleral and skin icterus. The initial blood work showed high reticulocyte count (10%), total serum bilirubin was 13mg/dL, and the direct serum bilirubin was 1 mg/dL. The direct Coombs test is +ve The most likely underling pathophysiology for hyperbilirubinemia in this baby is:

- A Physiological jaundice
- B Breast milk jaundice secondary to conjugation inhibitors in breast milk
- C Immune mediated hemolytic reaction secondary to blood group incompatibility
- D Biliary atresi

2 years old child, what he can do?

- A- Run and kick a ball
- B- Ride a bicycle
- C- Say 10 words in one sentence
- D- Drown a circle

Which of the following primitive reflexes at birth and disappear at 6 months:

- A- Moro reflex
- B- Palmar grasp reflex
- C- Tonic Neck reflex
- D- Rooting reflex

A 5 year old child with nephrotic syndrome was found to have renal vein thrombosis, which of the following blood test contribute to this complication?

- A- Hypocalcemia
- B- Thrombocytopenia
- C- Antithrombin 3
- D- DIC

A case about a fully vaccinated girl who has muscle weakness, loss of deep tendon reflex, and hypotonia. Her classmates have hand-foot-mouth disease. What virus could cause these symptoms or what is the most likely cause of these symptoms?

~~A- The following are part of high-quality CPR, EXCEPT:~~

- ~~A- Allow the chest to fully recoil~~
- ~~B- Compressions are at least 100 per/min~~
- ~~C- Chest should be depressed at least 2/3 of its AP~~
- ~~D- Minimize interruption~~

Non-Polio Virus

- B- Polio Virus
- C- Measles
- D- Herpes Simplex Virus

Which of the following is considered an indicator for kidney biopsy?

- A- Hypertension
- B- Hypercomplementemia
- C- Serum albumin concentration higher than 2.5g/dl
- D- Age between 1 and 8 years

18 months boy, what he can do?

- A- Copies line.
- B- Handle spoon well.
- C- Help to undress.
- D- Can stand on one leg for 10 sec.

A 6-week-old infant presented with a persistent non bilious vomiting. On examination, he was dehydrated with visible peristalsis on abdominal examination. What is the most likely electrolyte disturbance that would be found on laboratory investigation?

- A- Hypokalemia, hypernatremia, metabolic acidosis
- B- Hypokalemia, hyponatremia, metabolic alkalosis
- C- Hypokalemia, hypochloremia, metabolic acidosis
- D- Hypokalemia, hypernatremia, metabolic alkalosis

The following are part of high-quality CPR, EXCEPT:

- A- Allow the chest to fully recoil
- B- Compressions are at least 100 per/min
- C- Chest should be depressed at least 2/3 of its AP
- D- Minimize interruption

23-month female is hospitalized for severe malnutrition. She has inappropriately received rapid nasogastric feeding. Which of the following laboratory values is most concerning as iatrogenic complication?

- A- Hyponatremia
- B- Hypophosphatemia
- C- Hyperglycemia
- D- Hyperkalemia

You are awakened in the night by your 2-year-old son, who has developed noisy breathing on inspiration, marked retractions of the chest wall, flaring of the nostrils, and a barking cough. He had a mild upper respiratory infection for 2 days. Which of the following will be done in the emergency?

- A- Short-acting bronchodilators and a 5-day course of steroids
- B- Intubation and antibiotics
- C- Observation for hypoxia and dehydration alone
- D- Inhaled epinephrine and a dose of steroids

4 weeks infant on formula since birth with yellowish sclera and skin, poor feeding, vomiting, dark urine, pale stool. Bilirubin test showed Total Bilirubin 157, high direct bilirubin, imaging test was done. Which of the following is most important to investigate?

- A- Biliary atresia
- B- Hemolytic blood disorder
- C- Physiological breastfeeding hyperbilirubinemia
- D- Gilbert syndrome

A 5-year-old girl is brought to the physician because her mother has found her to be inattentive at home and has received multiple complaints from her teachers at school. She refuses to talk to her parents or peers. She prefers playing alone, and her mother reports that she likes playing throughout the visit. Physical and neurological examination show no abnormalities. Which of the following is the most likely diagnosis?

- A- Cerebral palsy
- B- Autism spectrum disorder
- C- Attention deficit hyperactivity disorder
- D- Normal child behaviour

12-year-old girl consumed food items that contained peanut in her breakfast. About 10 minutes after eating, she begins to feel a swelling in her face and lips. She experiences difficulty breathing and dizziness. Emergency medical services are called and she is assessed with the following finding: angioedema of her lip, urticaria, stridor and hypotension. What is the underlying pathophysiology responsible for this anaphylactic reaction?

- A- A delay-type hypersensitivity reaction that occurs due to the activation of T-cells and macrophages in response to the peanut allergen.
- B- An IgE-mediated hypersensitivity reaction leading to the degranulation of mast cells and basophils, resulting in release of histamine and other mediators.
- C- Activation of complement system leads to release of histamine and other inflammatory mediators from mast cells.

D- An autoimmune response where the body's immune system mistakenly attacks its own cells, causing systemic inflammation.

An 18-month-old boy presents with a cough and runny nose. His temperature is (37.9°C) Physical examination is only remarkable for a clear nasal discharge, and he is alert and playful. Which of the following is true?

A- The infant most likely has an undefined immunodeficiency.

B- A fever of 38°C after the first set of immunizations is an absolute contraindication to diphtheria, tetanus, and pertussis vaccine.

C- Minor illness with fever does not contraindicate immunization.

D- Fever alone is a contraindication to immunization.

An 18-hour old neonate observed in the nursery to have a yellowish discoloration of the skin. He is a product of full term and unremarkable pregnancy. The baby is formula-fed since birth. The blood group of the mother is (O+) and the blood group of the baby is (A+). On physical examination the neonate weighs 3200 g with scleral and skin icterus. Which of the following is the most important investigation that should be ordered?

A- Liver Function Test

B- Complete Blood Count "CBC"

C- Coombs test

D- Kidney Function Test

29 years old pregnant woman at 42 weeks of gestation, presented to the emergency department with labor pain. The baby delivered with thick meconium and required positive pressure ventilation and intubation. He had profound metabolic acidemia (pH was 6.8, Bicarbonate 8mmol/L) in the umbilical artery blood sample; his Apgar score was 3 at 1 minute of life and 4 at 5 minutes of life. The neurological examination showed hypotonic baby with abnormal movement suggestive of seizure. Which of the following treatment modalities is expected to improve the neurological long-term outcome of the above- mentioned case?

A- Thyroxin therapy

B- Therapeutic hypothermia

C- Inhaled nitric oxide

D- Fresh Frozen Plasma

In the delivery room, at 5 minutes after birth, a 28 weeks' gestation newborn who is wrapped in plastic bag is receiving continuous positive pressure ventilation of 5 cm H₂O with a fraction of inspired oxygen of 70%. He is pink and well perfused, with a heart rate of 135 beats/min and flexion of all extremities. Of the following, the main reason for this preterm infant respiratory distress is:

A- Transient tachypnea of the newborn

B- Surfactant deficiency

C- Pneumonia

D- Diaphragmatic hernia

A 6 year old girl known case of asthma, presented to the emergency room with dry cough and shortness of breath. On auscultation there was bilateral wheezing with good airtentry. Her chest X-Ray showed her inflated lungs and the blood gas was as follows: PH: 7.49, CO₂: 31, HCO₃: 20. What is your interpretation to the blood gas result?

A- Metabolic alkalosis

B- Respiratory alkalosis

C- Respiratory acidosis

D- Metabolic acidosis

10-year-old boy comes to the pediatric clinic with his father because of a 2-week history of recurrent fever spikes. He reports that his fevers occur one or two times per day with intermittent normalization of temperature. He also has fatigue, generalized joint pain, and profuse night sweats that make it necessary for him to change the sheets in the morning. He has not had a cough or shortness of breath. He is usually visiting his grandfather farm where he rides camels and drink fresh camel milk. He has no history of serious illness and takes no medications. His immunizations are up-to-date. He appears ill. His temperature is 39.4°C, pulse is 75/min, respirations are 14/min, and blood pressure is 115/65 mm Hg. Physical examination shows tender lymphadenopathy. Lungs are clear to auscultation. The remainder of the examination shows no other abnormalities. Laboratory studies show hemoglobin of 12.1 mg/dL (Normal), AST activity of 82 U/L(increased), and ALT activity of 85 U/L(increased). Which of the following is the most likely diagnosis?

- A- Malaria
- B- Tuberculosis
- C- Typhoid fever
- D- Brucellosis

20-year-old girl came to your clinic for a follow-up after her first febrile urinary tract infection (UTI); she finished the antibiotics course yesterday. All her symptoms resolved, and her examination was unremarkable. What is the next best step in her management?

- A- Reassurance and follow her in the clinic after 2-6 weeks
- B- Do a Renal and bladder ultrasound (RBUS) after 2-6 weeks
- C- Send another urine culture to document the sterility
- D- Do a Renal and bladder ultrasound (RBUS) today

You are a participant in a community campaign about the awareness of prematurity and its complications. Which of the following is the gestational age limit for the definition of prematurity?

- A- Less than 37 completed Gestational weeks
- B- Less than 38 completed Gestational weeks
- C- Less than 40 completed Gestational weeks
- D- Less than 34 completed Gestational weeks

Two weeks-old preterm baby is noted to have bloody stool. Her birth weight was 800 grams. She was given formula milk from day two of life and now feeding 14 ml every 2hours. She passed stools at birth. She looks pale, and sick looking. Her abdomen is distended and tender. Plain abdominal radiography showed air in the bowel wall. Her vital sign: temperature 36.8 heart rate: 160/min, Blood pressure 40/20mmg, respiratory rate 50/min. What is the MOST likely diagnosis?

- A- Meconium Ileus
- B- Hirschsprung Disease
- C- Necrotizing Enterocolitis
- D- Cystic fibrosis

A neonate born at 40 weeks' gestation, delivered by emergency cesarean section for umbilical cord prolapse, is being resuscitated. He was placed under the warmer and has a secure airway in place. Despite positive pressure ventilation and appropriate ventilation corrective maneuvers, his heart rate is still at 52 beats/min. Chest compressions are performed, and after 1 minute his heart rate is 48 beats/min. Of the following, the BEST next step in this neonate's management is to:

- A- Insert chest tube considering the possibility of pneumothorax
- B- Give surfactant through ETT
- C- Administer epinephrine through the endotracheal tube till the vascular access established
- D- Suction the mouth and nose

A 13-year-old woman comes to the physician because of a fever, nausea, and a nonproductive cough for 7 days. During this period, she has had headaches, generalized fatigue, and muscle and joint pain. She has also had increasing shortness of breath for 2 days. Her temperature is 38.1°C pulse is 102/min, respirations are 18/min. Lung auscultation revealed scattered wheezes. CXR showed bilateral interstitial opacities.

Which of the following is the most likely causal organism?

- A- Staphylococcus aureus
- B- Mycoplasma pneumoniae
- C- Haemophilus influenza
- D- Streptococcus pneumoniae

5-year-old child presents to the clinic with a high fever, cough, runny nose, and red eyes. The mother reports that the child has had a rash that started on the face and spread downwards to the trunk and extremities over the past three days. Upon examination, the rash is red, blotchy, and slightly raised, with some areas merging. Which of the following characteristics best describes the skin rash associated with measles in this scenario?

- A- The rash is characterized by small, flat, red spots that do not blanch when pressed.
- B- The rash consists of vesicular lesions that are itchy and crusty over time.
- C- The rash is maculopapular, begins on the face, and spreads to the trunk and limbs, often merging into larger areas.
- D- The rash is a well-defined, scaly plaque in a butterfly distribution on the face.

4-year-old girl presented with history of fever for 5 days. You suspect Kawasaki disease. Which of the following is a diagnostic criterion?

- A- Vesicular rash
- B- Inguinal lymphadenopathy
- C- Cracked lip
- D- Bilateral exudative conjunctivitis

5-year-old child is brought to the emergency department after 10 hours of accidentally ingesting 500 mg of paracetamol. The parents report that the child is asymptomatic, but they are concerned about potential overdose. The child weighs 20 kg. What is the next step in management?

- A- Discharge the child with follow-up instructions
- B- Perform gastric lavage
- C- Administer N-Acetylcysteine (NAC)
- D- Administer activated charcoal

Which of the following vaccines can be given to children with severe combined immunodeficiency syndrome?

- A- BCG Vaccine
- B- MMR Vaccine
- C- Rotavirus Vaccine
- D- Pneumococcal Vaccine

A 3-week boy who brought to emergency by his mother complained of high-grade fever, irritability, lethargy, poor feeding, and excessive crying (Sepsis Features). The lab test revealed leukocytosis. So, which of the following is the best empirical antibiotic therapy should be administered?

- A- Ampicillin + Ceftriaxone
- B- Vancomycin
- C- Ampicillin + Cefuroxime
- D- Meropenem

/ A 5-year-old boy has been experiencing episodes of waking up in the middle of the night, screaming and appearing very frightened. His parents describe that during these

episodes; he seems unaware of his surroundings and is difficult to console. These episodes last for several minutes, and he often does not remember them in the morning. In contrast, he also occasionally has vivid dreams that wake him up, and he can describe these dreams in detail. Based on the description, what is the most likely diagnosis for the episodes where the boy wakes up screaming?

- A- Sleep apnea
- B- Sleepwalking
- C- Night terrors
- D- Nightmares

A 10-year-old boy has a 2-year history of cough and dyspnea that is worse at night and with exercise. Symptoms are exacerbated by exposure to dust. You suspect that the boy has asthma. Which one of the following tests is most likely to assist you in making a diagnosis?

- A- Chest US
- B- Bronchoscopy
- C- Chest X-ray
- D- Spirometry

The mother of an 18-month-old calls the clinic to inform them that her child is scheduled to receive immunizations today, including a varicella vaccination. She is worried because her child was exposed to active varicella in his kindergarten class this morning. What instructions should be provided to the mother?

- A- Skip this set of immunizations and resume the schedule with the 24- month immunizations
- B- Bring the child to the clinic anyway to check the evidence of immunity with serologic testing
- C- There is no need to vaccinate your child in the future as he already got the natural immunization after this exposure
- D- Bring the child to the clinic anyway because immunizations should be kept on schedule

A 22 kg healthy infant is scheduled to receive intravenous fluids in preparation for surgery. The pediatrician determines the appropriate rate for fluid maintenance. What would this fluid rate be?

- A- 62 ml/hour
- B- 44 ml/hour
- C- 100 ml/hour
- D- 22 ml/hour

A 5-year-old unvaccinated girl developed a severe sore throat, drooling of saliva, a high fever, and increasing difficulty breathing. On examination, she was looking ill, sitting on her mother's lap, and refuses to move. She was tachypneic in respiratory distress. What is the most likely diagnosis?

- A- Bronchiolitis
- B- Croup
- C- Bronchitis
- D- Epiglottitis

A 10-week-old baby boy, full term, is brought by his mother for a general checkup. He has been delivered normally with a weight of 3.6 kg. Now he looks well, his systemic exam is unremarkable, with negative family history and newborn screen within normal limits. His Hb is 10 mg/dL with normal MCV and normal Ferritin level. He is active and feeding well. Of the following, what is the management in this infant?

- A- Iron supplement
- B- Blood transfusion

C- Reassurance

D- Folic acid

A neonate was brought to the emergency room with persistent vomiting for 2 days. He was hypoactive and having poor sucking and oral intake. The pregnancy history was unremarkable, and the child was not febrile with no gross dysmorphic features. Blood gas was obtained, which showed: pH 7.28 CO₂ 29 HCO₃ 14 In order to calculate the anion gap, which set of the following do you need?

A- Sodium, glucose, and magnesium

B- Sodium, chloride, and bicarbonate

C- Sodium, potassium, calcium, and chloride

D- Sodium, glucose, calcium

A 3-year-old boy is brought to the emergency department by his mother, who reports that he has been experiencing difficulty breathing for the past hour. She mentioned that he had a mild cold a few days ago, but his symptoms worsened quickly. The child is sitting upright, drooling, and is visibly distressed. Upon examination, you notice stridor and retractions. What is important when assessing a child with suspected upper airway obstruction?

A- Immediate intravenous access should be established

B- The child should be examined in a supine position

C- Avoid agitating the child during the assessment

D- The child should be kept in a crowded area for comfort

A 5-year-old presents to the emergency department with 6 days of fever, conjunctivitis, a "strawberry tongue," swelling of the hands, a diffuse rash, and cervical lymphadenopathy. Which medication should be included in the initial treatment regimen for this disorder to lessen the risk of complications?

A- IV antibiotics

B- Intravenous immunoglobulin

C- Nonsteroidal anti-inflammatory drugs

D- Intravenous steroids

A 3-month-old infant with fever has a catheterized urine sample that shows 100,000 CFU/mL of *Klebsiella pneumoniae* and ≥ 5 WBC/HPF on urine microscopy. How should this finding be interpreted?

A- Likely asymptomatic bacteriuria

B- Normal finding in this age group

C- Contaminated sample; repeat the test

D- Urinary tract infection requiring treatment

A 6-month-old infant presents with fever, irritability, and excessive crying. On examination, the infant's tympanic membrane appears dull and bulging with an absence of the light reflex. What is the most likely causative organism?

A- *Streptococcus pneumoniae*

B- *Pseudomonas aeruginosa*

C- *Escherichia coli*

D- *Staphylococcus aureus*

A 2-month-old formula-fed baby is brought in for a routine checkup. The parent of the baby tells that a friend has advised her to give fresh cow's milk to the baby instead of formula milk because it has high nutritional value. If the baby consumed fresh cow milk before reaching 1 year, he is at risk of developing which disease?

A- Thalassemia

B- Physiological anemia

C- Iron deficiency anemia

D- Sickle cell anemia

3-year-old child came to the pediatric clinic for developmental assessment. He should be able to:

- A- Skip with both feet
- B- Write their name
- C- Hop on one foot
- D- Ride a tricycle

You diagnosed a 14-year-old boy with constitutional growth delay. The parents asked you about this diagnosis and if it affects the boy's final height. Which of the following are consistent with this diagnosis?

- A- Prognosis for a normal adult height is good
- B- The child was undernourished during his first year of life
- C- The child was small for size at birth
- D- Bone age is normal compared to chronological age

While you were reviewing multiple medical files for different pediatric patients, you decided to analyze the growth chart of each. Which one will be considered abnormal?

- A- Six-month-old girl with her height corresponding to the 20th percentile
- B- Seven-year-old boy with his weight dropping from the 90th % to the 25th % percentile over one year
- C- Three-month-old baby with a weight corresponding to the 10th percentile
- D- Ten-year-old with BMI (body mass index) less than the 50th percentile

A family physician calls to ask you about the initial step in the management of a 3-week-old boy who presented with frequent vomiting episodes which are milk in content and abdominal distention, also there was a palpable, small, fixed mass in the right upper quadrant. What will be your advice as the first initial management?

- A- Reassuring
- B- Start intravenous fluid
- C- Give antiemetic and observe
- D- Pediatric surgery consultation

A 4-week-old male newborn is brought to the physician for a well-child examination. He was born at 40 weeks' gestation and weighed 3 kg. He now weighs 4.3 kg. There is no family history of serious illness. He is at the 50th percentile for length and weight. Vital signs are within normal limits. Examination shows an ejection systolic murmur best heard at the upper left sternal edge, grade 3/6 and a fixed and widely split-second heart sound. The lungs are clear to auscultation. The remainder of the examination shows no abnormalities. Which of the following is the most likely explanation for this patient's clinical findings?

- A- Left-to-right shunt through the ventricular septum
- B- Left-to-right shunt through the atrial septum
- C- Right-to-left shunt through the ventricular septum
- D- Right-to-left shunt through the atrial septum

A 1-year-old boy is brought to the hospital with low-grade fever, frequent vomiting, and acute diarrhea. Physical exam shows pulse 150/min, respiratory rate 50/min with deep breaths, decreased skin turgor, lethargy, and delayed capillary refill of 3 seconds. His weight is 10 kg. Which of the following is the next step in his management?

- A- Half normal saline bolus of 200 mL over 30 minutes
- B- Normal saline bolus of 200 mL over 30 minutes
- C- Normal saline at a rate of 40 mL/hour
- D- Half normal saline with dextrose 5 at a rate of 40 mL/hour

The expected post-natal normal cardiac and respiratory transition is very necessary for the newborn to achieve the normal adult flow circulation. Which of the following physiological changes is expected to happen in post-natal transition?

- A- Pulmonary vascular resistance decreases
- B- Decrease pulmonic flow
- C- Ductus arteriosus remains patent
- D- Increased prostaglandins concentration

2 weeks old infant was born at 36 weeks he weighed 2.4 kg at birth. His parents came to you worried about his intake. His current weight is 2.4 kg. What would you advice the parents?

- A- Encourage breastfeeding
- B- Prescribe premature formula
- C- 80 kcal/kg/day
- D- Prescribe soy formula

A patient had taken IVIG for kawasaki (before 2 weeks?) and according to Saudi Arabia immunization table he is now in time for his 2 years vaccine?

- A- Unchanged immune response
- B- Decreased immune response
- C- Augmented immune response
- D- Delayed immune response

12-hour old infant was born via cesarean section, found to be with jitteriness and his mother has a history of maternal diabetes, his blood sugar was low. what is the likely cause of hypoglycemia in this patient?

- A- Maternal Diabetes
- B- Intrauterine Growth Restriction (IUGR)
- C- Prematurity
- D- Neural tube defects

Four-month-old male infant presents with tachypnea, tachycardia and hepatosplenomegaly. On auscultation he had Soft Pan-Systolic Murmur in the Left sternal border. A chest radiograph shows cardiomegaly and increased pulmonary marking. Which of the following is the most likely diagnosis in this patient?

- A- Large ventricular septal defect
- B- Tetralogy of Fallot
- C- Small ventricular septal defect
- D- Inborn error of metabolism

A case of patient having red urine following upper respiratory tract infection. Urinalysis showed 2+ protein, 20-30 RBCs per high power field and dysmorphic red blood cells. Which of the following is the most likely diagnosis in this patient?

- A- Rhabdomyolysis
- B- Membranous glomerulonephritis
- C- IgA nephropathy
- D- Nephrolithiasis

An unvaccinated 7-year-old boy, not known to have any medical illness before, presented with history of fever, headache, and stiff neck for three days. On examination had a positive Kernig sign. CSF analysis showed a turbid color, high WBC count (mainly polymorphs), low glucose level compared to the serum glucose, and high protein. What is the most likely diagnosis?

- A- Viral meningitis
- B- Acute Encephalitis
- C- Optic neuritis
- D- Bacterial Meningitis

/ A school-age child presented to your office with a fever and sore throat history. You diagnose streptococcal pharyngitis based on a positive rapid test. You start the appropriate antibiotic treatment to prevent which one of the serious complications?

- A- Otitis media
- B- Post-streptococcal Glomerulonephritis
- C- Rheumatic fever
- D- Lower Respiratory Infections

A patient with beta thalassemia major on chronic blood transfusion. Which of the following complications will be seen?

- A- Vaso occlusive crisis
- B- Acute chest syndrome
- C- Iron overload
- D- Stroke

A case was brought about a child and mentioned that he had experienced cognitive decline. The parents said that the child had been infected 10 years ago with what they believed to be a viral exanthem. Which of the following is the most likely pathogen?

- A- Varicella
- B- Rubella
- C- Measles
- D- Mumps

A 14-year-old Saudi female presented with jaundice. One week ago, it started with flu-like symptoms of headache, low-grade fever, nausea, loss of appetite, and malaise. She self-treated herself with fevadol, but without improvement. Upon awakening this morning, she noticed her eyes and skin were yellow. She recently visited Bahrain on a two-week trip and swam in different swimming pools and beaches. She also mentioned the history of blood donation to the blood bank of KFSHRC one month ago. Her labs: ALT: 1225 IU/L (5-40 IU/L), AST: 340 IU/L (5-40 IU/L), GGT: 52 U/L (normal: 6-50 U/L). total bilirubin: 6.6 mg/dL (< 1.3 mg/dL), HbAg: negative, anti-HBs Ab.: positive, anti-HCV Ab.: negative, anti-HEV IgM: negative. Which of the following is the most likely diagnosis?

- A- Hepatitis A
- B- Hepatitis B
- C- Hepatitis C
- D- Hepatitis D

What is the appropriate daily fluid requirement for A-18 Kg-Child?

- A- 1400 ml
- B- 1700 ml
- C- 1000 ml
- D- 1500 ml

A patient who has asthma and he is in intermittent stage. So, what drug should be prescribed?

- A- Short-Acting Beta Agonist
- B- Short-Acting Beta Agonist and Inhaled Corticosteroid
- C- Long-Acting Beta Agonist
- D- Inhaled Corticosteroid

A 10-year-old boy presented with swelling in the face developed 8 days after tonsillitis treatment. You suspected acute post-streptococcal glomerulonephritis. Which of the following findings is most commonly associated with post-streptococcal glomerulonephritis?

- A- High serum IgA

- B- High Anti-GBM antibody
- C- Blood clots in the urine sample
- D- Low serum C3 levels

A 5-year-old boy presented to the ER with generalized edema. You suspected nephrotic syndrome. Which of the following is mandatory for the diagnosis of Nephrotic Syndrome?

- A- Presence of edema
- B- Urinary protein excretion greater than 100 mg/m/day
- C- Hyperlipidemia
- D- Serum albumin concentration less than 3 g/dL

A young boy presented to the hospital with his family for routine check-up, the mother mentioned that the family follows a strict vegan diet, which of the following would be expected to be deficient?

- A- Iron deficiency
- B- Folate
- C- Niacin
- D- Vitamin B12

8-year-old girl is brought to the clinic because she has been experiencing episodes of nighttime bedwetting. Her parents report that she was dry at night until the age of 6, but since then, she has had frequent wetting episodes. There is no history of urinary tract infections, and she does not exhibit any signs of daytime incontinence. Her parents are concerned that this might be related to stress at school due to recent changes in her routine. What is the most likely contributing factor to this child's enuresis?

- A- Neurological disorder
- B- Psychological stress
- C- Urinary tract infection
- D- Diabetes mellitus

A 16-year-old boy came to the pediatric clinic for developmental assessment. What is the initial physical manifestation of puberty in males?

- A- Testicular enlargement
- B- Growth spurt
- C- Emergence of early pubic hair
- D- Thelarche

A 9-month-old infant presents with a fever of 38.9°C for three days but appears well on examination with no focus of infection. What is the next best step in management?

- A- Start empirical antibiotics and reassess after 48 hours
- B- Perform a lumbar puncture
- C- Order a chest X-ray to exclude pneumonia
- D- Collect urine for microscopy and culture

A case about a 3-year-old boy with a fever for 6 days. His mother noticed conjunctivitis and lymph node enlargement. He has a maculopapular rash on examination. What will you see in a CBC of the patient.?

- A- Low platelet count
- B- High platelet count
- C- Low WBC count
- D- High RBC count

Neonate born with myelomeningocele, which of the following is the best time to do the surgery?

- A- Within 1 week
- B- Within 24-48 hours

- C- At the time of discharge
- D- After one month

/ A 4-month-old boy is brought by his parents to the primary health clinic. The mother is concerned because she feels the child's head is larger than his peers. On examination, his head circumference is measured above 98th percentile, but his developmental milestones are appropriate age, and his neurological exam is otherwise normal. She also mentioned that her husband and his family members have larger-than-average heads. What would be the most appropriate next step in management of this infant?

- A- Reassure the mother and continue to monitor the infant's development
- B- Refer to neurosurgery for urgent intervention
- C- Schedule urgent brain CT scan to rule out hydrocephalus
- D- Refer to a geneticist for evaluation of genetic syndromes

2-month-old formula-fed baby is brought in for a routine checkup. The parent of the baby tells that a friend has advised her to give cow milk to the baby instead of formula milk because it has high nutritional value. If the baby consumed fresh cow milk before he reaches at risk to develop which disease?

- A- Thalassemia
- B- Physiological anemia
- C- Sickle cell anemia
- D- Iron deficiency anemia

15-year-old female presents with a 2-month history of fatigue, generalized joint and muscle pain, and a malar rash. Labs show low complement levels, and proteinuria on urinalysis. What is the most likely diagnosis?

- A- Ankylosing spondylitis
- B- Systemic lupus erythematosus
- C- Sjogren's syndrome
- D- Rheumatoid arthritis

Q4/ A 6-year-old child presents with fever, malaise, and a slapped-cheek rash on the face, followed by a lacy rash on the body. The child is otherwise well. Which of the following complications is most associated with this infection in children with sickle cell anemia?

- A- Splenic infarction
- B- Myocarditis
- C- Hemolytic uremic syndrome
- D- Aplastic crisis

An infant is born to an HIV-infected mother. The mother received effective antiretroviral therapy (ART) during pregnancy, and undetectable at delivery. The infant is started on post-exposure prophylaxis. Which of the following diagnostic approaches is used to determine the HIV infection status of this infant in the first few months of life?

- A- HIV ELISA antibody test at 1 month of age.
- B- HIV DNA PCR testing at 2 weeks, 1-2 months, and 4-6 months of age.
- C- HIV RNA PCR (viral load) at 6 weeks of age as the sole diagnostic test.
- D- CD4+ T-cell count at birth and monthly thereafter.

A 2-year-old boy presents with a history of vomiting and diarrhea for several days. Physical examination reveals lethargy, tachypnea, and poor skin turgor. He refuses to take oral fluids, so you decide to begin intravenous fluid administration. What is the initial management?

- A- Normal saline at 10 mL/kg
- B- Bolus of isotonic solution at 20 mL/kg

- C- Dextrose and 0.45% normal saline at the maintenance rate
- D- Wait for laboratory results for serum electrolytes

A 15-year-old boy collapses suddenly while sitting during a class exam. There was no warning, and witnesses report a few seconds of twitching. He regained consciousness but was confused for several minutes. This is his first episode. What investigation should be done first?

- A- Brain CT scan
- B- Electrocardiogram
- C- Tilt-table testing
- D- Electroencephalogram

You are evaluating a healthy breastfed newborn. His mother is concerned about the safety of breastfeeding. In which of the following conditions would you strongly advise against breastfeeding?

- A- The mother will return to work as a full-time job.
- B- The mother is receiving acetaminophen as an analgesic medication.
- C- The infant is diagnosed with Congenital immunodeficiency.
- D- The mother is diagnosed with active untreated pulmonary tuberculosis.

A child begins to copy a square, hop on one foot, and ask questions using “why” and “how”. What is the most likely age?

- A- 3 years
- B- 4 years
- C- 5 years
- D- 6 years

A 4-year-old boy is brought to the clinic with multiple small vesicles around his mouth that rupture easily, leaving behind honey-colored crusts. He is otherwise well and afebrile. What is the most appropriate initial management?

- A- IV clindamycin
- B- Oral fluconazole
- C- Oral acyclovir
- D- Topical mupirocin

A 2-day-old newborn is noted to have bluish discoloration of the lips and tongue. Oxygen saturation is 84% on room air and does not improve significantly with 100% oxygen. Which of the following best differentiates central cyanosis from peripheral cyanosis?

- A- Discoloration of mucous membranes and tongue
- B- Degree of hypothermia
- C- Heart rate response to oxygen
- D- Presence of bluish hands and feet

A 4-week-old male newborn is brought to the physician for a well-child examination. He was born at 40 weeks' gestation and weighed 3 kg; he now weighs 4.3 kg. There is no family history of serious illness. He is at the 50th percentile for length and weight, and vital signs are within normal limits. Examination shows an ejection systolic murmur best heard at the upper left sternal edge, grade 3/6, and a fixed and widely split second heart sound. The lungs are clear to auscultation. The remainder of the examination shows no abnormalities. Which of the following is the most likely explanation for this patient's clinical findings?

- A- Right-to-left shunt through the ventricular septum.
- B- Left-to-right shunt through the ventricular septum.
- C- Left-to-right shunt through the atrial septum.

D- Right-to-left shunt through the atrial septum.

3-year-old boy presents to the clinic after experiencing two seizures six months apart. Both episodes involved generalized convulsive activity and occurred during fevers. He has no developmental delays or neurological deficits, and his interictal EEG is normal. Given this history, what is his risk of developing epilepsy?

A- Potentially increased if there is a history of...

B- Low, since the seizures are associated with fever.

C- Moderate, because he has had multiple seizures.

D- High, due to the recurrent nature of the seizures.

Q18/ A 6-month-old infant is brought to the pediatric clinic with episodes of drawing up the legs, crying inconsolably, and passing "red currant jelly-like" stool. The infant appears irritable and has a distended abdomen. On physical examination, a palpable mass is felt in the right lower quadrant. The most likely diagnosis is:

A- Acute appendicitis

B- Intussusception

C- Gastroenteritis

D- Mesenteric lymphadenitis

A 7-year-old boy is brought to the physician because his teachers have noticed him staring blankly on multiple occasions over the past month. These episodes last for several seconds and occasionally his eyelids flutter. He has met all his developmental milestones and appears healthy with a normal neurologic examination.

Hyperventilation for 30 seconds precipitates an episode of unresponsiveness and eyelid fluttering that lasts for 7 seconds. He regains consciousness immediately afterward. An electroencephalogram shows 3-Hz spikes and waves. Which of the following is the most appropriate pharmacotherapy for this patient?

A- Ethosuximide

B- Carbamazepine

C- Levetiracetam

D- Vigabatrin

A 6-month-old presents with a 3-day history of high fever followed by the appearance of a maculopapular rash as the fever subsided. What is the most likely diagnosis?

A- Measles

B- Scarlet fever

C- Roseola infantum (HHV-6)

D- Rubella

A 4-year-old previously healthy boy is brought to the clinic with a rash and fever for the past two days. His mother reports that the rash started on his chest and back and then spread to his face, scalp, and arms. The rash is itchy, and he has a mild fever and reduced appetite. On examination, the child is mildly febrile (38.3°C). He has multiple lesions at different stages: erythematous macules, papules, clear vesicles ("dewdrop on a rose petal"), and crusted lesions. The rash involves the trunk, face, scalp, and extremities, but spares the palms and soles. There are no signs of secondary infection or complications. What is the most likely diagnosis?

A- Hand, Foot, and Mouth Disease

B- Measles

C- Chickenpox (Varicella)

D- Impetigo

A 6-year-old child, weighing 20 kg, presents to the emergency department showing signs of severe dehydration. What is the appropriate volume of isotonic fluid (0.9% normal saline) to administer as an initial intravenous fluid bolus?

- A- 240 mL
- B- 120 mL
- C- 400 mL
- D- 60 mL

A 5-year-old boy is brought to the physician because of a 1-day history of abdominal pain and frequent urination. His mother reports that he has wet his bed overnight and that his urine smelled odd this morning. He has been toilet trained for 4 months. His last bowel movement was 3 days ago. Urine dipstick is positive for leukocyte esterase, nitrite, and blood; urinalysis shows white blood cells and gram-negative rods. Which of the following is the strongest risk factor for this patient's condition?

- A- Posterior urethral valves
- B- Constipation
- C- Respiratory tract infection
- D- Being a male gender

A 16-year-old otherwise healthy girl presents for evaluation of primary amenorrhea. Physical examination reveals a height of 55.5 in (140 cm), weight of 110 lb (50 kg), no breast tissue, and Sexual Maturity Rating (Tanner) stage 3 pubic hair development. Her mother's height is 68 in (173 cm), and her father's height is 70 in (178 cm). Of the following, the most likely diagnosis is:

- A- Androgen insensitivity (testicular feminization) syndrome
- B- Familial short stature
- C- Constitutional delay of growth and development
- D- Turner syndrome

A 14-year-old boy presents with asymptomatic proteinuria found on routine urinalysis. Repeat testing confirms isolated proteinuria with normal BP and renal function. What is the best next step?

- A- Order serum ANA and anti-dsDNA
- B- Perform kidney biopsy
- C- Repeat early morning urine protein/creatinine ratio
- D- Start prednisolone

A 3-year-old boy is brought to the emergency department by his parents with fever, barking cough, and stridor. He had a history of upper respiratory tract infection symptoms two days ago. On examination, the patient was vitally stable, and chest examination revealed inspiratory stridor with no other findings. What is the most appropriate next step?

- A- Start a course of oral Augmentin
- B- Reassure the parents that the febrile illness is due to croup and send the patient home
- C- Give one dose of dexamethasone
- D- Shift the patient to the operating room to secure the airway

A 3-year-old boy presents with recurrent skin abscesses caused by *Staphylococcus aureus* and several episodes of *Aspergillus* pneumonia. His past medical history is significant for delayed separation of the umbilical cord (at 5 weeks of age) and recurrent gingivitis. Physical examination reveals poor wound healing. A complete blood count shows marked neutrophilia. Which underlying immunodeficiency is most consistent with this clinical picture?

- A- Leukocyte Adhesion Defect (LAD)
- B- Severe Combined Immunodeficiency (SCID)
- C- X-linked Agammaglobulinemia (XLA)
- D- Complement C3 Deficiency

A 3-year-old child presents with high-grade fever, lethargy, capillary refill of 4 seconds, and a weak pulse. BP is 85/50 mmHg. He appears mottled and is minimally responsive. What is the most appropriate initial management for this patient?

- A- Start IV fluids and broad-spectrum antibiotics
- B- Administer antipyretics and observe
- C- Give high-dose steroids immediately
- D- Perform lumbar puncture urgently

A 2-year-old child presents with a painless limp noted by the parents over the last few months. On examination, there is limited abduction of the left hip, Trendelenburg gait, and leg length discrepancy. Radiographs reveal a shallow acetabulum with superior and lateral displacement of the femoral head. What is the most likely diagnosis?

- A- Proximal femoral focal deficiency
- B- Transient synovitis
- C- Legg-Calvé-Perthes disease
- D- Developmental Dysplasia of the Hip

A 6-year-old girl with no significant medical history is evaluated for an 8-month history of intermittent left knee pain with morning stiffness. Her mother has noticed her limping 2 to 3 times per week in the morning or after long car rides. Examination of the left knee demonstrates mild warmth, a moderate joint effusion, and a flexion contracture. Of the following, the laboratory finding most associated with an increased risk of ocular complications in this child is a(n):

- A- Elevated C-reactive protein level
- B- Elevated rheumatoid factor
- C- Microcytic hypochromic anemia
- D- Elevated antinuclear antibody titer

A healthy couple, who are first cousins, present for genetic counseling. They have a 3-year-old son diagnosed with Cystic Fibrosis, an autosomal recessive condition. They are planning another pregnancy and want to understand the risk of having another child affected with Cystic Fibrosis. What is the recurrence risk for Cystic Fibrosis in each of their future pregnancies?

- A- 100%, as both parents must be carriers.
- B- 75% (3 in 4)
- C- 25% (1 in 4)
- D- 50% (1 in 2)

A 3-year-old girl presents to the emergency department in an almost unresponsive state. Her parents say that she has been increasingly ill over the past 5 days and has been very thirsty, with increased urination. This morning she began to vomit. Findings on physical examination include rapid, sighing respirations and flushed cheeks. She is estimated to be 10% dehydrated. Initial laboratory studies reveal a blood glucose concentration of 700.0 mg/dL, sodium of 130.0 mEq/L, potassium of 4.6 mEq/L, chloride of 96.0 mEq/L, bicarbonate of 8.0 mEq/L, and a venous pH of 7.0. Of the following, the most appropriate action to decrease this child's risk for cerebral edema during treatment is to:

- A- Rehydrate slowly using 0.9% saline
- B- Correct acidosis rapidly with sodium bicarbonate
- C- Replace continuing urinary fluid losses with 0.2% saline
- D- Remove electrolyte initially with 3% saline

A newborn is delivered by emergency cesarean section at 38 weeks' gestation because of umbilical cord prolapse and fetal distress. At 30 minutes after birth, her oxygen saturation is 97%, and an arterial blood gas shows a pH of 6.89. The newborn is

lethargic with generalized hypotonia and a weak suck. Of the following, this neonate's management will include:

- A- Zinc supplementation
- B- Surfactant
- C- Coenzyme Q10
- D- Therapeutic hypothermia

An 8-year-old is brought to the clinic by his parents. The patient is overweight and looks much older than other children in the same age group. Weight and height are greater than the 95th percentile for the child's age, and body mass index is 30 kg/m². The rest of the examination is normal. Which of the following is the most appropriate management?

- A- Lifestyle modification
- B- Referral for surgical intervention
- C- Prescription obesity medication
- D- Observation and reevaluation in 12 months

A 4-week-old infant is referred to the pediatric surgical team due to persistent jaundice since birth. The parents report pale stools, dark urine, and poor weight gain. On examination, the infant has icterus, hepatomegaly, and no splenomegaly. Initial investigations reveal: Total Bilirubin 7.2 mg/dl (direct 5.6), AST/ALT mildly elevated, GGT highly elevated, HIDA scan showed no excretion into bowel after 24 hours. What is the next best step in management?

- A- Immediate Kasai portoenterostomy
- B- Oral corticosteroids
- C- Start ursodeoxycholic acid and observe for 2 months
- D- Liver transplant evaluation

A 2-month-old infant from Saudi Arabia is brought to the pediatric clinic for a routine check-up. The child was born full term with an uncomplicated delivery and has no significant medical history. The parents are unsure which vaccines their child should receive during this visit and are concerned about possible side effects. According to the local standard immunization schedule, which vaccines should be administered at this appointment?

- A- DTap, IPV, Hib, Hepatitis B, PCV, and Rotavirus
- B- MMR, Varicella, and Hepatitis A
- C- Influenza and Hepatitis A
- D- Tdap, HPV, and Meningococcal conjugate

A 4-year-old girl comes to the ED with sudden onset of wheezing. The patient's mother says she heard the wheezing right after the family opened up their holiday gifts. The patient's mother states that the girl's older brother received a set of toy cars with detachable wheels. What is the most appropriate test to order?

- A- Chest CT scan
- B- Chest ultrasound
- C- Decubitus films
- D- Inspiratory and expiratory films

A 5-year-old girl presents with an 8-week history of intermittent pain and swelling in her right knee. Exam confirms effusion. Labs show positive ANA. Other than uveitis, which systemic complication may develop in some children with oligoarticular Juvenile Idiopathic Arthritis?

- A- Macrophage activation syndrome
- B- Growth delay
- C- Hemophagocytic lymphohistiocytosis
- D- Vasculitis

Q46/ An 18-month-old girl is being evaluated because her mother thinks her abdomen seems "full." Physical examination reveals an abdominal mass and urinalysis showed hematuria. Ultrasonography identifies a solid renal mass. The most likely diagnosis is:

- A- Neuroblastoma
- B- Leukemia
- C- Lymphoma
- D- Wilms tumor

A new mother is seeking advice from a healthcare provider on the best way to introduce complementary foods to her infant. Which of the following pieces of advice is correct?

- A- Avoid introducing honey until the child is four years old.
- B- Start with homemade fresh juice.
- C- Avoid introducing allergenic foods until after the child turns one year.
- D- Introduce one new food every 3 to 4 days.

A child arrives at the emergency department with a history of bacterial gastroenteritis and subsequently develops hemolytic uremic syndrome as a rare complication. Which of the following laboratory findings would suggest this diagnosis?

- A- Positive direct Coombs test
- B- Decreased lactate dehydrogenase level
- C- Decreased reticulocyte count
- D- Thrombocytopenia

A 10-year-old boy presents to your clinic with fever, multiple joint pain, and swelling. On examination, the patient had tachycardia, tachypnea, and a systolic murmur. You suspect acute rheumatic fever. What is the next step to confirm your diagnosis?

- A- Chest X-ray
- B- Right knee MRI
- C- Cardiac enzymes
- D- Antistreptolysin

A ten-year-old boy presents with a four-week history of low-grade fever, fatigue, weight loss, and diffuse musculoskeletal pain. He lives in a rural area and helps his family care for livestock, including goats. He admits to drinking unpasteurized milk. Upon examination, he is febrile and shows mild hepatosplenomegaly. Blood cultures are pending. What is the most appropriate next laboratory test to perform?

- A- Antinuclear antibody (ANA) level
- B- Standard tube agglutination test (SAT)
- C- Antistreptolysin O (ASO) titer
- D- Tuberculin skin test (TST)

An exclusively breast-fed, 5-day-old boy is brought to the physician for a routine examination. He was born at term and delivery was uncomplicated. He received all standard treatment and testing prior to being discharged from the hospital 3 days ago. Examination shows no abnormalities. Without receiving additional supplementation at this time, this newborn is at greatest risk of developing which of the following conditions?

- A- Intracranial bleed
- B- Microcytic anemia
- C- Sealy dermatitis
- D- Rickets

A 9-month-old infant is brought to your clinic by his parents, who are concerned about his poor weight gain. The parents report that the child has been exclusively breastfed since birth and has been feeding well, but they have noticed that he is not

gaining weight as expected. On examination, the infant appears alert and active, with no abnormalities on physical examination. Which of the following is the most likely cause of the infant's failure to thrive?

- A- Inadequate caloric intake
- B- Psychosocial deprivation
- C- Metabolic disorder
- D- Gastrointestinal malabsorption

A 10-year-old child is brought to the clinic with a three-day history of watery diarrhea. The child is thirsty but appears active and has mild dry mucous membranes. His capillary refill time is 2 seconds, and he is taking small sips of water without vomiting. What is the most appropriate next step in management?

- A- Initiate oral rehydration therapy
- B- Prescribe antidiarrheal medications to reduce stool loss
- C- Admit for inpatient monitoring and keep NPO (nothing by mouth)
- D- Start intravenous fluids with a normal saline bolus

A 20-year-old Saudi mother in Riyadh visits the well-baby clinic for a routine checkup. She is worried about breastfeeding her newborn because she is currently undergoing treatment for a human immunodeficiency virus infection, and her viral load is high. She is adherent to her antiretroviral therapy and has never experienced an opportunistic infection. What advice is appropriate regarding feeding her infant?

- A. To start him exclusively on her breast milk until the age of 6 months
- B. To use heat-treated breast milk
- C. To use commercially prepared formula instead of breastfeeding
- D. To treat the infant with antiretrovirals and then start breastfeeding

2. A 4-week-old male newborn is brought to the physician for a well-child examination. He was born at 40 weeks' gestation and weighed 3 kg. He now weighs 4.3 kg. There is no family history of serious illness. He is at the 50th percentile for length and weight. Vital signs are within normal limits. Examination shows an ejection systolic murmur best heard at the upper left sternal edge, grade 3/6 and a fixed and widely split-second heart sound. The lungs are clear to auscultation. The remainder of the examination shows no abnormalities. Which of the following is the most likely explanation for this patient's clinical findings?

- A. Left-to-right shunt through the ventricular septum
- B. Left-to-right shunt through the atrial septum
- C. Right-to-left shunt through the ventricular septum
- D. Right-to-left shunt through the atrial septum

3. A family physician calls to ask you about the initial step in the management of a 3-week-old boy who presented with frequent vomiting episodes which are milk in content and abdominal distention, also there was a palpable small fixed mass in the right upper quadrant. What will be your advice as the first initial management?

- A. Reassuring
- B. Start intravenous fluid
- C. Give antiemetic and observe
- D. Pediatric surgery consultation

. While you were reviewing multiple medical files for different pediatric patients, you decided to analyze the growth chart of each.

Which one will be considered abnormal?

- A. Six-month-old girl with her height corresponding to the 20th percentile
- B. Seven-year-old boy with his weight dropping from the 90th % to the 25th % percentile over one year
- C. Three-month-old baby with a weight corresponding to the 10th percentile
- D. Ten-year-old with BMI (body mass index) less than the 50th percentile

5. Which of the following factors indicates pre-renal acute kidney injury (AKI) in pediatric patients?

- A. Urine osmolality of less than 300 mOsm/L
- B. Fractional excretion of sodium (FeNa) <1%
- C. Presence of proteinuria on urinalysis
- D. Hyperkalemia on serum testing

6. A one-year-old boy presents with fever, cough, coryza, conjunctivitis, and a descending maculopapular rash. Which of the following is a hallmark diagnostic feature of the disease in the prodromal stage?

- A. Redness and swelling of the Stensen duct opening
- B. Koplik spots
- C. Strawberry tongue
- D. Bullous skin lesions

7. You diagnosed a 14-year-old boy with constitutional growth delay. The parents asked you about this diagnosis and if it affects the boy's final height. Which of the following are consistent with this diagnosis?

- A. Prognosis for a normal adult height is good
- B. The child was undernourished during his first year of life
- C. The child was small for size at birth
- D. Bone age is normal compared to chronological age

8. A 10-year-old boy presented with swelling in the face developed 8 days after tonsillitis treatment. You suspected acute post-streptococcal glomerulonephritis. Which of the following findings is most commonly associated with post-streptococcal glomerulonephritis?

- A. High serum IgA
- B. High Anti-GBM antibody
- C. Blood clots in the urine sample
- D. Low serum C3 levels

9. A 3-year-old child came to the pediatric clinic for developmental assessment. He should be able to?

- A. Skip with both feet
- B. Write their name
- C. Hop on one foot
- D. Ride a tricycle

10. A 2-month-old formula-fed baby is brought in for a routine checkup. The parent of the baby tells that a friend has advised her to give fresh cow's milk to the baby instead of formula milk because it has high nutritional value. If the baby consumed fresh cow milk before reaching 1 year, he is at risk of developing which disease?

- A. Thalassemia

- B. Physiological anemia
- C. Iron deficiency anemia
- D. Sickle cell anemia

11. A mother has just given birth to a baby girl in a hospital. The midwife wants to administer an intramuscular injection for routine neonatal care. What would that injection be?

- A. Vitamin K
- B. Normal saline
- C. Hepatitis A vaccine
- D. Tetanus immunoglobulin

12. An 8-year-old girl is brought to the clinic because she has been experiencing episodes of nighttime bedwetting. Her parents report that she was dry at night until the age of 6, but since then, she has had frequent wetting episodes. There is no history of urinary tract infections, and she does not exhibit any signs of daytime incontinence. Her parents are concerned that this might be related to stress at school due to recent changes in her routine. What is the most likely contributing factor to this child's enuresis?

- A. Neurological disorder
- B. Psychological stress
- C. Urinary tract infection
- D. Diabetes mellitus

13. A 5-year-old boy presented to the ER with generalized edema. You suspected nephrotic syndrome. Which of the following is mandatory for the diagnosis of Nephrotic Syndrome?

- A. Presence of edema
- B. Urinary protein excretion greater than 100 mg/m²/day
- C. Hyperlipidemia
- D. Serum albumin concentration less than 3 g/dL

14. A 10-year-old girl presents with a history of chronic rhinorrhea, nasal itchiness, and sneezing. On physical examination, she reveals dark circles under her eyes and pale boggy nasal mucosa. Which of the following is true regarding her condition?

- A. Skin testing is performed to identify the causative allergens.
- B. Decongestants intranasally are the main treatment.
- C. This condition is an example of type III hypersensitivity disorder.
- D. Eosinophils release its mediators in the early phase.

15. A 6-month-old infant presents with fever, irritability, and excessive crying. On examination, the infant's tympanic membrane appears dull and bulging with an absence of the light reflex. What is the most likely causative organism?

- A. Streptococcus pneumoniae
- B. Pseudomonas aeruginosa
- C. Escherichia coli
- D. Staphylococcus aureus

16. A 3-month-old infant with fever has a catheterized urine sample that shows 100,000 CFU/mL of Klebsiella pneumoniae and ≥ 5 WBC/HPF on urine microscopy. How should this finding be interpreted?

- A. Likely asymptomatic bacteriuria

- B. Normal finding in this age group
- C. Contaminated sample; repeat the test
- D. Urinary tract infection requiring treatment

17. A 10-year-old boy has been recently discharged from the hospital after an asthma exacerbation. What advice can be given to the patient to prevent future exacerbation?

- A. Limit physical activity
- B. Start oral antibiotics at the onset of acute upper respiratory tract infection
- C. Start short-acting beta-2 agonist as needed for cough, shortness of breath
- D. Provide a written asthma action plan

18. A 4-year-old child presents to your office with fever, and increased work of breathing manifested by tachypnea and retractions. A chest x-ray confirms lobar pneumonia. What is the most likely etiology of pneumonia in this child?

- A. Streptococcus pyogenes
- B. Streptococcus pneumoniae
- C. Mycoplasma pneumoniae
- D. Chlamydia pneumoniae

19. Four-month-old male infant presents with tachypnea, tachycardia, and hepatomegaly. On auscultation, he has a soft pansystolic murmur at the lower left sternal border. A chest radiograph shows cardiomegaly and increased pulmonary markings. There are normal oxygen saturations in all 4 extremities. Which of the following is the most likely diagnosis in this patient?

- A. Inborn error of metabolism
- B. Large ventricular septal defect
- C. Small ventricular septal defect
- D. Tetralogy of Fallot

20. A 2-year-old boy with viral gastroenteritis is brought to the hospital. He weighs 12 kg and has a heart rate of 130 beats per minute, with otherwise normal vital signs. The examination reveals a tired-looking child who is crying with slight tears. An oral examination shows moist mucosa and a normal throat. What is the most appropriate rehydration strategy for this patient?

- A. Dextrose 10 with normal saline of 120 mL over 30 minutes
- B. Free water of 500 mL over 2 hours
- C. Commercial drink with low carbohydrates and no sodium 750 mL over 4

hours

- D. Compounded oral solution with 1:1 glucose/sodium content 600 mL over 6 hours

21. A 5-year-old presents to the emergency department with 6 days of fever, conjunctivitis, a "strawberry tongue," swelling of the hands, a diffuse rash, and cervical lymphadenopathy. Which medication should be included in the initial treatment regimen for this disorder to lessen the risk of complications?

- A. IV antibiotics
- B. Intravenous immunoglobulin
- C. Nonsteroidal anti-inflammatory drugs
- D. Intravenous steroids

22. Which of the following is used to diagnose HIV infection in infants less than 18 months of age?

- A. Viral culture
- B. ELISA
- C. HIV DNA PCR
- D. Western Blot

23. A 16-year-old boy came to the pediatric clinic for developmental assessment. What is the initial physical manifestation of puberty in males?

- A. Testicular enlargement
- B. Growth spurt
- C. Emergence of early pubic hair
- D. Thelarche

24. A 3-year-old boy is brought to the emergency department by his mother, who reports that he has been experiencing difficulty breathing for the past hour. She mentioned that he had a mild cold a few days ago, but his symptoms worsened quickly. The child is sitting upright, drooling, and is visibly distressed. Upon examination, you notice stridor and retractions. What is important when assessing a child with suspected upper airway obstruction?

- A. Immediate intravenous access should be established
- B. The child should be examined in a supine position
- C. Avoid agitating the child during the assessment
- D. The child should be kept in a crowded area for comfort

25. You meet parents in a well-baby clinic concerned about their child receiving the MMR vaccine. Which of the following would be a valid contraindication to his immunization?

- A. Older brother with a history of autism
- B. Illness with low-grade fever
- C. Low-dose corticosteroid treatment for a short period
- D. Anaphylaxis to a previous similar dose

26. A neonate was brought to the emergency room with persistent vomiting for 2 days. He was hypoactive and having poor sucking and oral intake. The pregnancy history was unremarkable and the child was not febrile with no gross dysmorphic features. Blood gas was obtained, which showed:

pH 7.28

CO₂ 29

HCO₃ 14

In order to calculate the anion gap, which set of the following do you need?

- A. Sodium, glucose, and magnesium
- B. Sodium, chloride, and bicarbonate
- C. Sodium, potassium, calcium, and chloride

26. A one-month-old infant presents with fever, irritability, and poor feeding. CSF analysis shows: (CSF analysis details were about Viral Meningitis, How would you interpret this CSF analysis?

- A. It is suggestive of partially treated meningitis
- B. Viral meningitis
- C. Bacterial meningitis
- D. Normal CSF

27. A 10-week-old baby boy, full term, is brought by his mother for a general checkup. He has been delivered normally with a weight of 3.6 kg. Now he looks well, his systemic exam is unremarkable, with negative family history and newborn screen within normal limits. His Hb is 10 mg/dL with normal MCV and normal Ferritin level. He is active and feeding well. Of the following, what is the management in this infant?

- A. Iron supplement
- B. Blood transfusion
- C. Reassurance
- D. Folic acid

28. A 5-year-old unvaccinated girl developed a severe sore throat, drooling of saliva, a high fever, and increasing difficulty breathing. On examination, she was looking ill, sitting on her mother's lap, and refuses to move. She was tachypneic in respiratory distress. What is the most likely diagnosis?

- A. Bronchiolitis
- B. Croup
- C. Bronchitis
- D. Epiglottitis

29. A 22 kg healthy infant is scheduled to receive intravenous fluids in preparation for surgery. The pediatrician determines the appropriate rate for fluid maintenance. What would this fluid rate be?

- A. 62 ml/hour
- B. 44 ml/hour
- C. 100 ml/hour
- D. 22 ml/hour

30. In children with Chronic Kidney Disease, which of the following is the most effective strategy to slow disease progression?

- A. Sodium bicarbonate to treat metabolic acidosis
- B. Blood pressure control with angiotensin-converting enzyme (ACE) inhibitors
- C. High-protein diet to prevent malnutrition
- D. Frequent diuretic use to maintain fluid balance

31. A 10-week-old baby boy, full term, is brought by his mother for a general checkup. He has been delivered normally with a weight of 3.6 kg. Now he looks well, his systemic exam is unremarkable, with negative family history and newborn screen within normal limits. His Hb is 10 mg/dL with normal MCV and normal Ferritin level. He is active and feeding well. Of the following, what is the management in this infant?

- A. Iron supplement
- B. Blood transfusion
- C. Reassurance
- D. Folic acid

32. The mother of an 18-month-old calls the clinic to inform them that her child is scheduled to receive immunizations today, including a varicella vaccination. She is worried because her child was exposed to active varicella in his kindergarten class this morning. What instructions should be provided to the mother?

- A. Skip this set of immunizations and resume the schedule with the 24-month immunizations
- B. Bring the child to the clinic anyway to check the evidence of immunity with serologic testing
- C. There is no need to vaccinate your child in the future as he already got the natural immunization after this exposure
- D. Bring the child to the clinic anyway because immunizations should be kept on schedule

33. A 10-year-old boy presents to your clinic with fever, multiple joint pain, and swelling. On examination, the patient has tachycardia, tachypnea, and a systolic murmur. You suspect acute rheumatic fever. Which of the following is considered a major criterion for diagnosis?

- A. Conjunctivitis
- B. Carditis
- C. Fever for 5 days
- D. Erythema nodosum

34. A 10-year-old boy has a 2-year history of cough and dyspnea that is worse at night and with exercise. Symptoms are exacerbated by exposure to dust. You suspect that the boy has asthma. Which one of the following tests is most likely to assist you in making a diagnosis?

- A. Chest US
- B. Bronchoscopy
- C. Chest X-ray
- D. Spirometry

35. An unvaccinated 4-year-old girl presents to the emergency department with a fever and generalized pruritic vesicular rash in varying stages of development. History of contact with her immunocompromised sister, who had a painful vesicular rash on one side of her trunk. Which of the following is true regarding this viral illness?

- A. Immunocompromised patients usually manifest as a mild breakthrough illness
- B. The causative organism is an RNA virus
- C. The feco-oral route is the known mode of transmission
- D. It can be complicated by necrotizing fasciitis due to GAS infection

36. Which of the following vaccines should NOT be given to children with severe combined immunodeficiency syndrome?

- A. Hepatitis B virus vaccine
- B. Measles virus vaccine
- C. DTP vaccine
- D. Pneumococcal vaccine

37. A 2-year-old child with osteomyelitis in the femur is started on IV antibiotics. After 72 hours of therapy, the fever resolves, and inflammatory markers improve. What is the next step in management?

- A. Continue IV antibiotics for 6 weeks
- B. Discontinue antibiotics and monitor clinically
- C. Switch to oral antibiotics and continue therapy for 3-4 weeks
- D. Perform a bone biopsy to confirm diagnosis

38. A 5-year-old boy has been experiencing episodes of waking up in the middle of the night, screaming and appearing very frightened. His parents describe that during these episodes, he seems unaware of his surroundings and is difficult to console. These episodes last for several minutes, and he often does not remember them in the morning. In contrast, he also occasionally has vivid dreams that wake him up, and he can describe these dreams in detail. Based on the description, what is the most likely diagnosis for the episodes where the boy wakes up screaming?

- A. Sleep apnea
- B. Sleepwalking
- C. Night terrors
- D. Nightmares

39. A 9-month-old infant presents with a fever of 38.9°C for three days but appears well on examination with no focus of infection. What is the next best step in management?

- A. Start empirical antibiotics and reassess after 48 hours
- B. Perform a lumbar puncture
- C. Order a chest X-ray to exclude pneumonia
- D. Collect urine for microscopy and culture

40. A 5-year-old child is brought to the emergency department after accidentally ingesting 500 mg of paracetamol. The parents report that the child is asymptomatic but they are concerned about potential overdose. The child weighs 20 kg. If the serum paracetamol level is above the treatment line on the Rumack-Matthew nomogram, what is the next step in management?

- A. Discharge the child with follow-up instructions
- B. Perform gastric lavage
- C. Administer N-acetylcysteine (NAC)
- D. Administer activated charcoal

Q1- A 2-month-old formula-fed baby is brought in for a routine checkup. The parent of the baby tells that a friend has advised her to give fresh cow's milk to the baby instead of formula milk because it has high nutritional value. If the baby consumed fresh cow milk before he reaches 1 year he is at risk of developing which disease?

- A. Thalassemia
- B. Physiological anemia
- C. Sickle cell anemia
- D. Iron deficiency anemia

Q2- 4 year-old girl presented with history of fever for 5 days. You suspect Kawasaki disease. Which of the following is a diagnostic criterion?

- A. Vesicular rash
- B. Inguinal lymphadenopathy
- C. Cracked lip
- D. Bilateral exudative conjunctivitis

Q3- 3 years old boy with history of recurrent wheezing episodes. He was born preterm 36 weeks, had respiratory distress syndrome, received oxygen via nasal cannula for 24 hours after birth. He was admitted last winter season with bronchiolitis. Since then, he had recurrent attacks of cough, shortness of breath. Symptoms usually start with upper respiratory tract infection, exposure to dust and cold weather. Parents mentioned that his older brother had similar symptoms. On examination, his temperature 36.9°C, pulse 100 bpm, respiratory rate 22 b/m, O₂ saturation was 98% on room air. He had allergic shiner, nasal congestion. Lungs are clear on auscultation. Which of the following best predicts future asthma development?

- A Presence of allergic shiner
- B Brother with asthma symptoms
- C History of prematurity
- D Having wheezing triggered by cold weather and dust

Q5- The expected post-natal normal cardiac and respiratory transition is very necessary for the newborn to achieve the normal adult flow circulation, which of the following physiological changes is expected to happen in post-natal transition?

- A Increased prostaglandins concentration
- B Ductus arteriosus remains patent
- C Decreased pulmonic flow
- D Pulmonary vascular resistance decreases

Q6- A 9-months-old boy presented to the clinic with a 2-days history of fever of (38.7°C), and ear pulling. On physical examination, you note an erythematous, bulging right tympanic membrane with purulent effusion. You diagnose otitis media.

What organism is the most likely cause of this infection?

- A Streptococcus pneumoniae
- B Group A streptococcus
- C Streptococcus viridians
- D Group B streptococcus

Q7- A 2-month-old girl presents with failure to thrive and persistent diarrhea. She has been hospitalized twice for pneumonia. On exam, she appears pale and malnourished. She has oral thrush and lymphadenopathy. Laboratories reveal severe neutropenia, lymphopenia. Flow cytometry shows absence of T cells with normal B and NK cells. Her condition is most consistent with a diagnosis of:

- A Sever combined immunodeficiency
- B X-linked agammaglobulinemia
- C Hyper IgM syndrome
- D Wiskott-Aldrich syndrome

Q8- 5-year-old child presents to the clinic with a high fever, cough, runny nose, and red eyes. The mother reports that the child has had a rash that started on the face and spread downwards to the trunk and extremities over the past three days. Upon examination, the rash is red, blotchy, and slightly raised, with some areas merging.

Which of the following characteristics best describes the skin rash associated with measles in this scenario?

- A The rash is characterized by small, flat, red spots that do not blanch when pressed.
- B The rash consists of vesicular lesions that are itchy and crusty over time.
- C The rash is maculopapular, begins on the face, and spreads to the trunk and limbs, often merging into larger areas.
- D The rash is a well-defined, scaly plaque in a butterfly distribution on the face.

Q9- Mother of a 3 month old girl noted asymmetry of the skin folds on the hips, shortening of the left lower extremity, not large external rotation of the hip, limited abduction of lower extremity in the hip joints. The mother also stated that the child was breech presentation.

The suitable screening tool for this patient is ?

- A X-ray
- B Computerized tomography
- C Magnetic Resonance
- D Ultrasound

Q10- infant is brought to the office for a routine health maintenance visit. The infant can sit with support, vocalize with mixed vowel and consonant sounds, and has just learned to transfer objects from hand to hand. Which is the most likely age of this infant?

- D 8 months of age
- C 4 months of age
- B 6 months of age
- A 9 months of age

Q11- 2-year-old boy is brought to the clinic by his mother, who is concerned about his

growth. He is below the 3rd percentile for weight and height. He has a history of frequent respiratory infections and has been increasingly irritable. On physical examination, the pediatrician notes visible muscle wasting and dry skin, but there is no edema.

What is the most likely diagnosis?

A Kwashiorkor

B Marasmus

C Organic Failure to Thrive (OFTT)

D Non-organic Failure to Thrive (NOFTT)

Q12- A 13-year-old woman comes to the physician because of a fever, nausea, and a nonproductive cough for 7 days. During this period, she has had headaches, generalized fatigue, and muscle and joint pain. She has also had increasing shortness of breath for 2 days. Her temperature is 38.1°C, pulse is 102/min, respirations are 18/min. Lung auscultation revealed scattered wheezes. CXR showed bilateral interstitial opacities. Which of the following is the most likely causal organism?

A Staphylococcus aureus

B Mycoplasma pneumoniae

C Haemophilus influenza

D Streptococcus pneumoniae

Q13- A 6-year-old child presents with persistent proteinuria detected on urine dipstick testing during a routine check-up. The child has no other abnormal findings on physical examination, and there is no family history of kidney disease. Which of the following is the most appropriate initial investigation to further evaluate this child?

A Urine protein-to-creatinine ratio

B Serum complement levels

C Kidney biopsy

D Renal ultrasound

Q15- A neonate born at 40 weeks' gestation, delivered by emergency cesarean section for umbilical cord prolapse, is being resuscitated. He was placed under the warmer and has a secure airway in place. Despite positive pressure ventilation and appropriate ventilation corrective maneuvers, his heart rate remains at 52 beats/min. Chest compressions are performed, and after 1 minute his heart rate is 48 beats/min. Of the following, the BEST next step in this neonate's management is to:

A Insert chest tube considering the possibility of pneumothorax

B Give surfactant through ETT

C Administer epinephrine through the endotracheal tube till the vascular access established

D Suction the mouth and nose

Q16- 9-year-old boy collapses during soccer practice. He had no warning signs before the episode. His father died suddenly at age 40. What is the most appropriate next step in evaluation?

A Perform an ECG and echocardiogram to evaluate for a cardiac etiology

B Advise increased fluid and salt intake

C Obtain a brain MRI to rule out neurological conditions

D Conduct a tilt-table test for further assessment

Q18- 46-hour-old neonate has not yet passed meconium. He was born at 39 weeks' gestation via cesarean section to a 32-year-old, gravida 1 para 0 woman. She had an uncomplicated pregnancy with no significant findings on prenatal follow up. The neonate had been breastfeeding every 2 to 3 hours, but this morning his mother noted that he was fussy while trying to feed along with vomiting which is greenish in color. His physical examination findings are notable for distended abdomen and patent anal opening. The remainder of his examination findings are normal. Of the following the MOST likely cause of this infant's findings is:

A Hirschsprung's disease

B Pyloric stenosis

C Tracheoesophageal fistula

D Duodenal atresia

Q19- two weeks-old preterm baby is noted to have bloody stool. Her birth weight was 800 grams. She was given formula milk from day two of life and now feeding 14 ml every 2 hours. She passed stools at birth. She looks pale, and sick looking. Her abdomen is distended and tender. Plain abdominal radiography showed air in the bowel wall. Her vital sign : temperature 36.8 heart rate : 160/min, Blood pressure 40/20mmg, respiratory rate 50/min. What is the MOST likely diagnosis ?

A Meconium Ileus

B Hirschsprung Disease

C Necrotizing Enterocolitis

D Cystic fibrosis

Q21- NEONATE WAS BROUGHT TO THE EMERGENCY ROOM WITH PERSISTENT VOMITING FOR 2 DAYS, HE WAS HYPOACTIVE AND HAVING POOR SUCKING AND ORAL INTAKE. THE PREGNANCY HISTORY WAS UNREMARKABLE AND THE CHILD WAS NOT FEBRILE WITH NO GROSS DYSMORPHIC FEATURES. BLOOD GAS WAS OBT.

A SODIUM, CHLORIDE AND BICARBONATE

B SODIUM, GLUCOSE AND MAGNESIUM

C SODIUM, POTASSIUM, CALCIUM AND CHLORIDE

D SODIUM, POTASSIUM AND GLUCOSE

Q23- A 10 year old boy has a 2 years history of cough and dyspnea that is worse at night, and with exercise. Symptoms are exacerbated by exposure to dust. Which one of the following tests is most likely to assist you to make a diagnosis:

A chest X ray

B chest CT scan

C Spirometry

D Bronchoscopy

Q25- 2-year-old girl came to your clinic for a follow-up after her first febrile urinary tract infection (UTI); she finished the antibiotics course yesterday. All her symptoms resolved, and her examination was unremarkable.

What is the next best step in her management?

A Reassurance and follow her in the clinic after 2-6 weeks

B Do a Renal and bladder ultrasound (RBUS) after 2-6 weeks

C Send another urine culture to document the sterility

D Do a Renal and bladder ultrasound (RBUS) today

Q26- You are a participant in a community campaign about the awareness of prematurity and its complications. Which of the following is the gestational age limit for the definition of prematurity?

A Less than 37 completed Gestational weeks

B Less than 38 completed Gestational weeks C Less than 40 completed Gestational weeks

D Less than 34 completed Gestational weeks

Q28- full-term infant presents with acrocyanosis but no respiratory distress. His oxygen saturation is 98%, and his

mucous membranes remain pink. What is the most appropriate next step in management?

A Obtain an echocardiogram

B Start prostaglandin E1 infusion

C Perform a hyperoxia test

D Reassure parents and provide routine neonatal care

Q31- A 6 YEAR OLD GIRL KNOWN CASE OF ASTHMA, PRESENTED TO THE EMERGENCY ROOM WITH DRY COUGH AND SHORTNESS OF BREATH. ON AUSCULTATION THERE WAS BILATERAL WHEEZING WITH GOOD AIR ENTRY. HER CHEST X-RAY SHOWED HER INFLATED LUNGS AND THE BLOOD GAS WAS AS FOLLOWS: PH

7.49, CO2 31, HCO3 20 WHAT IS YOUR INTERPRETATION TO THE BLOOD GAS RESULT?
A METABOLIC ALKALOSIS
RESPIRATORY ALKALOSIS
C RESPIRATORY ACIDOSIS
D METABOLIC ACIDOSIS

Q32- A 7-year-old boy is brought to the physician by his mother because his teachers have noticed him staring blankly on multiple occasions over the past month. These episodes last for several seconds and occasionally his eyelids flutter. He was born at term and has no history of serious illness. He has met all his developmental milestones. He appears healthy. Neurologic examination shows no abnormalities. Hyperventilation for 30 seconds precipitates an episode of unresponsiveness and eyelid fluttering that lasts for 7 seconds. He regains consciousness immediately afterward. An electroencephalogram shows 3-Hz spikes and waves. Which of the following is the most appropriate pharmacotherapy for this patient?
A Levetiracetam
B Vigabatrin
C Ethosuximide
D Carbamazepine

Q33- A 5-year-old boy presents with chronic diarrhea, abdominal pain, and failure to thrive. His mother reports that he often complains of bloating after meals. The family history is significant for autoimmune diseases. A serological test reveals elevated levels of tissue transglutaminase antibodies (tTG-IgA).
Based on this scenario, what is the most appropriate next step in managing this child?
A Referral for an upper gastrointestinal endoscopy with biopsy
B Testing for other food allergies
C Immediate initiation of a gluten-free diet
D Prescribing anti-inflammatory medication

Q34- A 6-year-old boy is admitted to the hospital for elective surgery. On physical examination, he looks well-hydrated. His weight is 30 kg, and his height is 110 cm. The medical team decides to run a maintenance IV fluid for him. What is the patient's most appropriate volume in the next 24 hours?
A 1900 mL
B 1700 mL
C 1100 mL
D 2200 mL

Q34- In the delivery room, at 5 minutes after birth, a 28 weeks' gestation newborn who is wrapped in a plastic bag is receiving continuous positive pressure ventilation of 5 cm H₂O with a fraction of inspired oxygen of 70%. He is pink and well perfused, with a heart rate of 135 beats/min and flexion of all extremities. Of the following, the main reason for this preterm infant respiratory distress is:
A Transient tachypnea of the newborn
B Surfactant deficiency
C Pneumonia
D Diaphragmatic hernia

Q35- A 15-year-old female presents with a 2-month history of fatigue, generalized joint and muscle pain, and a malar rash. Labs show positive ANA, low complement levels, and proteinuria on urinalysis. What is the most likely diagnosis?
A Ankylosing spondylitis
B Systemic lupus erythematosus
C Sjogren's syndrome
D Rheumatoid arthritis

Q36- A 14-year-old girl comes to the physician with her father for evaluation of short stature. She feels well overall, but is concerned because all of her friends are taller than her. Her birth weight was normal. Her father reports he had a short stature during his

teenage years; he is currently 177 cm tall. The daughter is at the 2nd percentile for height and 35th percentile for weight. Breast development is Tanner stage 2. Pubic and axillary hair is absent. An x-ray of the left hand and wrist shows a bone age of 11 years. Which of the following is the most appropriate next best step in management?

- A Pelvic ultrasound
- B Growth hormone therapy
- C Reassurance and follow-up
- D Genetic karyotyping

Q39- An exclusively breast-fed, 5-day-old boy is brought to the physician by his mother for a routine examination. He was born at term and delivery was uncomplicated. He received all standard treatment and testing prior to being discharged from the hospital 3 days ago. Examination shows no abnormalities. Without receiving additional supplementation at this time, this newborn is at greatest risk of developing which of the following conditions?

- A Rickets
- B Microcytic anemia
- C Scaly dermatitis
- D Intracranial bleed

Q41: Newborn is delivered at term. Apgar scores are 6 and 8 at 1 and 5 minutes, respectively. Examination showed that he is at the 2nd percentile for head circumference and at 10th percentile for length. He has a small jaw, small eyes, and low-set ears. There is a keyhole-shaped defect in the iris of the left eye. He has seven fingers on each hand. There is a cleft palate. A holosystolic murmur is heard at the left lower sternal border. The heels are prominent, with convex-shaped deformity of plantar surface of the feet.

Which of the following is the most likely cause of these findings?

- A Trisomy 13
- B Fragile X syndrome
- C Trisomy 18
- D Trisomy 21

Q42- An 18-hour old neonate observed in the nursery to have a yellowish discoloration of the skin. He is a product of full term and unremarkable pregnancy. The baby is formula-fed since birth. The blood group of the mother is (O+ve) and the blood group of the baby is A+ve. On physical examination the neonate weighs 3200 g with scleral and skin icterus. The initial blood work showed high reticulocyte count (10%), total serum bilirubin was 13 mg/dL, and the direct serum bilirubin was 1 mg/dL. The direct Coombs test is +ve. The most likely underlying pathophysiology for hyperbilirubinemia in this baby is:

- A Physiological jaundice
- B Breast milk jaundice secondary to conjugation inhibitors in breast milk
- C Immune mediated hemolytic reaction secondary to blood group incompatibility
- D Biliary atresia

Q43- A 2-month-old girl presents to the emergency department with fussiness, increased sleeping, and poor feeding. No history of vomiting, diarrhea, or fever. On examination, she is difficult to console, her temperature is 36.8°C, heart rate is 160 beats/min, and her respiratory rate is 30 breaths/min. Her anterior fontanelle is full, and her pupils are dilated. The remainder of the physical examination findings are unremarkable. A complete blood count with differential count, electrolytes, and urinalysis results are normal. Computed tomography of the brain shows subdural hematoma. What is the most likely diagnosis?

- A Galactosemia
- B Meningoencephalitis
- C Von Willebrand disease
- D Nonaccidental trauma

Q44- A 7-year-old boy, not known to have any medical illness before, presented with a history of fever and headache for 3 days and abnormal movement for one day. On examination, he was lethargic and had a decreased level of consciousness. CS analysis showed

a clear color, mild WBC count elevation (mainly lymphocyte), normal glucose level, and slightly elevated protein.

What is the most likely diagnosis?

- A Autoimmune encephalitis
- B Tuberculous meningitis
- C Viral encephalitis
- D Bacterial meningitis

Q45- An 18-month-old boy presents with a cough and runny nose. His temperature is (37.9°C) Physical examination is only remarkable for a clear nasal discharge, and he is alert and playful. Which of the following is true?

- A The infant most likely has an undefined immunodeficiency.
- B A fever of 38°C after the first set of immunizations is an absolute contraindication to diphtheria, tetanus, and pertussis vaccine.
- C Minor illness with fever does not contraindicate immunization.
- D Fever alone is a contraindication to immunization

Q49- A 1-year-old boy is brought to the physician because of irritability and poor feeding that began 2 days ago. His mother reports that he has been crying more than usual during this period. He refused to eat his breakfast that morning and has not taken in any food or water since that time.

He has not vomited. When changing the boy's diapers this morning, the mother noticed his urine had a strong smell and pink color. He has not passed urine since then. He was born at term and has been healthy. He appears ill. His temperature is 38.8°C, pulse is 116/min, and blood pressure is 98/54 mm Hg. The boy cries when the lower abdomen is palpated.

Which of the following is the most appropriate next step in management?

- A Perform voiding cystourethrogram
- B Perform renal ultrasound
- C Perform transurethral catheterization
- D Administer cefixime

Q54- At a 4-month well-child visit, a mother is distressed because her baby's grandmother believes she should have started feeding her baby solid food at two months of age. What is the recommended age to introduce foods other than milk into the infant's diet?

- A 12 months
- B 8 months
- C 6 months
- D 10 month

Q56- A mother came to your clinic with her 9 years old girl. She asked you how she will know that her daughter is beginning puberty. Which of the following is the first visible sign of puberty?

- A Development of breast buds
- B Menarche
- C Appearance of early pubic hair
- D Appearance of axillary hair

57- A 3-month-old infant presents with jaundice that has progressively worsened since birth. The infant's stool appears pale, and urine is dark. On examination, the infant has hepatomegaly. The mother reports that the baby has been feeding well but has not gained weight appropriately.

What is the most definitive diagnostic test to confirm biliary atresia in this infant?

- A Abdominal ultrasound
- B CT scan of the liver and biliary tract
- C liver function tests
- D intrahepatic cholangiography

Q1- A 5 year old child with nephrotic syndrome was found to have renal vein thrombosis, which of the following blood test contribute to this complication?

- A. Hypocalcemia
- B. thrombocytopenia
- C. Antithrombin 3
- D. DIC

Q2- a 10 year old female presented with episodic hematuria following upper respiratory infections, which of the following is the the most likely diagnosis?

- A. Post strep glomerulonephritis
- B. Lupus nephritis
- C. Membranoproliferative glomerulonephritis
- D. IgA nephropathy

Q3- patient had taken IVIG for kawasaki (before 2 weeks?) and according to Saudi Arabia immunization table he is now in time for his 2 years vaccine?

- A. Unchanged immune response
- B. Decreased immune response
- C. Augmented immune response
- D. Delayed immune response

Q4- 3 year old boy with recurrent wheezing, admitted to the hospital at 9 months for bronchiolitis, the parents concern if he will develop asthma later in life, according to modified asthma predictive index, which of the following increase the risk?

- A. Male gender
- B. Prematurity
- C. Siblings diagnosed with asthma
- D. Wheezing not related to colds

Q5- School age boy that always interrupt the classes in school. He doesn't like to sit down. He did not like to follow) (حضانة لاوسلا rules. What is the diagnosis and treatment (ADHD) (وم تارايخلا ي قاذب) B. ODD
A. Attention deficit/ hyperactive disorder: behavioral therapy
C. Autism spectrum: speech and language therapy

Q6- A 7 years old boy have a cough and shortness of breath triggered by activity and improved with albuterol. You diagnosed him with asthma. What is the next step?

- A. Inhaled corticosteroids when take albuterol

Q7- 15 month old unvaccinated boy presented with a fever then a maculopapular rash that spread caudally. He also has a cough, coryza, and conjunctivitis. which one of the following is also seen in this disease?

- A. Subacute sclerosing panencephalitis after 7-10 years
- B. mouth ulcers
- C. Orchitis is well known postpubertal complication

Q8- End-stage renal Disease case, what is the cause of Anemia?

- A. inadequate erythropoietin

Q9- Which of the following primitive reflexes at birth and disappear at 6 months?

- A. Moro reflex
- B. Palmar grasp reflex
- C. Tonic Neck reflex
- D. Rooting reflex

Q10- 2 years old child what he can do?

- A.Run and kick a ball
- B.Ride a bicycle
- C.Say 10 words in one sentence
- D.Drown a circle نظا بهضاو تارايخلاو جضاو ناك لاوسلا

Q11- A. A child weight was on the 50th percentile and is now on the 25th percentile this happened in a one year

- B. A child's height on the 10th percentile

Q12- 3 year old with moderate respiratory distress and shortness of breath for three day, investigations and ABG was done :

ph 7.25

Co2 50

O2 82 ?

hco3 28

- A.Respiratory acidosis

Q13- 3 years boy come to ER with 2 day of fever and submandibular lymphadenopathy, what is the Red flags of lymphadenitis ?

A- 2 day fever

B- Local tenderness

C- Recent cold symptoms

D- Lymphadenopathy more than 4 weeks

Q14- 5 day old girl presented to the ED with tachycardia, fever, the antibiotic given should target which organism?

A-strep pneumonia

B-E.coli

Q15- 13 year old girl with abdominal pain and multiple episodes diarrhoea ate from a burger restaurant a week ago, has low hgb, high reticulocytes, high lactate dehydrogenase, high urea, high creatinine PBS showed schistocytes most likely?

A. Non Typhoid salmonella

B. Vibrio choleraF. Enterohemorrhagic

E. coli

NOT
SURE

Q16-

Q17-

Q18-12 year old female ESRD bruising and nose bleeding normal platelet count, prolonged bleeding

A.Immune thrombocytopenic purpura B.Disseminated intravascular coagulation DIC

C. UREMIC platelet dysfunction

D.VON WILLBRAND

NOT
SURE

Q19- a patient has IgA deficiency. Which of the following vaccines can be given to him?

A.Pneumococcal vaccine

B.Yellow fever vaccine

C.Oral polio vaccine

D.BCG vaccin

5. A four-month-old male infant presents with tachypnea, tachycardia, and hepatosplenomegaly. On auscultation, he had a soft pansystolic murmur in the left sternal border. A chest radiograph shows cardiomegaly and increased pulmonary markings. Which of the following is the most likely diagnosis in this patient? + Normal 0 oz Satwatin Equal pressure In Both Extremities

- A. Large ventricular septal defect

- B. Tetralogy of Fallot
- C. Small ventricular septal defect
- D. Inborn error of metabolism

Q24/ A long case mentions that there was an ejection systolic murmur in the upper left sternal and a fixed split second heart sound?

- A) Left to right, ASD
- B) Right to left, ASD
- C) Right to left, VSD
- D) Left to right, VSD

Q20- which of the following is a Risk factor for poor asthma outcome?

A. incorrect inhaler technique

Q21- 14 years old girl presented to the hospital with her family for routine check up, she didn't eat animal products, she has a pale conjunctiva, investigation showed high corpuscular volume. which of the following would be **هنا هو ما؟** expected to be deficient

- A. Iron deficiency
- B. Folate
- C. Niacin
- D. Oral Vitamin B12

Q22- 4 month old with digeorge syndrome what is one of the diagnostic criteria of digeorge syndrome (what is the cause)

1. 2. 3. 4. A. Digeorge syndrome is caused by gene mutation of the humoral immunity

B. Digeorge syndrome is caused by absence of the thymus and impaired T cell function

- C. Digeorge syndrome is caused by a defect in the natural killer cells
- D. Digeorge syndrome is caused by a defect in the complement system

Q23-10 weeks infant uneventful post-natal course present 10with pallor his HB

A. Physiologic anemia

Q24- Findings of leukocytosis present with persistent necrotising bacterial infection , in child known to have late separation of umbilical cord, most likely is :

- A. Shwachman Diamond syndrome- agammaglobulinemia
- B. chronic granulomatous disease –
- C. leukocyte adhesion deficiencies

Q25--A child has experienced two febrile seizures with no developmental delay, neurological abnormalities, and a normal EEG. What is the likelihood of developing epilepsy?

- A. high,
- B. low , because it is associated with fever
- C. potentially increased due to his history
- D. medium ,

Q26- The child brought by his parents due brief spells of staring into space and quite

unaware his EEG generalized spike and wave what is the diagnosis and its treatment

- A. Child absence seizure, ETHOSUXIMIDE and VPA
- B. Juvenile myoclonic epilepsy, lamotrigine
- C. INFANTILE SPASMS
- D. ROLANDIC EPILEPSY

Q27- A 10 year old boy presented to your clinic with fever and multiple joint swelling and pain, physical examination revealed tachycardia, tachypnea, and systolic murmur you suspect rheumatic fever what is your next step in the diagnosis?

- A. Cardiac enzyme
- B. antistreptolysin O
- C. Radiological X-ray

Q28- A 4 year old girl presented to your clinic with history of 5 day of fever you suspect Kawasaki, what is the diagnosis criteria?

- A. mouth ulcer
- B. cervical lymphadenopathy
- C. arthritis
- D. conjunctivitis bilateral exudative

Q29- 6 years old is brought to the emergency department. He has ingested many aspirin tablets, (few hours have passed since?), he appears lethargic and his breathing rate is 30 breaths per minute. The nurse took his vital signs as follows:

Blood pressure:

Heart rate:

Temperature:

What is the best initial management?

- A. activated charcoal
- B. gastric lavage
- C. intravenous fluids
- D. Sodium bicarbonate

Q30- 3 week old girl vomiting after breastfeeding, The vomit in large amount. In physical examination, there is a mass in the epigastric area. She is vigorous and crying and (nothing in the history). What is the Gold Standard for diagnosis?

- A. US
- B. Fluoroscopy
- C. X-ray

Q31- a 14 years old female has Erythema, tenderness, and proptosis of her left eye, on examination she has limited ocular movement and reduced visual acuity.

What is your management?

- A. Oral antibiotics and discharge home with close follow-up in the clinic
- B. Prescribe antibiotics & Urgent ophthalmological consultation

Q32- An unvaccinated 7-year-old boy, not known to have any medical illness before, presented with a history of fever, headache, and stiff neck for three days. On examination had a positive Kernig sign.

CSF analysis showed (Table)

turbid color, 500 WBC count (mainly polymorphs), glucose level <50% compared to the serum glucose, and 2.1g protein. What is the most likely diagnosis?

- A. Viral meningitis
- B. Acute Encephalitis
- C. Optic neuritis
- D. Bacterial Meningitis

Q32-A five-year-old girl presented to the clinic with a history of fever, mild ear pain and ear discharge for two days. On examination, she looks well, STD

Temp: 38.5°C, heart rate: 150 beats per minute, and other vital signs were unremarkable. A left ear examination showed bulging, dullness of the tympanic membrane, and otorrhea. What are the next best steps in her management?

- A. High dose amoxicillin for 10 Days.
- B. Symptomatic therapy and re-asses after 48-72 hours
- C. Consult otolaryngologist for tympanic surgery
- D. Blood culture from ear and other tests

Q33- Case of 5 years patient with fever, barking cough and inspiratory stridor (CROUP features). Which of the following is the appropriate next step?

Reassure and send home

Oral dose dexamethasone

Q34- 2 years with stridor, barking cough (croup features) what is the diagnosis?

- A. Croup
- B. Exacerbated asthma

Q35-2weeks old infant was born at 36 weeks he weighed 2.4 kg at birth. His parents came to you worried about his intake. His current weight is 2.4 kg.

What would you advice the parents A. Encourage breastfeeding (dr. Ahmed says this is the right answer)

- B. Prescribe premature formula
- C. 80 kcal/kg/day
- D. Prescribe soy formula

Q36-14 years old boy has constitutional growth disease and his parents are concerned about his normal growth. ?Which one is correct

A. good prognosis to adult height NOT SURE!

- B. bone age is equal to chronological
- C.
- D.

Q37- calculate Daily maintenance fluid 18kg

A-1400L

Q38/39

. A baby is presented to your clinic. He is able to sit with support, babble, and now it's just started to transfer objects between his hands. What is his age?

- A. 10 months
- B. 4 months
- C. 8 months
- D. 6 months

2. The expected postnatal normal cardiac and respiratory transition is very necessary for the newborn to achieve normal adult flow circulation. Which of the following physiological changes is expected to happen in postnatal transition?

- A. Pulmonary vascular resistance decreases
- B. Pulmonary flow decreases
- C. Ductus arteriosus remains patent
- D. Prostaglandin concentration increases

28) A 1-year-old boy is brought to the hospital with low-grade fever, frequent vomiting, and acute diarrhea.

Physical exam shows pulse 150 bpm, respiratory rate 50 bpm with deep breaths, decreased skin turgor, lethargy,

and delayed capillary refill of 3 seconds. His weight is 10 kg. Which of the following is the next step in his management?

- A. Half normal saline bolus of 200 mL over 30 minutes
- B. Normal saline bolus of 200 mL over 30 minutes
- C. Normal saline at a rate of 40 mL/hour
- D. Half normal saline with dextrose 5 at a rate of 40 mL/hour

Q1: 14 yrs girl, comes for short stature in comparison to her classmates. Her father states that he had short stature throughout his teenage years and is now 177cm. She is 3th percentile for length and 35th for weight. Her breasts are tanner stage 2, absent pubic and axillary hair. Xray of her left wrist and hand shows 11 years bone age. What would be the management?

- A. Ressurreance and follow up
- B. Pelvic ultrasound
- C. Growth hormone
- D. Karyotyping

Q3: A girl in her breakfast had peanuts, 10 minutes later had trouble breathing etc. paramedics found angioedema, urticaria, and hypotension

What is the pathophysiology of anaphylaxis?

- A. IgE mediated with mast and etc
- B. Autoimmune the body attacks itself
- C. Delayed hypersensitivity
- D.

Q4: — months old has (36.8 or 37.8) fever. He looks alert and playfull what is correct?

- A. Minor illness and fever are not contraindication to immunization
- B. Fever alone is contraindicated
- C. If a child has a fever it is contradicted to have DTaP
- D. “ rest of the options were unlikely to be the answer”

Q5: 16 years old girl has primary amenorrhea, both her mother and sister had their menstrual period. “On examination she has a blind vaginal pouch (not sure if it was part of this question or not)”

- A. Karyotype
- B.
- C.
- D.

Q6: A case of a patient who has features of celiac disease and asked about the diagnosis?

- A. Celiac antibodies
- B. Gluten free diet
- C. CT for detecting structural abnormalities

Q10: Case of a patient with light hair and musty urine, he was diagnosed with phenylketonuria. What is management? how to manage patient with PKU

- A. lot of protein diet
- B. Phenylalanine free diet (phenylalanine diet restrictions)
- C.
- D.

Q11: Calculate the fluid maintenance of child 6 years old, and his weight is 30 kg

- A. 1700 ml/kg
- B. 1500 ml/kg
- C.
- D.

Q13: A baby delivered by caesarean section because of ruptured membranes, his apgar score were 1-3 at 3-5 minutes respectively. PPV, intubation and chest compression were done and IV epinephrine was given. What measurement should be done to preserve neurological function?

- A. Therapeutic hypothermia
- B. Mannitol
- C. Antibiotics
- D. Thiamine

Q14: Patient presented with +ANA Oligo juvenile idiopathic arthritis, of this disease? other than uveitis what would be a systemic complication

- A. Growth delay
- B. Macrophage activate syndrome

Q15: A patient had petechiae and ecchymosis with no family history of such a condition. He had a PLT count of 33000. What is the best therapy?

- A. Observe
- B. Steroid
- C. Immunoglobulin
- D. Prophylaxis antibiotic

Q16: Case of an infant that is exclusively breastfed, He is at risk in future to develop?

- A. Rickets
- B. Microcytic anemia

Q17: Case of patient with RDS symptoms. What is the cause for this condition?

- A. Surfactant deficiency
- A.
- B.
- C.
- D.

Q18: Case of 9-month-old, his parents are saying that he is not growing well. He is exclusively breastfeeding. His mother says he is feeding well. The examination showed that he was normal and alert. Which of the following is the reason for failure to thrive?

- A. Calories intake
- B. Psychosocial
- C. Gastric malabsorption

Q20: Newborn had a unilateral patch in the upper eyelid and forehead. What is the rash?

- A. Port- wine
- B. Salmon patch (stork bites)
- C.
- D

Q22: Case of a newborn that is unstable, they have given him positive pressure ventilation, then chest compressions, it showed slight improvement but still tachycardiac. What is the best next step? A. Give epinephrine by ET tube until vascular access is ready

- B.
- C.
- D.

Q23: 2 year old came to ER with noisy breathing, cough, and chest retractions, with a history of upper respiratory infection for 2 days? (not 100% sure)

- A. Give IV antibiotics
- B. Observe for hypoxia and dehydration
- C. Give steroid and epinephrine
- D.

Q24: A 7 year old patient presents with pallor fatigue and hepatosplenomegaly labs show low Hb and low Platelets which of the following is the most likely diagnosis?

- A. Wilms tumor
- B. Hodgkin lymphoma
- C. B cell lymphoblastic leukaemia
- D. medulloblastoma

Q25: A mother asking about preterm complications?

- A. Child will not complete high school
- B. Spastic diplegia is a risk of preterms
- C.
- D.

Q27: A mother noticed a difference in skin folds in her girl's thigh ,she was born breech, what is the screening done?

- A. Ultrasound
- B. Xray
- C. CT

Q28: a long case about an asthmatic patient. Ph 7.41 Co2 29 Bicarb 21

- A. Metabolic acidosis
- B. Metabolic alkalosis
- C. Respiratory alkalosis
- D. Respiratory acidosis

Q29: a case of a 2 months child with persistent vomiting for 2 days and other unimportant details. How to calculate anion gap?

- A. Sodium, chloride, and bicarbonate
- B. Glucose,...etc
- C. Magnesium,...etc

Q30: a long case about 1 year old male, looks ill his fever 36.5, he refused to feed since 2 days, he didn't urinate much these period, the mother when she changed his diaper she found pink color on the diaper, and the tender in palpating pubic area , what is the next to do for this baby?

- A. Transurethral Catheterization (confirmed by dr)
- B. Renal Ultrasound
- C. VCUG
- D. Give cefixime

Q31: A kid hit his knee on the car door, and it swelled up, examination revealed that it is full of blood, we suspect haemophilia A, what helps in diagnosis?

- A. Prolonged PTT
- B. Low platelets
- C.
- D.

Q32: 4 weeks infant on formula since birth with yellowish sclera and skin, poor feeding, vomiting, dark urine, pale stool. Bilirubin test showed Total Bilirubin 157, high direct bilirubin, imaging test was done. Which of the following is most important to investigate?

- A. Biliary atresia
- B. Hemolytic blood disorder
- C. Physiological breastfeeding hyperbilirubinemia

Q33: New mother was wondering when to introduce solid food to her baby?

- A. 2 months
- B. 6 months
- C. 9 months
- D. Any age

Q34: Baby girl, she can transfer object, sit with support, mimicking voices, what is her age?

- A. 6 months
- B. 4 Months
- C. 3 Months
- D. 9 Months

Q35: Type 1 diabetic girl presenting with fever, diarrhea, vomiting for 3 days. She appeared well hydrated. Her fasting blood glucose is 260 mg/dl and normal bicarbonate levels, Urine analysis showed ketone bodies. After adjusting the insulin dose.

What is the next step?

- A. admission immediately
- B. telling her to measure her urine ketones daily
- C. telling her to measure her blood glucose daily
- D. tell her to increase her hydration

Q36: 18-month-old female child presents with a blood-streaked stool. The stool is positive on occult blood testing. Which of the following diagnoses is most likely?

- A. Anal fissure
- B. Peptic ulcer disease
- C. Mallory- Weiss tear
- D. Inflammatory bowel disease

Q37: 13 years old girl amenorrhea short (said how tall she is) her mother is (taller) father is (taller) what is most likely diagnosis for this?

- A. Testicular feminization
- B. Turner syndrome

Q38: A 4-year-old girl presented with fever, slapped cheeks rash, she has 2 brothers with sickle cell anemia, what complications will they have if the infection transmits?

- A. Transient aplastic crises
- B. Spleen rupture
- C. Pure cell aplasia.

Q39: Which of the following is the definition of prematurity.

- A. Less than 38 weeks
- B. less than 37 weeks
- C. less 40 weeks
- D. less than 34 weeks

Q41: newborn done all examination but discharged without any supplements he is at risk of? (Vit K was not in the question)

- C. Rickets

Q42: hyperammonia and metabolic acidosis, what is a urea cycle defect?

- D. Ornithine transcarbamylase (OTC)

Q43: A 9-year-old boy presented with joint pain and fever. Upon further history taking, he told you that he went to his grandfather's farm one week ago, drinks cow milk. What's the most likely causative agent?

- A. Brucella
- B. Staphylococcus aureus
- C. Campylobacter

Q44: A patient with malar rash, abdominal pain and arthritis and later developed proteinuria, what is your diagnosis?

- A. SLE
- B. Rheumatoid Arthritis

Q45: Pt developed jaundice, Mother O+ baby A+, Coombs test +ve his condition is due to?

- A. ABO incompatibility
- B. Rh incompatibility
- C. Isoimmunization

Q46: a mother give her 6 month old babe a cow milk he is at risk of(long question)

- A. IDA

Q48: A child has gastroenteritis and diarrhea from Rota after treatment and going home the mother supplied him with cow milk, later he developed diarrhea again which of the following is more likely?

- A. give the child lactose free milk for a period of time ☒
- B. give him diluted milk until symptoms resolve

Q49: Case with symptoms of encephalitis and with following glucose.

- A. viral encephalitis
- B. bacterial meningitis
- C. TB CSF findings: clear, mild increase in WBCs, normal
- D. Autoimmune encephalitis

A child presents with fever , lethargy, neck stiffness and irritability CSF analysis were done and showed turbid fluid with high WBC mainly polymorph high protein and low

glucose which of the following is most likely diagnosis ?

- A. bacterial meningitis
- B. viral meningitis
- C. meningitis due to TB
- D. viral encephalitis

Q50: A 9 months old presented to the clinic with a fever of 38.7°C and ear pulling. On physical examination you found erythematous bulging right tympanic membrane with purulent effusion. You diagnosed him with otitis media. What is the most

likely organism that caused this infection?

- A. Group b streptococcus
- B. Streptococcus pneumoniae
- C. Group a streptococcus
- D. Streptococcus viridans

Q51: 11 year old boy is brought to the emergency department with a 1-day history of right sided abdominal pain, fever and vomiting. On examination, he has a temperature of 37.9°C and heart rate of 112. Abdominal

examination revealed rebound tenderness in the right iliac fossa. What is the most likely cause?

- A. Diverticulitis
- B. Appendicitis
- C. Pancreatitis
- D. Carcinoma

Q52: A case about a girl with recurrent chest infection and oral thrush, labs show neutropenia, leukopenia and further investigation revealed no T-cells, but B-cell and NK are present. What is your diagnosis?

- A. SCID
- B. X-linked Agammaglobulinemia
- C. hyper IgM
- D. Wiskott-Aldrich syndrome

Q53: patient came with persistent proteinuria dipstick no symptoms or complications seen otherwise what is your next step in investigation?

- A. Kidney biopsy
- B. Renal US
- C. complement level
- D. See the protein/creatinine ratio

Q54: The expected post-natal normal cardiac and respiratory transition is very necessary for the newborn to achieve the normal adult flow circulation. Which of the following physiological changes is expected to happen in post-natal transition?

- A. Pulmonary vascular resistance decreases
- B. Decrease pulmonary flow
- C. Ductus arteriosus remains patent
- D. Increased prostaglandins concentration

Q57: What is the first sign of female puberty?

- A. Formation of breast buds

Q59: Child with fever for 5 days and has Kawasaki. What would you find in an examination?

- A. bilateral exudative conjunctivitis
- B. Cracked lip

Q60: Patient was diagnosed with asthma what drug should he start with?

- A. SABA as needed
- B. SABA + ICS as needed

Q63: A patient with Slurred speech and left face twitching at night time no focal deficit what you will see on ECG ?

- A. Occipital spikes
- B. Central temporal spikes
- C. spikes upon hyperventilation
- D. Three hertz spikes

Q64: (case) most common cause of resp distress in

١٧ صيغة أخرى للسؤال رقم? preterm

A. Surfactant deficiency

B.

C.

D.

Q65: You are awakened in the night by your 2-year-old son, who has developed noisy breathing on inspiration, marked retractions of the chest wall, flaring of the nostrils, and a barking cough. He had a mild upper respiratory infection for 2 days. Which of the following will be done in

٢٣ صيغة أخرى للسؤال رقم? the emergency?

“ we are not sure if this was the question about a case of croup, or if it was another question about a case of bronchiolitis but it was easy and from previous patches”

A. Short-acting bronchodilators and a 5-day course of

Steroids

B.. Intubation and antibiotics

C. Observation for hypoxia and dehydration alone

D. Inhaled epinephrine and a dose of steroid

1. A 9-months old girl presented with a history of fever (38.5-C); she is clinically well and has no clear focus. She is up to date on her vaccination, and her growth parameters are appropriate for her age. What is the best next step?
 - A. Blood culture and CSF studies
 - B. Chest X-ray
 - C. Urine analysis and culture
 - D. Start empiric antibiotics

2. The expected post-natal normal cardiac and respiratory transition is very necessary for the newborn to achieve the normal adult flow circulation. Which of the following physiological changes is expected to happen in post-natal transition?
 - A. Pulmonary vascular resistance decreases
 - B. Decrease pulmonic flow
 - C. Ductus arteriosus remains patent
 - D. Increased prostaglandins concentration

3. 23- month female is hospitalized for severe malnutrition. She has inappropriately received rapid nasogastric feeding. Which of the following laboratory values is most concerning as iatrogenic complication?
 - A. Hyponatremia
 - B. Hypophosphatemia
 - C. Hyperglycemia
 - D. Hyperkalemia

4. You are following a 13 old boy with asthma. Which of the following is a reliever therapy?
 - A. Seretide (salmeterol and Fluticasone)
 - B. Fluticasone
 - C. Zafirlukast
 - D. Symbicort (Budesonide and formoterol)

5. Four-month-old male infant presents with tachypnea, tachycardia and hepatosplenomegaly. On auscultation he had Soft pan-systolic murmur in the Left sternal border. A chest radiograph shows cardiomegaly and increased pulmonary marking. Which of the following is the most likely diagnosis in this patient?
 - A. Large ventricular septal defect
 - B. Tetralogy of Fallot
 - C. Small ventricular septal defect
 - D. Inborn error of metabolism

6. Case of patient with recurrent infections and pneumonia, CBC showed leukopenia and lymphopenia. Flow cytometry showed no T cells, normal NK and B cells. Which of the following is the most likely diagnosis in this patient?
 - A. X-linked agammaglobulinemia
 - B. Severe combined Immunodeficiency
 - C. Hyper Ig-A syndrome
 - D. Wiskott-Aldrich syndrome

7. Case of patient having Red urine. He has recently common cold
Urinalysis showed +2 protein, 20-30 RBCs per high power field and dysmorphic red blood cells. Which of the following is the most likely diagnosis in this patient?

- A. Rhabdomyolysis
- B. Membranous glomerulonephritis
- C. Ig A nephropathy
- D. Nephrolithiasis

8. A five-year-old girl presented to the clinic with a history of fever, mild ear pain and ear discharge for two days. On examination, she looks well, Temp: 38.5°C, heart rate: 150 beats per minute, and other vital signs were unremarkable. A left ear examination showed bulging, dullness of the tympanic membrane, and otorrhea. What are the next best steps in her management?

- A. High dose amoxicillin for 10 Days.
- B. Symptomatic therapy and re-asses after 48-72 hours
- C. Consult otolaryngologist for tympanic surgery
- D. Blood culture from ear and other tests

9. The following are part of high-quality CPR, EXCEPT:

- A. Allow the chest to fully recoil
- B. Compressions are at least 100 per/min
- C. Chest should be depressed at least 2/3 of its AP
- D. Minimize interruption

10. What is the first pubertal changes in female?

- A. Axillary hair
- B. Menarche
- C. Pubic hair
- D. Breast bud

11. You are evaluating a healthy breastfed newborn. In which of the following conditions would you strongly advise against breastfeeding?

- A. The mother has active pulmonary TB infection
- B. Baby with congenital infection
- C. The mother will return to work as a full-time job
- D. The mother is receiving Acetaminophen as analgesic medication

12. Child has wheezing and sudden cough when he was alone in the room. Chest x-ray show unilateral hyperinflation. Asthma therapy started but did not show any improvement. what's the next step?

- A. Bronchoscope
- B. Oral steroid
- C. More dose of inhaled corticosteroids
- D.

13. A 9-month-old girl is diagnosed with iron deficiency anemia. Her past medical history includes an uncomplicated delivery full term baby. The infant was fed with formula until 6 months of age, at which time she was switched to baby foods and whole cow milk. Which one of the following is correct regarding her iron-deficiency anemia?

- A. Her age is atypical for the presentation of iron deficiency anemia
- B. Full term baby has high risk of iron deficiency anemia
- C. Early introduction of cow's milk is the likely cause of her anemia
- D. The patient's early introduction of solids is the likely cause of her

Anemia

14. An 8-year-old boy has a 1 year history of cough that is worse at night and with exercise. Which one of the Following tests is most likely to assist you to make a diagnosis?

- A. Barium swallow
- B. Bronchoscopy
- C. Chest radiograph
- D. Spirometry

15. Indication of Renal biopsy in patient with nephrotic syndrome?

- A. Gross hematuria
- B. Age between 1 and 8 years
- C. Protein/creatinine ratio of 2.5 mg/dl
- D. Serum albumin concentration less than 2.5g/dl

16. An unvaccinated 7-year-old boy, not known to have any medical illness before, presented with history of fever, headache, and stiff neck for three days. On examination had a positive Kernig sign. CS analysis showed a turbid color, high WBC count (mainly polymorphs), low glucose level compared to the serum glucose, and high protein. What is the most likely diagnosis?

- A. Viral meningitis
- B. Acute Encephalitis
- C. Optic neuritis
- D. Bacterial Meningitis

17. A 9-month-old presents to your office with 2-day history of temperature of (38.7°C) and fussiness. Mother reports that the child has been pulling at her ear. On physical examination, you note an erythematous, bulging, right tympanic membrane with purulent effusion. You diagnose otitis media. What organism is the most likely cause of this infection?

- A. Streptococcus pyogenes
- B. Streptococcus pneumoniae
- C. Staphylococcus aureus
- D. Pseudomonas aeruginosa

18. A school-age child presented to your office with a fever and sore throat history. You diagnose streptococcal pharyngitis based on a positive rapid test. You start the appropriate antibiotic treatment to prevent which one of the serious complications?

- A. Otitis media
- B. Post-streptococcal Glomerulonephritis
- C. Rheumatic fever
- D.

19. Neonate case of a patient that have tachycardia, tachypnea, prolonged capillary refill more than 4 seconds (shock). Cultures was taken and the result show Gram + cocci. What is the organism?

- A. Group B streptococcus
- B. Streptococcus pneumoniae
- C. Group A Streptococcus
- D. Streptococcus viridians

20. Case of female that had a seizure, after detailed history it shows that she had also another seizure before 6 months. Both episodes were associated with fever. There isn't family history of epilepsy. Examination showed no developmental delay. What is the probability to develop another epilepsy?

- A. Low
- B. Moderate
- C. High
- D. Mildly increased because it is associated with fever

21. 6 years old child with recurrent brief seizure, EEG showed generalized spikes what is the diagnosis + and appropriate treatment for this condition:

- A. Childhood Absence Epilepsy/ valproic acid
- B. Benign rolandic epilepsy / no need for treatment
- C. Juvenile myoclonic epilepsy (ناسين السيناريو، لكن بسهولة تستبعد هالاجابة)
- D.

22. 2-week neonate on formula was 3.6 kg in birth. Now he is 2.8 Kg, He sleeps around 18 hours your concern about what?

- A. Sleep hours
- B. Calorie intake
- C. Ingestion of solid food
- D. Vaccination

23. Patient came with abdominal pain, joint pain and rash in the lower limbs (Henoch Schoenlein purpura symptoms); which of the following you should monitor?

- A. IgA
- B. Urinalysis (متأكد من الإجابة)
- C.
- D.

24. Which of the following is true about PCV vaccine?

- A. Vaccine work against 13 serotypes
- B. Cannot be given after antibiotic
- C.
- D.

25. Patient with fever for 5 days + cervical lymphadenopathy + red eyes + dry lip + rash

- A. Rheumatic fever
- B. Kawasaki disease
- C.
- D.

26. A 6-week-old infant presented with a persistent non bilious vomiting. On examination, he was dehydrated with visible peristalsis on abdominal examination. What is the most likely electrolyte disturbance that would be found on laboratory investigation?

- A. Hypokalemia, hypernatremia, metabolic acidosis
- B. Hypokalemia, hyponatremia, metabolic alkalosis
- C. Hypokalemia, hyponatremia, metabolic acidosis
- D. Hypokalemia, hypernatremia, metabolic

27. Case of a mother came with her daughter to the clinic; the daughter is 13 years old and had never menstrual period. After assessing the growth parameters, it found that she is below the 5th percentile in height. She has 2 older sisters but without the same complain. Which of the following do you think is the cause?

- A.
- B.
- C. Turner syndrome
- D. Adrenal hypoplasia

28. 5y old Boy present with GERD and failed with conservative management what is the next step?

- A. Initiate medical therapy
- B. Gastric arthroplasty
- C. Diet
- D.

29. all of the following is true about salicylate except :

- A. Charcoal is effective
- B. It cause respiratory acidosis and metabolic Alkalosis
- C. Alkalization of urine increase salicylate elimination
- D.

E. Hyperglycaemia followed by hypoglycaemia

30. A child with fever , headache and behavioral changes, the did a CSF analysis turned to be clear , glucose is normal, protein slightly increased, and lymphocyte increased, imaging was done showed temporal lobe localization, what is the best next thing in management

- A. lumbar puncture
- B. Antibiotics
- C. IV acyclovir
- D.

31. Female patient had oligoarthritic and ANA +. Which of the following is a complication?

- A.
- B.
- C. Uveitis (ركزت عليها في شرح المحاضرة)
- D.

32. Case of patient with fever, barking cough and inspiratory stridor (CROUP features). Which of the following is the appropriate next step?

- A. Reassure and send home
- B. One dose dexamethasone (ركزت عليها في المحاضرة)
- C.
- D.

33. patient with beta thalassemia major on chronic blood transfusion, what complication will be seen?

- A. Vaso occlusive crisis
- B. Acute chest syndrome
- C. Iron overload
- D. Stroke

34. School age boy that always interrupt the classes in school. He don't like to sit down. He did not like to follow rules. What is the diagnosis?

- A. ADHD
- B. Autism spec
- C.
- D.

36. Case of patient with constitutional growth delay

Which of the following is correct?

- A. Bone age equal to chronological age
- B. Prognosis to get normal hight is good
- C.
- D

37. Case of patient 4 years old comes with his mother, he didn't get medical advice from long time. After reviewing his immunization history, they found that he received only the 2 months and 4 months immunization. Which of the following is the appropriate thing to do?

- A. Give him the immunization for his current age and catch up for the previous immunization
- B. Give him the immunization for his age only and ignore the before
- C. You should repeat the 2 and 4 months because he had long time without immunization
- D.

38. A child with Renal failure patient missed dialysis and looking sick which of the following will be seen in his blood gas?

- A.bone age equal to chronological age
- B. High anion gap metabolic acidosis
- C. Metabolic alkalosis
- D. Respiratory acidosis

39. Child came with blood pressure 114/74. After plotting it in the appropriate chart, it was between the 50th and 90th percentile. What is the next step?

- A. Start appropriate diet and healthy life style
- B. Start treatment of hypertension
- C.
- D. Reassure the parents that he is normal

40. 10 weeks infant, uneventful post-natal course present with pallor. His Haemoglobin is 10. He has normally developed milestones. What is the diagnosis?

- A. Iron deficiency anemia
- B.
- C.
- D. physiologic anemia

41. A young boy presented to the hospital with his family for routine check-up, the mother mentioned that the family follows a strict vegan diet, which of the following would be expected to be deficient?

- A. Iron deficiency
- B. Folate
- C. Niacin
- D. Vitamin B12

42. A 4-month-old boy sees his general practitioner for an ear infection. On listening to his chest, a heart murmur is heard. Which of the following features most suggests that it requires further investigation?

- A. Presence of a thrill
- B. Disappearance of murmur on lying flat
- C. A body weight of 6 kg
- D. Systolic murmur

43. case about developmental milestones. He mentioned that he can draw a cross/ square. What is his age?

- A. 2 years
- B. 3 years
- C. 4 years
- D. 5 years

1. A 12-year-old boy presented to your clinic with a history of myoclonic seizure and loss of neurological function; upon taking a further history from the mother, she recalled that ten years back, he had a history of maculopapular rash and was diagnosed as a viral illness, which one of the following viruses present with his complication?

- A. Measles
- B. Mumps
- C. Rubella
- D. Herpes simplex virus

2. 10 years old child developed fever and exudative tonsillitis. Prominent lymphadenopathy and lethargy. Amoxicillin was prescribed. The next day after she developed a maculopapular rash. Abdominal exam shows splenomegaly. Which of the following is the most likely cause of rash?

- A. Epstein Barr virus infection- Amoxicillin reaction
- B. Immune complex
- C. Streptococcus pharyngitis
- D. Parvovirus infection (fifth disease)

3. During your discussion with a mother, you mention that her boy will be getting two immunizations that day: varicella vaccine and measles-mumps-rubella vaccine. The mother states that she does not want her infant to get two vaccines in 1 day and asks if she can bring him back in 2 weeks for the second vaccine.

Which of the following is the best response?

- A. If live vaccines are not given simultaneously, administrations should be separated by at least 28 days.
- B. Live vaccines may be given within 4 days of one another.
- C. Live vaccines may be given within 2 weeks of one another.
- D. There is an increased risk of adverse reactions if more than one live vaccine is given on the same day

4. The mother of an 8-year-old boy is present with him for a preschool check-up and is concerned because he has not grown in height for the last 2 years. Which of the following best approximates the prepubertal annual increase in height?

- A. 1-2 cm
- B. 2-4 cm

- C. 4-8 cm
- D. 8-12 cm

5. Case of patient 7kg with gastroenteritis that had unstable vital signs (shock) a bolus was given. The vital signs improved a little but still have BP 85-64 and tachycardia. What is the best next step?

- A. Repeat another bolus with 140ml 0.9% NS in 30 minutes
- B.
- C.
- D.

6. Rides a tricycle and draws a cross and helps to dress himself what is the expected age?

- A. 16 months
- B. 24 months
- C. 30 months
- D. 36 months

8. Patient had E coli 157 present with acute renal failure and anemia (Hemolytic uremic syndrome). What do you expect to see?

- A. Thrombocytosis
- B. High haptoglobin
- C. Fragmented RBC
- D.

9. Case of infant that is exclusively breastfed, He is at risk in future to develop?

- A. Rickets
- B. Microcytic anemia
- C.
- D.

10. Which of the following is the earliest time you can give cow milk?

- A. 3 months
- B. 6 months
- C. 1 year
- D. 2 years

11. Case of neonate 4 days old with jaundice, Bilirubin was measured and it was in the phototherapy zone. What is the next step?

- A. Oral hydration and phototherapy
- B. Exchange transfusion
- C.
- D.

12. Case of Neonate has been 46 hour without passing meconium, He had bilious vomiting.

- A. Duodenal atresia
- B. Hirschsprung disease
- C. Necrotic enterocolitis
- D.

13. Case of patient with RDS symptoms. What is the cause for this condition?

A. Surfactant deficiency

B.

C.

D.

17. 3-week-old infant, is noted to have microcephaly, periventricular calcification on skull x-ray, and blindness. Which of the following is the most likely cause of these findings?

A. Bilateral subdural hemorrhage

B. Cerebral agenesis

C. Primary microcephaly

D. Congenital infection (cytomegalovirus)

21. Case of patient with recurrent chest infections, failure to thrive, weight loss despite good appetite. His skin tastes salty. Which of the following is most likely diagnosis?

A. Cystic fibrosis

B.

C.

D.

25. A 4-year-old came to the clinic complaining of freckles on the axilla, 6 café au- lait spots and some other neuro manifestation. What was the diagnosis?

A. Tuberous sclerosis

B. Neurofibromatosis type 1

C. Sturge- weber

D.

26. Newborn had unilateral patch in upper eyelid and forehead. What is the rash?

A. Port- wine

B. Salmon patch (stork bites)

C.

D.

30. Hyperbilirubinemia case, High direct bilirubin. Doctor suspect biliary atresia and Order further investigation. Which of the following is next step in the management of the case?

A. Kasai Surgical procedure (Treatment)

B. Liver biopsy to confirm diagnosis (gold standard for diagnosis)

C. ERCP (investigation if Ultrasound is negative)

D. (Forget it but it was not ultrasound)

(السؤال حذف لأن صار في لخبطة ما ندري هل هو يبغى التشخيص والعلاج)

32. Case of 9-month-old that his parents saying that he is not growing well. He is exclusively breastfeeding. His mother says he is feeding well. The examination showed was normal and he was alert. Which of the following is the reason of failure to thrive?

A. Calories intake

B. Psychosocial

C. Gastric malabsorption

D. ناسين الخيار لكنه خيار محتمل ويشكك

36. Case about a boy with a previous history of Upper respiratory tract infection 1 week ago, as well as he didn't go to toilet for the past 3 days. presents to you now with an odd smelling urine. Which of the following is the strongest risk factor for this condition?

- A. URTI
- B. Male gender
- C. Constipation
- D. Posterior Urethra Valve

37. An infant with visible skin findings as shown and a diagnosis of Tuberous Sclerosis Complex (TSC) presents with clusters of sudden, symmetric muscular contractions, especially upon waking. The episodes are characteristic of infantile spasms. An EEG demonstrates disorganized pattern with high amplitude spikes and slow waves, indicative of hypsarrhythmia. Given the clinical presentation and the skin findings, which of the following is the most appropriate initial treatment?

- A. Vigabatrin as 1st line therapy
- B. Anti-epileptic drugs
- C.
- D.

38. An 8-year-old boy was seen in the clinic for evaluation of nighttime bedwetting. He has never been dry at night and has urinary accidents 3-4 nights per week. The boy's parents report that he has been completely continent during the day since 3 years of age. He has no history of urinary frequency, urgency, or hesitancy. He has a normal daily bowel movement and has not had fecal soiling. There is no snoring or breathing issues during sleep. He is embarrassed to sleep at his cousin's house. His father also had nighttime bedwetting until he was 9 years old. He is at the 50th percentile for weight and height. He has no skin lesions on his back. His abdomen is soft with no organomegaly. His neurological examination findings are normal. A urinalysis has normal results. Which of the following is considered the best next step?

- A. A trial of decreasing fluids at bedtime and use of bedwetting alarms
- B. A trial of oxybutynin
- C. A trial of imipramine
- D. Request for lower spine MRI

39. Case of 3 month infant female with hip pain (suspected DDH). She delivered in breech position. What is the best modality for diagnosis.

- A. US
- B. X-ray
- C. MRI
- D. CT scan

40. Female with knee pain. It has association with ocular defect. What will be also associated?

- A. Erythrocyte sedimentation rate
- B. Anti-nuclear antibody
- C.
- D.

41. A 9-month-old girl had fever for the past 3 days (40c), she developed a maculopapular rash after her fever went away. What organism is the most likely cause of this infection?

- A. Roseola infantum
- B. Parvovirus B19
- C. Measles virus
- D. Varicella zoster virus

42. Patient had 2 generalized tonic clonic seizures in 6 hours each one taken 7 minutes patient had febrile seizure probability of getting epilepsy?

- A. 0% as it's a provoked seizure
- B. 1% as general population risk
- C. Higher risk 6% because it is atypical
- D. Higher risk 38 %

(السؤال حذف لأن كثير غلطوا فيه لكن ركزوا على الجدول في محاضرة العنزي لأنه جاب منه كثير في الميد والفاینل)

43. 10 year old patient had periorbital edema, pitting edema, frothy urine, hypertension, hypoalbuminemia, hypercholesterolemia. What is the most likely diagnosis?

- A.
- B.
- C.
- D. Minimal change disease

44. Non bilious vomiting right upper quadrant abdominal mass (pyloric stenosis), what is the next step?

- A.
- B.
- C. Pyloromyotomy
- D. Correct electrolyte imbalance

47. DiGeorge is a defect in which of the following?

- A. Cellular
- B. Humoral
- C. Complement
- D. Phagocytic

49. Case about a mom who found an abdominal mass for her daughter, she's hypertensive and have hematuria. What is the diagnosis?

- A.
- B.
- C. Wilms tumor
- D. Nueroblastoma

51. Patient had G6PD, which lab findings is true?

- A. High reticulocytes
- B. Thrombocytosis
- C. High haptoglobin
- D.

52. Infant present with fever they did urine analysis and tranurethral cath it shows > 50,000 CFU E.coli.

- A. It is diagnosed of UTI that require treatment
- B. Is not significant

53. Patient with hepatosplenomegaly (Henoch-scolion- purpura) , what will you see in lab?

- A. IgA
- B.
- C.

54. Oral rehydration solution contains?

- A. Half spoon salt and 6 spoons of sugar dissolved in 1 liter of water
- B. Half spoon salt and 6 spoons of sugar dissolved in 1 liter of apple

juice

- C. Half spoons sugar and 6 spoons of salt dissolved in 1 liter of apple

juice

- D. Half spoons sugar and 6 spoons of salt dissolved in 1 liter of apple

Juice

55. An 6-month-old boy is brought to the emergency department because of intermittent episodes of inconsolable crying for 6 hours. The episodes have occurred at roughly 15-minute intervals and have each lasted a few minutes before subsiding. He has also vomited twice since these episodes began. His temperature is 36.9°C, pulse is 125/min, respirations are 33/min, and blood pressure is 90/55 mm Hg. During the examination, the patient draws his knees up to his chest. Shortly thereafter, he passes red currant jelly stool. Abdominal examination shows a palpable abdominal mass in the right lower quadrant. Which of the following is the most likely diagnosis?

- A. Acute Appendicitis
- B. Gastroenteritis
- C.
- D. Intussusception

57. Case of patient had 166 Na. what is the appropriate time to correct his sodium?

- A. 36-48 hours
- B. 12-24 hours
- C.
- D.

58. Case of DKA, Na 130, glucose 700. What is the next step in management?

- A. Rehydration slowly with 0.9 NS
- B.
- C.
- D.

60. Case of suspected meningitis (neck stiffness etc..) which of the following CSF finding indicate bacterial meningitis

- A. Turbid colour - >95% polymorph - glucose < 50
- B. clear colour - lymphocytes predominant - >50 glucose
- C.
- D.

1- A 3month, Preterm maybe, was in the ICU for couple of weeks, Then has been discharged, يخافون عليه من ال

Sever Lower resp infection

وش انتصح بهالحالة؟

- A. stick with the vaccination schedule
- B. Palizumab injection once per month during winter

C omalizumab injection once per month during winter

D

2- :All of the following cause hypoglycemia AT THE beginning except?

- A salicylate
- B
- C
- D

3- chronic bloody diarrhea and came to hospital and two previous admissions, with bilateral wheeze and coarse crackles.

- A cystic fibrosis
- B
- C
- D

5- A 12 year old girl presents with a two week history of fever, polyarthralgia. She looks well but pale. Urine analysis shows traces of protein and blood

- A rheumatic fever
- B systemic juvenile rheumatoid arthritis
- C systemic lupus erythematosus
- D Goodpasture syndrome

6- On urine dipstick which of the following is heme negative?

- A alport syndrome
- B urate crystals
- C Poststreptococcus glomeronephritis
- D

14. a 14 years old girl travelled to Bahrain 2 weeks ago and went to multiple swimming pools. She developed jaundice, etc two weeks later. A month ago, she donated blood at Al-yamamah hospital. HBsAg: negative, Anti-HBsAg: positive

- A-hepatitis A
 - B-hepatitis B
 - C-Hepatitis C
 - D-Hepatitis D
- Answer: A

10-fever sore throat (image unilateral erythematous adenopathy



- A-adenovirus
 - B-s.aureus
 - C-EBV
 - D-CAT scratch disease (كان اسم ال causative agent)
- ال caustive agent)

11-3 years

- A-draw cross and sqoure
- B-stand on one foot for 10s
- C. يفهمونه الناس وهو يتكلم

13 case about Down syndrome (they write the dysmorphic features of Down syndrome like low set ears and flat nasal bridge) what of the following is most likely seen with this patient?

A atrioventricular septal defect

B TOF

C

D

14- case of mother considering when to give her baby vitamin D supplements?

A within the 1st month

B

C

D

15- introduces vitamin D to excessively breastfeeding baby?

A 1st month of life

B

C

D

16- Patient with fever after taking history and doing physical examinations and investigation everything came negative

Ans/

A-Fever of unknown origin

B-Persistent fever

C-Intermittent fever

A They are more predisposed to thrombosis

17- a case about proteinuria what's right?

B

C

D

27- periorbital edema , lower extremity edema , frothy urine , what is consistent with this condition?

- prone to thrombosis.

26. A 2-year-old boy is brought to the general practitioner by his mother. He has skin rash in both his feet. He also has a few lesions on both hands and tongue. They are vesicular and tender to touch. He is well in himself. Other family members are healthy. What is the most likely cause?

A. Coxsackie A

B. Flea bites

Hand-Fart-Mouth disersé

C. Mumos

D. Ebolavirus

37. 18 days old baby girl, delivered prematurely at 27 weeks gestational age with birth weight of 850 gm. The baby is on full feeding by nasogastric tube. The bedside nurse addressed that the baby had abdominal distention and vomiting and passed a smear of blood in the stool. On examination, the baby looks

lethargic, with frequent apneas and an increase in abdominal girth by 2 cm compared with 2 days ago. Blood work showed thrombocytopenia and sodium level was 130mmol/L (normal range from 135-145mmol/L). What would you find in the X-ray? (Similar case)

A. Pneumatosis intestinalis

6. Child with moderate dehydration his weight is 8kg with sodium 155. Calculate the maintenance?

A-Normal saline 0.09% D5 32 ml per hour.

B-Normal saline 0.09% DS 42 ml per hour.

C-Normal saline 0.45% D5 32 ml per hour.

D-Normal saline 0.45% D5 42 ml per hour.

13 case about Down syndrome (they write the dysmorphic features of Down syndrome like low set ears and flat nasal bridge) what of the following is most likely seen with this patient?

A atrioventricular septal defect

B TOF

C

D

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D

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D

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Ans/

A-Fever of unknown origin

B-Persistent fever

C-Intermittent feve

← الجواب A They are more predisposed to thrombosis

← السؤال 17- a case about proteinuria what's right?

B

C

D

27- periorbital edema , lower extremity edema , frothy urine , what is consistent with this condition?

- prone to thrombosis.

3 years child what's expected?

language understandable to strangers.

vesicular rash in immunocompromised, what is the complication?

*necrotizing fasciitis by group a strept

(picture of pupura in buttocks and lower limbs) , what will you see ?

* IgA deposits.

unilateral tender erythema lymphadenopathy, what is the organism ?

s. aureus.

whats true about pneumococcal conjugate vaccine ?

conjugated to a protein carrier.

10 days of fever , no symptoms and nothing on investigation and examination, what type of fever ?

what are lab results of G6PD?

- a. high reticulocytes count.
- b. high Haptoglobin

28- pt with asthma and he used drugs (I don't remember the names). The patient complete 4 weeks without using these drugs , awakening in night and asthma doesn't affect his life, what to do in his treatment?

A step down according to asthma action

- C
- D

A 10-months-old infant who weighs 8kg is admitted to the pediatric ward. The physician orders the fluids to run at as a maintenance. At which rate should the infant's intravenous fluids be infused?

- A. 8 mL/hr
- B. 17 mL/hr
- C. 24 mL/hr
- D. 32 mL/hr

What is the appropriate daily fluid requirement for a 18 kg child?

A. 1400ml

A 14-year-old patient with constitutional growth delay, the parents ask about their child's length in adulthood. Which of the following is true? (the question was not literally like this, but it had the same meaning) (He's asking about the meaning of constitutional growth delay)
Bone age is normal to chronological age

36- 18 months boy with cough and runny nose,temp (37.9) with clear nasal discharge , alert and play , what is true?

- A - is undefined immunodeficiency
- B - fever is absolute contraindication after first vaccine
- D -
- C - minor illness and fever is not contraindication of immunization

37-MENINGOCOCCAL conjugate vaccine whats true about it

- A-its given at birth
- B- its given for small children
- C
- D

38-child with Constitutional Delay what's true about hes future

- A- he will be short
- B- his height and puberty will be completely normal
- C
- D

39- long case about asthmatics patient came with exacerbating attack , blood gas was almost like ph 7.49 , co was low bicarb was 21 , what gas changes he is shown

- A respiratory alkalosis
- B respiratory acidosis

C metabolic acidosis
D metabolic alkalosis

40- which one of the following poisoning wouldn't cause hypoglycemia at the beginning

A - salicylate
B- acetaminophen
C
D

41- case about a child was having excessive saliva secretion, drooling, stridor and asked about diagnosis

1- epiglottitis

42- case about pyloric stenosis and asked what is the first step in management

A- pediatric surgery consultation
B- iv fluid and electrolytes correction

43- they gave a pic of HSP and asked about whats true about diagnosis?

A- Histopathology; vasculitis with predominant IgA deposit

44- what is the heart defect in Down syndrome patients

A- ASD
B- TOF
C- atrioventricular defect

45- case about child with soft pansystolic murmur asked about diagnosis

A- large VSD

46- uni lateral , tender and pain what is the most likely organism

A- EBV
B- s.aureus

47- what is a major criteria to diagnose Rheumatic disease

A- recurrent GAS infection
B- carditis

48- newborn in they were interested to give him an IM injection at birth what would it be

A- Vit K

Q1/ An 8-year-old boy was seen in the clinic for evaluation of nighttime bedwetting. He has never been dry at night and has urinary accidents 3-4 nights per week. The boy's parents report that he has been completely continent during the day since 3 years of age. He has no history of urinary frequency, urgency, or hesitancy. He has a normal daily bowel movement and has not had fecal soiling. There is no snoring or breathing issues during sleep. He is embarrassed to sleep at his cousin's house. His father also had nighttime bedwetting until he was 9 years old. He is at the 50th percentile for weight and height. He has no skin lesions on his back. His abdomen is soft with no organomegaly. His neurological examination findings are normal. A urinalysis has normal results. Which of the following is considered the best next step?

A. A trial of decreasing fluids at bedtime and use of bedwetting alarms
B. A trial of oxybutynin
C. A trial of imipramin
D. Request for lower spine MRI

Q2/ a long case about 1 year old male, looks ill his fever 36.5, he refused to feed since 2days, he didn't urinate much these priod, the mother when she changed his diaper she found pink color on the diaper, and the tender in palpating pubic area , what is the next to do forthis baby?

- A) Transurethral Catheterization (confirmed by dr)
- B) Renal Ultrasound
- C) VCUG

Q3/ Case 5 months, bilateral wheezing, something like not feeding well, diagnosed with bronchulitis?

(the question already mentioned the diagnosis)

- A- oral antibiotics
- B- corticosteroids
- C- IV fluid + I naso gastric type
- D- albuterol

Q4/ Which of the following is not correct?

- A) don't exceed wthi ventaltion
- B) Rate 100bpm
- C) Need complete recoil
- D) Compression $\frac{2}{3}$ of anterior to posterity

Q6/ In salicylates poisoning hyperglycemia (early) them hypoglycemia (late) described. which of the following is an indication to administer IV glucose?

- A- vomiting
- B- convulsions
- C- diaphoresis
- D- altered mental status

Q8/ a 5 yo diagnosed with asthma what is appropriate way to take the treatment?

- A) Spacer + metered dose
- B) MDI
- C) Dry powder

Q10/ A 6-week-old infant presented with a persistent non bilious vomiting. On examination, he was dehydrated with visible peristalsis on abdominal examination. What is the most likely electrolyte disturbance that would be found on laboratory investigation?

- 1. Hypokalemia, hypernatremia, metabolic acidosis
- 2. Hypokalemia, hypochloremia, metabolic alkalosis
- 3. Hypokalemia, hypochloremia, metabolic acidosis
- 4. Hypokalemia, hypernatremia, metabolic

Q11/ A 13-month-old child presented to the emergency department with severe abdominal pain and irritability. The mother gave a history of constipation with small amount of black blood passed per rectum. The child has on and off episodes of crying with drawing of legs to the abdomen. In between the episodes, the child is calm and relieved. On examination, the vital signs are stable and the child is dehydrated. What is the most likely diagnosis? (similar; maroon colored diarrhea)

- 1. Intussusception
- 2. Appendicitis
- 3. Celiac
- 4. IBD

Q12/ A 4 years baby boy has Otitis media, Recurrent pneumonia, also his uncle has the same problem ?

- A) Quantitative immunoglobulin
- B) Flow T cell cytometry
- C) X-ray to check of thymus
- D) diaphtheria Ab

Q13/ Baby girl, she can transfer object, sit with support, mimicking voices what her age?

- A. 6 months
- B. 4 Months
- C. 3 Months
- D. 9 Months

Q14/ The mother ask you when her child can ride tricycle, draw a circle?

- A. 20 Months
- B. 36 Months
- C. 16 Months

Q15/ A newborn has a significant enlargement of the head circumference, with bulging of anterior fontanel, otherwise healthy: which next step you will do?

- A) CT scan, it's craniocytosis
- B) Reassure it's a variable genetic
- C) It's congenital hydrocephalus, Transcranial Ultrasound
- D) Seizure, EEG (something I forgot)

Q17/ Case about scarlet fever it was very clear (pt with prnlgd fever with sore throat, maculapaular rash, sand paper, strawberry tounge what is the causative agent?

- A) GAS
- B) S.aurus
- C) Coxa

Q18/ A girl diagnosed with persistant oligoarthritis involved four joints (knee, elbow, wrist, ankle),
+ve ANCA

which next step you will do ?

- A) Slit lamp
- B) Monitoring the patient through ESR
- C)

Q22/ Unilateral red covering the eyelid and forehead, what is this abnormal cutaneous?

- A) port wine
- B) Salmon patch

Q23/ kid receives varicella ig , what is the type of immunity?

- A) passive artificial immunity
- B) Active artificial
- C) Passive natural
- D) Active natural

Q24/ a long case mention that there was an ejection systolic murmur in the upper left sternal and fixed split second heart sound ?

- A) left to right , ASD
- B) Right to left, ASD
- C) Right to left, VSD
- D) Left to right, VSD

Q25/ a preterm present with ataxia after perinatal damage there was a damage on the cortex and motor pathway, which of the following is your diagnosis?

- A) Spastic cerebral palsy
- B) Dyskintic cerebral palsy
- C) Mixed Cerebral palsy

Q26/ Patient with angioma and history of seizure, which of the following is consistent with tuber sclerosis?

- A) hypopigmented spot
- B) hyperpigmented macula on back
- C) Family history of renal carcinoma

Q28/ Case of vesicular papular rash, itchy, brother is immunocompromised. What's true about this virus?

- A. it's an RNA virus
- B. feco/oral transmission
- C. Necrotizing vasculitis caused by group A Strept.

Q29/ Pt developed jaundice, Mother O+ baby A+ , Coombs test +ve his condition is due to?

- A. ABO incompatibility
- B. Rh incompatibility
- C. Isoimmunization

Q30/ A 4 month old Unvaccinated patient has fever, vomiting and diarrhea without blood nor mucus. What is the likely causative agent?

- A. Influenza A
- B. Salmonella
- C. Rotavirus

Q31/ A 1 year old child present with Otitis media, Fever, looks well, what is your next step to manage the patient?

- A) discharge
- B) Start with high dose amoxicillin for 10 days

Q33/ A child presents with repeated seizures on physical examination cafe-au-lait spots on his body which of the following is most likely diagnosis?

- A- epilepsy with neurofibromatosis type 1
- B- epilepsy with neurofibromatosis type 2
- C- epilepsy with Sturge Weber syndrome
- D- epilepsy with TS

Q34/ A child presents with fever , lethargy, neck stiffness and irritability CSF analysis were done and showed turbid fluid with high WBC mainly polymorph high protein and low glucose which of the following is most likely diagnosis ?

- A- bacterial meningitis
- B- viral meningitis
- C- meningitis due to TB
- D- viral encephalitis

csf positive for lymphocytes, normal glucose and in the temporal lobe , what is the management
ما جاء كذا السؤال كان حاط
iv acyclovir.

Q35/ A 16 months old has been exclusively breastfeeding present with swelling in the arms which the following is seen in his labs ?

A- decreased calcium , increased phosphate, normal alkaline phosphatase , increased PTH
B- decreased calcium , decreased phosphate , decreased alkaline phosphatase , decreased

PTH

C- increased calcium , decreased phosphate , decreased alkaline phosphatase, increased

PTH

D- decreased calcium , decreased phosphate , increased alkaline phosphatase , increased

PTH

Q36/ 54.9 years old boy recently diagnosed with nephrotic syndrome, which of the following is indication for renal biopsy in child with nephrotic syndrome ?

A-Age between 1 and 8

B-Protein/creatinine ration of >2.5 mg/ dl

C-Serum albumin concentration less than 2.5 g/dl

D-Hypertension

(It was about minimal change diseases, which is true)

what is true about minimal change

- most common nephrotic syndrome in childhood.

- normal lipid and cholesterol.

- resistant to steroids.

- Common to cause End stage renal failure in 5-10yrs

Q38/ Present for one month with polydipsia, polyurea, weight loss. Which of the following will not seen:

1- metabolic alkalosis

2- hyperglycemia

3- kussmal breathing

4- dehydration

Q40/ 12-month-old boy manifests failure to thrive, poor appetite, abdominal distention, diarrhea, and irritability. He had been well until 9 months of age. Thereafter, he was weaned from breast milk to regular foods. The most likely diagnosis is?

A. Giardiasis .

B. Celiac Disease

C. Cystic Fibrosis

D. Pancreatitis

Q43/ Clear case about Bilaty atresia, there was pale stool, the direct bilirubin is high. What is the golden standard for biliary atresia

A. ALT (elevated liver enzymes)

B. US Ultrasound

C. Liver biopsy

Q45/ child brought to ER with fever and pain on the right side after a sliding accident, her mother noticed abdominal distention on the right, and examination showed high BP and hematuria, what is the most likely diagnosis? (similar case)

A. Renal contusion

B. Wilms tumor

C. neuroblastoma

Q46/ features of trisomy 21?

A. single crease + hypotonia

Q47/ *newborn baby develops apnea , what is the cause ?

A. immature central control

B. wide nasal area

Q49/ 22 kg Maintenance iv rate ?

A. 62

B. 100

C. 400

D. 32

Q52/ A case about a young boy who's always had a large head since he was born (Above the 95th percentile). Upon further history taking, his father informs you that most family members have the same condition (having a large head). What's the next step in management?

A. Genetic testing

B. Reassurance

C. CT head

Q53/ case of focal epilepsy (staring and motionless, frequency 2-3 times per week and epigastric symptoms before seizure)

A. Juvenile myoclonic syndrome

B. Childhood absence syndrome

C. Focal epilepsy

Q54/ case of recurrent pneumonia, eczema and easy bleeding, what other finding will confirm diagnosis?

A. Small platelets on peripheral smear

B.

Q55/

Q56/ child with 2 days history of vomiting and diarrhea, upon examination there is tachypnea, tachycardia dry mucous membranes and poor skin turgor, what is your next step in fluid management?

A. bolus of normal saline 20 mg/kg

B. D5 0.45% normal saline on maintenance rate

C.

D.

Q59/ Pt have abdominal distention and ascites, under FTT?

A. Marasmus

B. Kwashi

1- someone was given Ab against varicella and asking what type of immunity is this

A- active artificial

B- active natural

C- passive artificial

D- passive natural

2- case about a BOY with growth chart and in the case delayed bone age and height

A- turner syndrome

B- growth hormone deficiency

C- skeletal dysplasia

3- another case with growth chart about a 14 year old girl , bone age was delayed, tanner stage 2 of breast , no pubic hair growth no menstruation, her dad also was delayed for height then now he's about 177cm

A- constitutional delay

B- turner syndrome

C- Familial short stature

4- An 8-year-old boy was seen in the clinic for evaluation of nighttime bedwetting. He has never been dry at night and has urinary accidents 3-4 nights per week. The boy's parents report that he has been completely continent during the day since 3 years of age. He has no history of urinary frequency, urgency, or hesitancy. He has a normal daily bowel movement and has not had fecal soiling. There are no snoring or breathing issues during sleep. He is embarrassed to sleep at his cousin's house. His father also had nighttime bedwetting until he was 9 years old. He is at the 50th percentile for weight and height. He has no skin lesions on his back. His abdomen is soft with no organomegaly. His neurological examination findings are normal. A urinalysis has normal results. Which of the following is considered the best next step?

A. A trial of decreasing fluids at bedtime and use of bedwetting alarms

B. A trial of oxybutynin

C. A trial of imipramine

D. Request for lower spine MRI

5- a case about a child with sandpaper rash with throat infection (pt with prolonged fever with sore throat, maculopapular rash, sand paper, strawberry tongue what is the causative agent) and asked about most likely organism

A- s.aureus

B- GAS

C- kawasaki

7- A 1-year old boy is brought to the clinic for evaluation after 2 days of low-grade fever, irritability, and decreased activity. His mother reports that he has ear pain (ear pulling) , His external ear canal is normal, Unilateral bulging of tympanic membrane and erythema, what is the best next step in management?

A. give amoxicillin for 10 days

8- The following are part of high quality CPR, EXCEPT :

A. allow the chest to fully recoil

B. Compressions are at least 100 per/min

C. Chest should be depressed at least 2/3 of its AP

D. Minimize interruption

9- In salicylates poisoning which of the following is indication to administer IV glucose

A. vomiting ¹

B. convulsions

C. diaphoresis

D. altered mental status

10- A girl was diagnosed with Oligo JIA involved four joints (knee, elbow, wrist, ankle) and she was significantly positive for ANA. What is the best thing to do next ?

A. To monitor ANA

B. Slit lamp examination (uveitis)

11- a case about a boy with palpable purpura on lower extremities and buttocks , and joint pain with abdominal pain and asked what immunoglobulin is involved?

A- IgA

B- IgG

C- IgM

12- case about CHD with An ejection systolic murmur best heard at the upper left sternal edge, fixed and widely split second heart sound asked about most likely diagnosis

A- left to right shunt ASD

B- Right to left ASD

C- left to right VSD

13- a child was kept in hospital due to child neglect, then noted to have pedal edema, abdominal ascites , hepatomegaly he was apathetic and hard to console, most likely diagnosis ?

A- nephrotic syndrome

B- marasmus

C- Kwashiorkor

14- 7 days old developed Small, red macules and papules that progress to pustules white to yellow no erythema noted what is the diagnosis

A- Erythema Toxicum Neonatorum

B- transient pustular melanosis (not sure)

15- a child was diagnosed with minimal change disease what's true about it

A- most common nephrotic disease at childhood

16- a mother give her 6 month old baby a cow milk he is at risk of? **السؤال طويل** long question

A- IDA

17- a child with fever , headache and behavioral changes, the did a CSF analysis turned to be clear , glucose is normal, protein slightly increased , and lymphocyte, imaging was done showed temporal lobe localization, what is the best next thing in management

A- IV acyclovir

18- Rides a tricycle and draws a cross what is the expected age?

A. 16 months

B. 24 months

C. 30 months

D. 36 months

19- A baby presented to your clinic. He is able to sit with support, babble, and now it's just started to transfer objects between hands. What is his age?

A. 10 month

B. 4 months

C. 8 months

D. 6 months

20- 5-year-old boy presents to his pediatrician with a history of eczema, rhinitis, chronic nocturnal cough and intermittent wheeze. Asthma is suspected and a bronchodilator is prescribed. How should his bronchodilator be delivered?

- A. Dry powder inhaler
- B. Metered dose inhaler with spacer
- C. Nebulizer with face mask

21- 12 hours old baby girl, noticed to have yellowish discoloration of the eyes and skin, the mother blood group is O tve and the baby blood group is A tve, serum bilirubin level came 12 mg/dL.. The baby was admitted to NICU for phototherapy: what is most likely diagnosis and test to be done

- A. Immune issay due to ABO incompatibility شىء زى كذا

22- Two weeks infant on formula since birth with yellowish sclera and skin, poor feeding, vomiting. dark urine.pale stool. Bilirubin test showed T otal Bilirubin 157, high direct bilirubin, asked about gold standard to diagnose this case

- A- abdomen US
- B- liver biopsy

(not sure about the answer)

24- A 13 year old girl presents with episodes of staring and becoming motionless this happen 3 times a week with a duration of 2 minutes which is affecting her school performance during the episode she feels abdominal pain neurological exam is normal which of the following is the most likely cause of her condition ?

- A. childhood absence epilepsy
- B. juvenile myoclonic epilepsy
- C. focal epilepsy

25- a case about a mom who found an abdominal mass for her daughter, she's hypertensive etc (idont really remember)

- A- wilms tumor
- B- nueroblastoma

26- child previously healthy until yesterday sudden acute cyclical colicky abdominal pain not relieved except when he pull his legs to his abdomen , (dark red stool) vomiting, was noted too , most likely diagnosis

- A- intussusception
- B- Appendicitis

27- A 6-week-old infant presented with a persistent non bilious vomiting. On examination, he was dehydrated with visible peristalsis on abdominal examination. What is the most likely electrolyte disturbance that would be found on laboratory investigation?

- 1. Hypokalemia, hypernatremia, metabolic acidosis
- 2. Hypokalemia, hypochloremia, metabolic alkalosis
- 3. Hypokalemia, hypochloremia, metabolic acidosis
- 4. Hypokalemia, hypernatremia, metabolic

28- a case about GAS infection (gave symptoms of RF) asked how to confirm

- A- ASO

30- a case about a boy with eczema and recurrent infection (immunodeficiency case) asked what will confirm the diagnosis (wiskott case)

- A- small platelet size Peripheral blood smear

1-baby presented to your clinic. He is able to sit with support, babble, and now it's just started to transfer objects between hands. What is his age?

- A-10 month
- B-4 months
- C-8 months
- D-6 months

Q1) A 2-day-old full term neonate experiences multiple firm yellow-white, 1-2mm papules with a surrounding erythematous flare on the trunk. Wright stain of the lesions shows numerous eosinophils. He is active and feeding well. What is the LIKELY diagnosis?

- A-Herpes simplex infection
- B-Pustular melanosis
- C-Erythema toxicum NOT SURE!
- D-Folliculitis

Q89) 2 years old boy presents with a history of vomiting and diarrhea for several days. Physical examination reveals lethargy, tachycardia, and dry mucous and poor skin turgor. He refuses to take oral fluids, so you decide to begin intravenous fluid administration. What is the best next step in fluid management?

- A-3% saline at 10ml/kg
- B-5% dextrose and 0.45 NS at maintenance rate
- C-Wait for laboratory results for serum electrolytes
- D-Bolus of isotonic fluid at 20ml/kg

1. A Child came to your clinic as the photo shows. What is your management?



- IV/Oral antibiotics.

2. 13 case about Down syndrome (they write the dysmorphic features of Down syndrome like low set ears and flat nasal bridge) what of the following is most likely seen with this patient?

- A. atrioventricular septal defect V
- B. TOF

3. 6 year old female came to emergency with red urine, she is in good health and show no symptoms otherwise what could cause this reading in the dipstick?

- A. Myoglobinuria.
- B. Urate crystals.

4. 10 week old boy normal delivery at 37weeks 3.6kg, no family history normal reading except Hb 10 what is the cause?

- A. Iron deficiency anemia.
- B. Physiological anemia.

5. Girl with migratory arthritis and previous positive GAS infection, what is true about this condition?

- A. Unpainful joints.
- B. Heal without deformity.
- C. Affect large and small joints.

6. Unvaccinated child who has excessive saliva secretion and drooling what is the diagnosis ?

A. Epiglottitis.

7. Sick neonate with features of hypoxic ischemic encephalopathy (HIE). What is your management?

A. Therapeutic hypothermia.

10. 18 months boy, what he can do?

A. Copies line.

B. Handle spoon well.

C. Help to undress (doctor said this is the right answer).

D. Can stand on one leg for 10 sec.

11. Patient came to you with group A streptococcus pharyngitis you will treat him to avoid which complication?

A. Otitis media.

B. Post streptococcal glomerulonephritis.

12. Which of the following consider abnormal?

A. 3 months old with weight crossbond 10th percental.

B. 10 years old with BMI less than 50 percental.

C. 6 months old female with her hight crossbond 20th percental.

D. 7 years old boy has a decrease in weight from 90th to 25th in one year.

13. Long case and he was having soft pansystolic murmur?

A. Large VSD.

14. 4 months, what can he do?

A. Reaches for objects.

15. What vaccine is given to the children at 6 months?

A. BCG.

16. Goat milk could cause?

A. Folate deficiency.

17. A child was planning to take MMR vaccine, what could effect his vaccine?

A. IG last month.

18. How to calculate anion gap?

A. Sodium – (chloride + bicarbonate).

19. Patients had AKI then he developed HUS. His lab results will show?

A. Hypocalcemia.

20. Pateint came with BP of 120/70 was in the 90th to 95th percentile and a BMI of 25 what is the next step in management?

A. Life modification and increase physical activities.

21. 6Kg and 10% dehydration. Calculates the maintenance and deficit?

A. 600-600.

22. Child diagnosed with asthma but his parents refuse treatment, at counseling what would you tell them ?

- A. treatment has shown to have adverse side effects.
- B. exacerbation of asthma may cause damage and remodeling.

23. 4 months unvaccinated infants present with fever (39c), nausea and vomiting....?

- A. S. Pneumonia.
- B. GBS.
- C. GAS.

24. 14 years old, what is the cause of pneumonia?

- A. Mycoplasma pneumoniae.

25. Marked for kidney biopsy?

- A. Hypertension.

26. A 14 year old child with asthma on bronchodilator. He can exercise normally, he reported no night waking. He

had one exacerbation last year treated in ER with nebulizer, corticosteroid, oxygen. What is the state of his asthma?

- A. well controlled.
- B. partially controlled.
- C. poor controlled.

27. diplococci gram positive on culture?

- A. Streptococcus pneumoniae.

28. Constitutional growth delay?

- Bone age less than chronological age.

37. Q about the route of fluid hypertonic/hypotonic from extra cellular to intracellular.

38. A 4 year old girl present to the ER due to severe sudden wheezing with no fever and no prior episodes.

History revealed

her and her brother received gifts, with the brother getting a toy car with tires which can be removed, which of the

following is the next step of investigation?

- A. chest CT.
- B. decubitus film.
- C. inhalation and exhalation film (answer by dr. Dana).
- D. chest ultrasound.

1- A 4-year-old girl presented with fever slapped cheeks rash, she has 2 brothers with sickle cell anemia, what complications will they have if the infection transmits?

- A- Transient aplastic crises
- B- Spleen rupture
- C- Pure cell aplasia

2- Patient presented with +ANA Oligo juvenile idiopathic arthritis, other than uveitis what would be a systemic complication of this disease?

- A- Growth delay
- B- Macrophage activation syndrome

4- A 30 kg child what is the maintenance?

A- 1700

B- 1500

6- New mother was wondering when to introduce solid food to her baby?

A- 2 months

B- 6 months

C- 9 months

D- Any age

7- A 4 year old came to the clinic complaining of freckles on the axilla , café spots and some other neuro manifestation. What was the diagnosis?

A. NF1

B. Tuberous sclerosis

8- You are evaluating a 5-year-old boy who presented to the emergency department with a history of fever for two days (39°C), swelling, and tenderness in his right eye. Local examination of his right eye revealed proptosis and limited eye movement. The rest of the exam is unremarkable. What is the appropriate management?

A. Oral antibiotics and discharge home with close follow-up in the clinic

B. Urgent ophthalmological examination and IV antibiotics

C. To do MRI Brain

D. IV antibiotics only

9- 4 weeks infant on formula since birth with yellowish sclera and skin, poor feeding, vomiting, dark urine, pale stool. Bilirubin test showed Total Bilirubin 157, high direct bilirubin, imaging test was done. Which of the following is most important to investigate?

A. Biliary atresia

B. Hemolytic blood disorder

C. Physiological breastfeeding hyperbilirubinemia

14- A child has gastroenteritis and diarrhea from Rota after treatment and going home the mother supplied him with cow milk, later he developed diarrhea again which of the following is more likely?

A- give the child lactose free milk for a period of time

B- give him diluted milk until symptoms resolve

15- 18-month-old child presents to the emergency center having had a brief, generalized tonic-clonic seizure. He is now postictal and has a temperature of 40°C (104°F). During the lumbar puncture (which ultimately proves to be normal), he has a large, watery stool that has both blood and mucus in it. Which of the following is the most likely diagnosis in this patient?

A- Salmonella

B- Enterovirus

C- Rotavirus

D- Shigella

16- Case with symptoms of encephalitis and with following CSF findings: clear, mild increase in WBCs, normal glucose.

- A. viral encephalitis
- B. bacterial meningitis
- C. TB
- D. autoimmune encephalitis

17- Baby girl, she can transfer object, sit with support, mimicking voices what is her age?

- A. 6 months
- B. 4 Months
- C. 3 Months
- D. 9 Months

18- Unilateral red covering the eyelid and forehead, what is this abnormal cutaneous?

- A) port wine
- B) Salmon patch

19- Pt developed jaundice, Mother O+ baby A+, Coombs test +ve his condition is due to?

- A. ABO incompatibility
- B. Rh incompatibility
- C. Isoimmunization

22- 12-month-old boy manifests failure to thrive, poor appetite, abdominal distention, diarrhea, and irritability. He had been well until 9 months of age. Thereafter, he was weaned from breast milk to regular foods. The most likely diagnosis is?

- A. Giardiasis
- B. Celiac Disease
- C. Cystic Fibrosis
- D. Pancreatitis

23- Clear case about Biliary atresia, there was pale stool, the direct bilirubin is high. What is the golden standard for biliary atresia

- A. ALT (elevated liver enzymes)
- B. US Ultrasound
- C. Liver biopsy

24- case about a mom who found an abdominal mass for her daughter, she's hypertensive what is the diagnosis?

- A- Wilms tumor
- C- neuroblastoma

25- case about a boy with palpable purpura on lower extremities and buttocks, and joint pain with abdominal pain

and asked what immunoglobulin is involved?

- A- IgA
- B- IgG
- D- IgM

26- mother give her 6-month-old babe a cow milk he is at risk for which of the following?

- A- Iron deficiency anemia

30-13 years old girl amenorrhea short(said how tall she is) her mother is (taller) father is (taller) what is most likely diagnosis for this?

- A-Testicular feminization
- B-Turner syndrome

31- 18-month-old female child presents with a blood-streaked stool. The stool is positive on occult blood testing. Which of the following diagnoses is most likely?

- a) Anal fissure.
- b) Peptic ulcer disease,
- c) Mallory- Weiss tear
- d) Inflammatory bowel disease.

32- A 9 months old presented to the clinic with a fever of 38.7°C and ear pulling. On physical examination you found erythematous bulging right tympanic membrane with purulent effusion. You diagnosed him with otitis media. What is the most likely organism that caused this infection?

- A. Group b streptococcus
- B. Streptococcus pneumoniae
- C. Group a streptococcus
- D. Streptococcus viridans.

33- Type 1 diabetic girl presenting with fever, diarrhea, vomiting for 3 days. She appeared well hydrated. Her fasting blood glucose is 260 mg/dl and normal bicarbonate levels, Urine analysis showed ketone bodies. After adjusting the insulin dose.

What is the next step?

- A: admission immediately
- B: telling her to measure her urine ketones daily
- C: telling her to measure her blood glucose daily
- C- tell her to increase her hydration

35-A case about a patient with recurrent chest infection and oral thrush, labs show neutropenia, leukopenia and further investigation revealed no T-cells, but B-cell and NK are intact What is your diagnosis?

- A-SCID
- B- X-linked Agammaglobulinemia
- C- hyper IgM
- D- Wiskott-Aldrich syndrome

36- DiGeorge is a defect in which of the following?

- A- humoral
- B- Cellular
- C- Complement
- D- Phagocytic

37-9-month child with a bulging fontanelle and head size above the 98% percentile and has hydrocephalus which of following is done next?

- A- Head CT
- B- Start diuretic treatment and measure later
- C- Head X-ray

38- A case about a boy with a previous history of Upper respiratory tract infection 1 week ago as well constipation presents to you now with an odd smelling urine which of the following is a risk factor for this condition?

- A- URTI
- B- Male urethra
- C- Constipation

Do Posterior urethral valve مب متأكد إذا
موب متأكد إذا جاء هذا الخيار

39- 2-year-old patient took IVIG a week ago for Kawasaki disease, and according to Saudi Arabian vaccine schedule he should get his 2-year vaccine this week, what effects will this have on the IVIG?

- A- decrease the immune response
- B- augment the immune response
- C- delay the immune response
- D- unchanged immune response (not sure)

40- A patient with malar rash Abdominal pain and arthritis and later developed proteinuria, what is your diagnosis?

- A-SLE
- B- Rheumatoid Arthritis

41- A case about cerebral palsy and asked which of the following statements is correct?

- 1. Diplegia CP risk increases with prematurity

42- patient came with proteinuria dipstick no symptoms or complications seen otherwise what is your next step in investigation?

- A- Kidney biopsy
- B- Renal US
- C- complement level
- D- See the protein/creatinine ratio

44-The expected post-natal normal cardiac and respiratory transition is very necessary for the newborn to achieve the normal adult flow circulation. Which of the following physiological changes is expected to happen in post-natal transition?

- A. Pulmonary vascular resistance decreases
- B. Decrease pulmonic flow
- C. Ductus arteriosus remains patent
- D. Increased prostaglandins concentration

46-a question about how to measure the anion gap

- A- $NA - (CL + \text{bicarbonate})$

47- Most serious bacterial infection for a febrile neonate?

- A. E. coli septicemia

49-2 weeks old baby girl, delivered prematurely at 27 weeks gestational age with birth weight of 850 gm. She developed frequent apneas with desaturations. Her abdomen was distended, and she passed stool mixed with blood. The orogastric tube was draining greenish output. Initial blood work revealed metabolic acidosis, thrombocytopenia, and low sodium. The most likely diagnosis that can explain the above-mentioned scenario is:

- a) Apnea of prematurity
- b) Patent ductus arteriosus
- c) Pyloric stenosis
- D) Necrotizing Enterocolitis

50- What is the first sign of female puberty?

- a- Formation of breast bud

51- would find in examination?

- A. bilateral exudative conjunctivitis
- B. Cracked lip

52-Patient was diagnosed with asthma what drug should he start with?

- A- SABA as needed
- B- SABA + ICS as needed

53- long case about asthmatics patient came with exacerbating attack, blood gas was pH 7.49, CO₂ was low bicarb was 21, what gas changes he is showing?

- A respiratory alkalosis
- B respiratory acidosis
- C metabolic acidosis
- D metabolic alkalosis

55-A 3-months child with symptoms of DDH and asked about the best screening method

- A- US

B- X-ray

56- An 8 months old child, he is exclusively breast feed and she informs you that she feeds him well, his mother is concerned he is not gaining weight what is the most likely cause?

A- psychosocial

C- inadequate calorie intake

57- Case about child with cardiomegaly, soft pansystolic murmur asked about diagnosis

A-Large VSD

B-Small VSD

C-TOF

D-ASD

58- Case about a child born in his home and exclusively breast feed without taking any supplements which of the following is he at risk for?

A-Rickets

2. A 6 day old birth weight 3000 g and then came with a birth weight of 2950g?

A-Reassure

B-CBC LFT and urinalysis

C-Admit

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A-Reassure

B-CBC LFT and urinalysis

C-Admit

8- The following are part of high quality CPR, EXCEPT :

A allow the chest to fully recoil

A-Compressions are at least 100 per/min

B-Chest should be depressed at least 2/3 of its AP

C-Minimize interruption

Q6/ In salicylates poisoning hyperglycemia (early) then hypoglycemia (late) described. which of the following is an indication to administer IV glucose?

A-vomiting

B-convulsions

C-diaphoresis

D-altered mental status M

7-A 1-year old boy is brought to the clinic for evaluation alter 2 days of low-grade fever. irritability, and decreased activity. His mother reports that he has ear pain (ear pulling). His external ear canal is normal. Unilateral bulging of tympanic membrane and erythema. what is the best next step in management?
 A give amoxicillin for 10 days

36. Viral lymphadenitis. جاء سؤال نسبياً تفاصيله بين كان الجواب

Viral Lymphadenitis	Most common form of reactive lymphadenopathy
Common viruses involved:	1. Adenovirus 3. Coxsackie virus A and B 2. Rhinovirus 4. EBV
- Lymphadenopathy often bilateral, diffuse, non-tender	
- Other Signs/Symptoms are consistent with URI	
Management is expectant but they are often biopsied due to slow regression	
Pathologic examination: Nodal architecture and hilar vascularity are normal	

37. Q about the route of fluid hypertonic/hypotonic from extra cellular to intracellular.
 Common Respirary

38. A4 year old girl present to the ER due to severe sudden wheezing with no fever and no prior episodes. History revealed her and her brother received gifts, with the brother getting a toy car with tires which can be removed, which of the following is the next step of investigation? chest CT.
 decubitus film.
 inhalation and exhalation film (answer by dr. Dana).
 D. chest ultrasound.

39. A child presents with fever, lethargy, neck stiffness and irritability CF analysis were done and showed turbid fluid with high WBC mainly polymorph high protein and low glucose which of the following is most likely diagnosis?
 A-meningitis due to TB **Bacterial meningitis**
 B-viral encephalitis
 Answer is A

24- A 13 year old girl presents with episodes of staring and becoming motionless. This happens 3 times a week with a duration of 2 minutes, which is affecting her school performance. During the episode she feels abdominal pain. Neurological exam is normal. Which of the following is the most likely cause of her condition?
 A. Childhood absence epilepsy
 B. Juvenile myoclonic epilepsy
C. Focal epilepsy

42- Case about pyloric stenosis and asked what is the first step in management.
 A- Pediatric surgery consultation
B- IV fluid and electrolytes correction

خَتَامًا...

إن أصبنا فمن الله وإن أخطأنا فمن أنفسنا والشيطان،
لا تنسوا القائمين على الملف من دعواتكم.

Contact:

 @IMC_444_bot